



GENERATIVE BIONICS

Analisi Strategica del Fit con i Bandi Horizon Europe
2026-2027

PORTFOLIO

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Executive Summary

Fit medio cluster

46.38%

Call analizzate

38

Top 3 call

#1 · HORIZON-HLTH-2027-03-TOOL-08

GO

Towards Artificial General Intelligence (AGI) for healthcare
Score: 65.18%

#2 · HORIZON-HLTH-2026-02-DISEASE-12

GO

European Partnership on Rare Diseases (ERDERA)
(Phase 2)
Score: 62.87%

#3 · HORIZON-HLTH-2026-01-TOOL-07

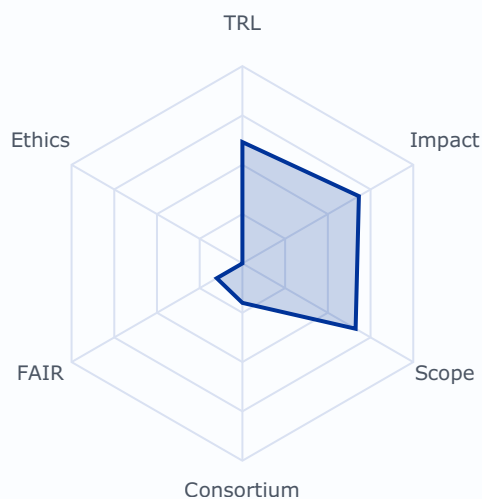
GO

Establishing a European network of Centres of Excellence
(CoEs) for Advanced Therapies Medicinal Products
(ATMPs)
Score: 62.50%

Distribuzione Top 10

#1		65.2
#2		62.9
#3		62.5
#4		62.0
#5		61.1
#6		60.4
#7		60.2
#8		60.0
#9		58.3
#10		49.5

Radar complessivo cluster



Rank #1 · HORIZON-HLTH-2027-03-TOOL-08

GO

Towards Artificial General Intelligence (AGI) for healthcare

Fit: 65.18 / 100 · Solver: Optimal · CR: 0.0168

Perché questa call ha preso 65/100

La call HORIZON-HLTH-2027-03-TOOL-08 ottiene un fit del 65.2% dopo ottimizzazione LP. Il criterio dominante e' excellence (contributo 32.2/100). Solver: Optimal, CR AHP: 0.0168.

"This topic aims at supporting activities that are enabling or contributing"

*"to one or several expected impacts of destination
"Developing and using new tools,""*

"technologies and digital solutions for a healthy society". To that end, proposals under this"

Gap principali da colmare

- ⚠ Implementation debole: piano esecutivo/consortium readiness non sufficiente
- ⚠ Requisito SME non soddisfatto dal profilo corrente

Azioni concrete (prossimi 3-6 mesi)

- ✅ Costruire shortlist partner (end-user, integrator, academia) e governance WP (90-120 giorni)
- ✅ Valutare ruolo da partner o struttura legale/cap table compatibile con requisiti SME

Spiegazione Tecnica AHP + LP

Matrice confronto a coppie (AHP)

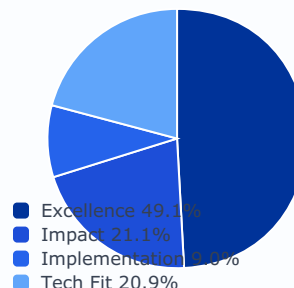
1.000	3.000	5.000	2.000
0.333	1.000	3.000	1.000
0.200	0.333	1.000	0.500
0.500	1.000	2.000	1.000

Pre-LP vs Post-LP

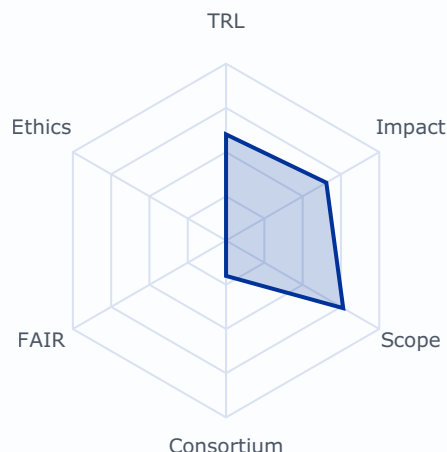
Criterio	Peso AHP	Local pre-LP	Optimal post-LP	Breakdown
excellence	0.4914	0.6546	0.6546	0.3216
impact	0.2105	0.7253	0.7253	0.1527
implementation	0.0896	0.4400	0.2000	0.0179
tech_fit	0.2086	0.7648	0.7648	0.1595

Consistency Ratio: 0.0168 — Matrice consistente

Pesi finali AHP (pie chart)



Spider chart call





Vincoli LP applicati e effetto

- **trl_violation:** False
- **budget_company_available:** 0.0
- **budget_max:** 2900000.0
- **sme_required:** True
- **sme_ok:** False
- **ssh_required:** False
- **gender_balance_required:** False

Expected Outcomes

This topic aims at supporting activities that are enabling or contributing

to one or several expected impacts of destination

“Developing and using new tools, technologies and digital solutions for a healthy society”.

To that end, proposals under this

417 The guarantees shall in particular substantiate that, for the purpose of the action, measures are in place

to ensure that: a) control over the applicant legal entity is not exercised in a manner that retrains or restricts its ability to carry out the action and to deliver results, that imposes restrictions concerning its infrastructure, facilities, assets, resources, intellectual property or know-how needed for the purpose of the action, or that undermines its capabilities and standards necessary to carry out the action; b) access by a non-eligible country or by a non-eligible country entity to sensitive information relating to the action is prevented; and the employees or other persons involved in the action have a national security clearance issued by an eligible country, where appropriate; c) ownership of the intellectual property arising from, and the results of, the action remain within the recipient during and after completion of the action, are not subject to control or restrictions by non-eligible countries or non-eligible country entity, and are not exported outside the eligible countries, nor is access to them from outside the eligible countries granted, without the approval of the eligible country in which the legal entity is established.

418 This decision is available on the Funding and Tenders Portal, in the reference documents section for Horizon Europe, under ‘Simplified costs decisions’ or through this link:

https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/ls-decision_he_en.pdf

topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- Researchers and innovators benefit from an improved understanding of how to develop and use the next generation of frontier Artificial Intelligence (AI) models for healthcare, including how to leverage AI Factories and how to combine and expand the capabilities of existing foundation models towards inclusive and personalised medicine.
- Researchers and innovators benefit from an improved understanding of how to leverage highly heterogeneous and multimodal health data

Scope

The AI Continent Action Plan⁴¹⁹ identifies the health sector, encompassing life sciences, medical devices and healthcare delivery, as one of the key strategic sectors. The action will contribute to making European life sciences⁴²⁰ and healthcare more impactful and productive by fostering the full integration of advanced AI in the health sector and biomedical research, along the objectives of the AI in Science strategy⁴²¹ and Apply AI strategy⁴²².

Healthcare typically involves the combining of multimodal data, ranging from electronic health records through imaging and laboratory to molecular and omics data. This information combination is performed by specialists and is often challenging towards optimised patient care. In Europe, the growing amount of accessible multimodal health data, including via the forthcoming European Health Data Space (EHDS)⁴²³, combined with the increasing availability of high-performance computing facilities (e.g. AI Factories), presents a unique opportunity to develop the next generation of frontier AI models for healthcare. This action anticipates and operationalises the use of such federated infrastructures for research and innovation. Moreover, regulations such as the EHDS regulation and AI Act⁴²⁴ steer the direction into building an ecosystem fostering ethical and safe innovation on AI in healthcare.

AI models are becoming increasingly complex and able to tackle increasingly challenging tasks. The next generation of frontier AI models are expected to make strides towards AGI, a type of AI capable of tackling highly complex and diverse tasks with proficiency comparable

419 <https://digital-strategy.ec.europa.eu/en/library/ai-continent-action-plan>

420 https://research-and-innovation.ec.europa.eu/strategy/strategy-research-and-innovation/jobs-and-economy/towards-strategy-european-life-sciences_en; https://ec.europa.eu/commission/presscorner/detail/en/ip_25_1686

421 https://research-and-innovation.ec.europa.eu/research-area/industrial-research-and-innovation/artificial-intelligence-ai-science_en

422 <https://digital-strategy.ec.europa.eu/en/consultations/commission-launches-public-consultation-and->



Rank #2 · HORIZON-HLTH-2026-02-DISEASE-12

GO

European Partnership on Rare Diseases (ERDERA) (Phase 2)

Fit: 62.87 / 100 · Solver: Optimal · CR: 0.0168

Perché questa call ha preso 63/100

La call HORIZON-HLTH-2026-02-DISEASE-12 ottiene un fit del 62.9% dopo ottimizzazione LP. Il criterio dominante e' excellence (contributo 35.4/100). Solver: Optimal, CR AHP: 0.0168.

"This topic aims at supporting activities that are enabling or contributing"

*"to one or several expected impacts of destination
"Tackling diseases and reducing disease"*

*"burden". To that end, proposals under this topic
should aim to deliver results that are directed"*

Gap principali da colmare

- ⚠ Implementation debole: piano esecutivo/consortium readiness non sufficiente
- ⚠ La call richiede copertura SSH esplicita

Azioni concrete (prossimi 3-6 mesi)

- ✅ Costruire shortlist partner (end-user, integrator, academia) e governance WP (90-120 giorni)
- ✅ Integrare partner SSH e work package dedicato a impatti sociali/adozione

Spiegazione Tecnica AHP + LP

Matrice confronto a coppie (AHP)

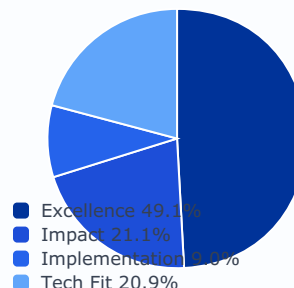
1.000	3.000	5.000	2.000
0.333	1.000	3.000	1.000
0.200	0.333	1.000	0.500
0.500	1.000	2.000	1.000

Pre-LP vs Post-LP

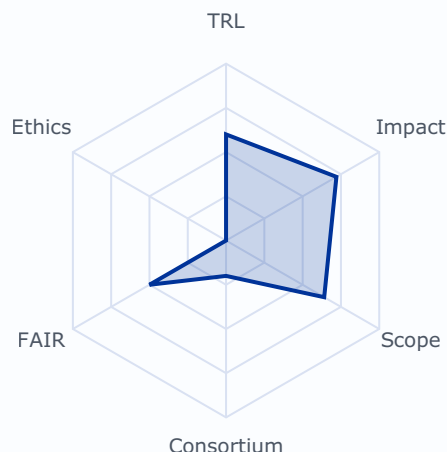
Criterio	Peso AHP	Local pre-LP	Optimal post-LP	Breakdown
excellence	0.4914	0.7204	0.7204	0.3540
impact	0.2105	0.5687	0.5000	0.1052
implementation	0.0896	0.4400	0.4000	0.0358
tech_fit	0.2086	0.6407	0.6407	0.1336

Consistency Ratio: 0.0168 — Matrice consistente

Pesi finali AHP (pie chart)



Spider chart call





Vincoli LP applicati e effetto

- **trl_violation:** False
- **budget_company_available:** 0.0
- **budget_max:** 91300000.0
- **sme_required:** False
- **sme_ok:** False
- **ssh_required:** True
- **gender_balance_required:** False

Expected Outcomes

This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Tackling diseases and reducing disease burden”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- The EU is reinforced as an internationally recognised driver of research and innovation in Rare Diseases (RD) and thereby substantially contributing to the achievement of the Sustainable Development Goals related to rare diseases.
 - Research funders align, adopt and implement their RD research policies allowing for the optimal generation and translation of knowledge into meaningful health products and interventions responding to the needs of people living with a rare disease across Europe and globally.
 - The RD research community at large benefit from and use an improved comprehensive knowledge framework and cross-border FAIR¹⁹¹ data access and analysis, including rare diseases registries, by integrating the EU, national/ regional data and information infrastructures to improve translational research.
 - People living with a rare disease, including those from underrepresented communities, benefit from a more timely, equitable access to innovative, sustainable and high-quality
- ¹⁹¹ See definition of FAIR data in the introduction to this Work Programme part.

healthcare including novel diagnosis and treatments, taking stock of highly integrated research and healthcare systems.

- Researchers, innovators -as well as people living with a rare disease and their advocates (as co-creators)- effectively constitute and operate into an integrated research and innovation ecosystem to deliver cost-effective diagnosis and treatments.
- Public and private actors, including civil society (e.g. Non-Governmental Organisations, charities), establish coordinated and efficient multi-stakeholder collaborations at EU and national (including regional) levels, allowing for more effective clinical research, for example aiming at improved success rates of therapeutic development.

Scope

This topic targets an action under Article 24(2) HE Regulation aiming to add additional activities to existing grant agreements, together with additional partners (if relevant) that would deliver on those activities. The award of a grant to continue the partnership in accordance with this call should be based on a proposal submitted by the coordinator of the consortium funded under topic HORIZON-HLTH-2023-DISEASE-07-01: “European Partnership on Rare Diseases” and the additional activities (which may include additional partners) to be funded by the grant should be subject to an evaluation. Taking into account that the present action is a continuation of the topic HORIZON-HLTH-2023-DISEASE-07-01: “European Partnership on Rare Diseases” and foresees an amendment to an existing grant agreement, the proposal should present the additional activities (including additional partners) to be covered by the award primarily in terms of grant agreement revisions. The existing action, the “European Rare Diseases Research Alliance” (ERDERA) can only reasonably be enhanced and enlarged on the basis of the existing consortium¹⁹², as the co-funded framework established cannot simply be replaced without significant disruption, given the top-quality, long-term expertise and wide coverage of the beneficiaries comprising this consortium.

The proposal should thus present the specific additional activities (including, if relevant, additional partners) foreseen for the second instalment of the partnership. The partnership should continue to contribute to priorities of the communication “On effective, accessible and resilient health systems” (COM(2014) 215 final)¹⁹³, the “Communication from the Commission on enabling the digital transformation of health and care in the Digital Single Market; empowering citizens and building a healthier society” (COM(2018) 233 final)¹⁹⁴, the “Council conclusions on the Future of the European Health Union: A Europe that cares, prepares and protects” (9900/24)¹⁹⁵ and support the objectives of the EU4Health Programme (2021-2027)¹⁹⁶.

¹⁹² Consortium which was awarded the grant under topic HORIZON-HLTH-2023-DISEASE-07-01: “European Partnership on Rare Diseases”.

¹⁹³ https://health.ec.europa.eu/publications/communication-commission-com2014-215-final_en

Rank #3 · HORIZON-HLTH-2026-01-TOOL-07

GO

Establishing a European network of Centres of Excellence (CoEs) for Advanced Therapies Medicinal Products (ATMPs)

Fit: 62.50 / 100 · Solver: Optimal · CR: 0.0168

Perché questa call ha preso 62/100

La call HORIZON-HLTH-2026-01-TOOL-07 ottiene un fit del 62.5% dopo ottimizzazione LP. Il criterio dominante e' excellence (contributo 35.3/100). Solver: Optimal, CR AHP: 0.0168.

"This topic aims at supporting activities that are enabling or contributing"

*"to one or several expected impacts of destination
"Developing and using new tools,""*

"375 https://strategic-technologies.europa.eu/about/step-seal_en"

Gap principali da colmare

- ⚠ Impact non ancora distintivo su outcomes e policy relevance
- ⚠ Implementation debole: piano esecutivo/consortium readiness non sufficiente

Azioni concrete (prossimi 3-6 mesi)

- ✅ Allineare proposition agli expected outcomes della call con KPI misurabili (60 giorni)
- ✅ Costruire shortlist partner (end-user, integrator, academia) e governance WP (90-120 giorni)

Spiegazione Tecnica AHP + LP

Matrice confronto a coppie (AHP)

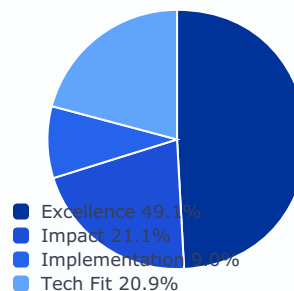
1.000	3.000	5.000	2.000
0.333	1.000	3.000	1.000
0.200	0.333	1.000	0.500
0.500	1.000	2.000	1.000

Pre-LP vs Post-LP

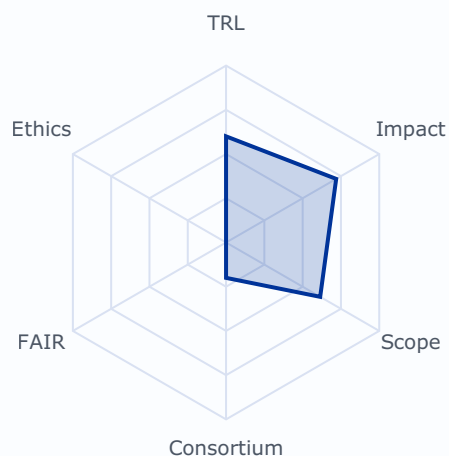
Criterio	Peso AHP	Local pre-LP	Optimal post-LP	Breakdown
excellence	0.4914	0.7189	0.7189	0.3532
impact	0.2105	0.5118	0.5118	0.1077
implementation	0.0896	0.4400	0.4000	0.0358
tech_fit	0.2086	0.6145	0.6145	0.1282

Consistency Ratio: 0.0168 — Matrice consistente

Pesi finali AHP (pie chart)



Spider chart call



Vincoli LP applicati e effetto

- **trl_violation:** False
- **budget_company_available:** 0.0
- **budget_max:** 3900000.0
- **sme_required:** False
- **sme_ok:** False
- **ssh_required:** False
- **gender_balance_required:** False

Expected Outcomes

This topic aims at supporting activities that are enabling or contributing

to one or several expected impacts of destination

“Developing and using new tools,

375 https://strategic-technologies.europa.eu/about/step-seal_en

376 This decision is available on the Funding and Tenders Portal, in the reference documents section for Horizon Europe, under ‘Simplified costs decisions’ or through this link:

https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/lis-decision_he_en.pdf

technologies and digital solutions for a healthy society”.

To that end, proposals under this

topic should aim to deliver results that are directed at, tailored towards and contributing to all

the following expected outcomes:

- In line with the objectives of the “Strategy for European Life Sciences”³⁷⁷, the EU successfully translates a strategic priority into an implementable action plan for building technological and innovation leadership on a global stage.
- Europe, including regions, profits from an increased capacity, accessibility, coordination of its Advanced Therapies Medicinal Products (ATMP) infrastructures.
- The European economy benefits from more ATMP innovations being developed and commercialised in the EU.
- Patients across the EU gain faster access to innovative ATMPs thanks to increased and focused public Research and Development (R&D) investment, harmonised policies and strategies for ATMP development and uptake in healthcare systems.

Scope

ATMPs represent a frontier in medicine, offering groundbreaking treatments such as

gene therapies, cell therapies, and tissue-engineered products that hold the promise of

addressing complex and previously untreatable

conditions. The European ATMP landscape is dynamic and promising, with 28 products having

received marketing authorisation and many

more in the pipeline³⁷⁸. The development of

specialised infrastructures in Europe for these

cutting-edge therapies is crucial for fostering innovation and ensuring fast and efficient

delivery to patients in an equitable way. The report by

Mario Draghi ³⁷⁹ on EU

competitiveness, highlights that the EU’s share of the fast-growing global ATMP market is

small, suggesting that to remain competitive, increased and focused public R&D investment is

needed to complement ongoing efforts to streamline regulations and ensure faster pricing and

reimbursement. The report recommends building on existing innovation hubs and expanding

the capacity of the EU to conduct ATMPs R&D by the consolidation of EU public funds. By

strengthening this budding innovation ecosystem,

Europe can position itself as a leader in the

ATMP sector, ultimately improving patient access to life-saving treatments and stimulating

economic growth.

The EU has several scattered ATMP centres with divergent capacities and capabilities³⁸⁰,

limiting its attractiveness for scaling up R&D in the field, calling for a coherent and

coordinated approach. The aim of this topic is to

establish a European network of Centres of

Excellence (CoEs) for ATMPs, building on existing centres and coordinating their further

³⁷⁷ https://research-and-innovation.ec.europa.eu/strategy/strategy-research-and-innovation/jobs-and-economy/towards-strategy-european-life-sciences_en; https://ec.europa.eu/commission/presscorner/detail/en/ip_25_1686

³⁷⁸ Committee for Advanced Therapies (CAT) quarterly highlights and approved ATMPs Feb-May 2025:

https://www.ema.europa.eu/en/documents/committee-report/cat-quarterly-highlights-approved-atmps-may-2025_en.pdf

³⁷⁹ The future of European competitiveness, Mario Draghi: https://commission.europa.eu/topics/eu-competitiveness/draghi-report_en

³⁸⁰ Advanced Therapy Medicinal Products - EATRIS: <https://eatris.eu/atmp>

Rank #4 · HORIZON-HLTH-2026-03-DISEASE-13

GO

European Partnership for Pandemic Preparedness (Phase 2)

Fit: 61.96 / 100 · Solver: Optimal · CR: 0.0168

Perché questa call ha preso 62/100

La call HORIZON-HLTH-2026-03-DISEASE-13 ottiene un fit del 62.0% dopo ottimizzazione LP. Il criterio dominante e' excellence (contributo 35.8/100). Solver: Optimal, CR AHP: 0.0168.

"This topic aims at supporting activities that are enabling or contributing"

*"to one or several expected impacts of destination
"Tackling diseases and reducing disease"*

"burden". Proposals under this topic should aim to deliver results that are directed at, tailored"

Gap principali da colmare

- ⚠ Impact non ancora distintivo su outcomes e policy relevance
- ⚠ Implementation debole: piano esecutivo/consortium readiness non sufficiente
- ⚠ Requisito SME non soddisfatto dal profilo corrente
- ⚠ La call richiede copertura SSH esplicita

Azioni concrete (prossimi 3-6 mesi)

- ✅ Allineare proposition agli expected outcomes della call con KPI misurabili (60 giorni)
- ✅ Costruire shortlist partner (end-user, integrator, academia) e governance WP (90-120 giorni)
- ✅ Valutare ruolo da partner o struttura legale/cap table compatibile con requisiti SME
- ✅ Integrare partner SSH e work package dedicato a impatti sociali/adozione

Spiegazione Tecnica AHP + LP

Matrice confronto a coppie (AHP)

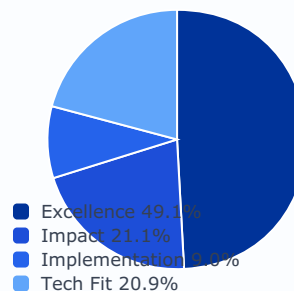
1.000	3.000	5.000	2.000
0.333	1.000	3.000	1.000
0.200	0.333	1.000	0.500
0.500	1.000	2.000	1.000

Pre-LP vs Post-LP

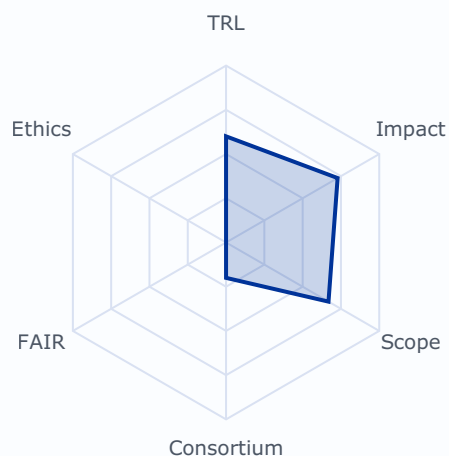
Criterio	Peso AHP	Local pre-LP	Optimal post-LP	Breakdown
excellence	0.4914	0.7284	0.7284	0.3579
impact	0.2105	0.4956	0.4956	0.1043
implementation	0.0896	0.4400	0.2000	0.0179
tech_fit	0.2086	0.6687	0.6687	0.1395

Consistency Ratio: 0.0168 — Matrice consistente

Pesi finali AHP (pie chart)



Spider chart call



Vincoli LP applicati e effetto

- **trl_violation:** False
- **budget_company_available:** 0.0
- **budget_max:** 63000000.0
- **sme_required:** True
- **sme_ok:** False
- **ssh_required:** True
- **gender_balance_required:** False

Expected Outcomes

This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Tackling diseases and reducing disease burden”. Proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- The EU offers a valued operational network of clinical research sites (both interventional and observational) that have the capacity to implement well-coordinated large-scale multi-country quality clinical studies in different target populations, which are able to smoothly transition to interventions relevant for cross-border health threats in readiness for or response to a public health emergency.
- Key stakeholders, including relevant EU and national entities, the scientific communities, policymakers and funders enhance their collaboration and coordination to strengthen research on pandemic preparedness and response, forming a strong, structured and comprehensive ecosystem with shared evidence, tools and methodologies cutting across sectors.
- Research funders, policymakers, relevant EU and national entities, and the research community recognise and rapidly close relevant research and related infrastructure gaps and break existing silos on pandemic preparedness research and response, adopting a One Health approach.
- Healthcare authorities, regulatory authorities, policymakers and other stakeholders utilise research results to develop evidence-based strategies and policies for pandemic preparedness and response, and deploy good practices to European countries and regions, and beyond whenever relevant.
- The research community benefits from and uses an improved comprehensive knowledge framework integrating the EU, national/regional data and information infrastructures to improve transnational research in the area of pandemic preparedness and response.
- The EU is strengthened as an internationally recognised actor for pandemic preparedness research and response, as such substantially contributing to global cooperation and coordination.

Scope

This topic targets an action under Article 24(2) HE Regulation aiming to add additional activities to existing grant agreements, together with additional partners (if relevant) that would deliver on those activities. The award of a grant to continue the partnership in accordance with this call should be based on a proposal submitted by the coordinator of the consortium funded under topic HORIZON-HLTH-2024-DISEASE-12-01:

“European Partnership for Pandemic Preparedness” and the additional activities (which may include additional partners) to be funded by the grant, such as the close coordination with the Clinical Trial Coordination Mechanism (CT-CM)²⁰³, should be subject to an evaluation. The partnership should be firmly anchored within the framework of the European Health Union package²⁰⁴ and ensure synergies with the European Health Emergency Preparedness and Response Authority (HERA) and other relevant Commission services. The partnership’s



Rank #5 · HORIZON-HLTH-2026-01-STAYHLTH-03

GO

Building public trust and outreach in the life sciences

Fit: 61.13 / 100 · Solver: Optimal · CR: 0.0168

Perché questa call ha preso 61/100

La call HORIZON-HLTH-2026-01-STAYHLTH-03 ottiene un fit del 61.1% dopo ottimizzazione LP. Il criterio dominante e' excellence (contributo 32.9/100). Solver: Optimal, CR AHP: 0.0168.

"This topic aims at supporting activities that are enabling or contributing"

*"to one or several expected impacts of destination
"Staying healthy in a rapidly changing"*

*"society". To that end, proposals under this topic
should aim to deliver results that are directed"*

Gap principali da colmare

- ⚠ Implementation debole: piano esecutivo/consortium readiness non sufficiente

Azioni concrete (prossimi 3-6 mesi)

- ✅ Costruire shortlist partner (end-user, integrator, academia) e governance WP (90-120 giorni)

Spiegazione Tecnica AHP + LP

Matrice confronto a coppie (AHP)

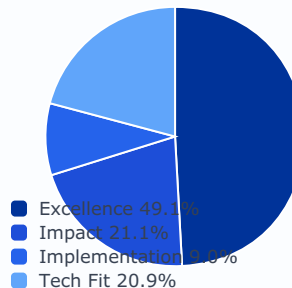
1.000	3.000	5.000	2.000
0.333	1.000	3.000	1.000
0.200	0.333	1.000	0.500
0.500	1.000	2.000	1.000

Pre-LP vs Post-LP

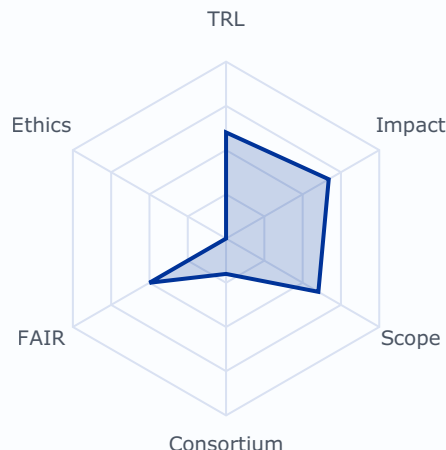
Criterio	Peso AHP	Local pre-LP	Optimal post-LP	Breakdown
excellence	0.4914	0.6703	0.6703	0.3294
impact	0.2105	0.5719	0.5719	0.1204
implementation	0.0896	0.4400	0.4000	0.0358
tech_fit	0.2086	0.6027	0.6027	0.1257

Consistency Ratio: 0.0168 — Matrice consistente

Pesi finali AHP (pie chart)



Spider chart call





Vincoli LP applicati e effetto

- **trl_violation:** False
- **budget_company_available:** 0.0
- **budget_max:** 1900000.0
- **sme_required:** False
- **sme_ok:** False
- **ssh_required:** False
- **gender_balance_required:** False

Expected Outcomes

This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Staying healthy in a rapidly changing society”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- Strengthened capacity of life sciences actors in science communication, risk communication, public outreach, and citizen engagement.
- Strengthened awareness of risks and benefits of life sciences by the public, by showcasing the latest Research and Innovation (R&I) developments in the life sciences, and their societal impact.

64 This decision is available on the Funding and Tenders Portal, in the reference documents section for Horizon Europe, under ‘Simplified costs decisions’ or through this link:

https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/lis-decision_he_en.pdf

- New and innovative approaches developed to engage the public in R&I activities in the life sciences, with an emphasis on inclusive and participatory approaches, involving relevant stakeholders (e.g. researchers, research funders, policymakers, publishers, civil society organisations, business, and citizens).

Scope

Life science innovations significantly contribute to peoples’ daily life and to individual and social well-being. To foster public trust, people must understand how life sciences work and how these technologies may impact people’s lives.

This trust is not guaranteed. It is increasingly threatened by the rapid spread of mis- and disinformation and by insufficient outreach to and involvement of people to address their concerns and expectations. To maintain and deepen trust, especially among young people, R&I policymakers, researchers and industry players must be better equipped to engage with the public and to pursue responsible research and innovation.

Citizen engagement is particularly critical in areas like agriculture and food technology, where innovation intersects with health and sustainability considerations and values.

Furthermore, citizen participation is key to build trust in the life sciences and ensure that they meet societal needs. By engaging citizens early on, we can ensure that research and innovation align with their values, concerns and expectations. This involves designing research and innovation processes that incorporate citizen input, such as setting research priorities, and create outcomes that are responsive to their needs.

To this end, proposals should address all the following activities:

- Provide advisory support and training to life science stakeholders, in order to upskill them in science communication and risk communication.
- Produce, publish and advertise to the relevant actors, guidance to engage citizens upstream in the development, co-production, and co-design of life sciences innovation.
- Produce, publish, and advertise to the relevant public an accessible repository of tools for life science stakeholders on risk communication.
- Design and run community engagement activities in the life sciences, in partnership with relevant local actors, such as science museums, R&I organisations, and/or community organisations.

Regarding advisory support and training, proposals should present how they will engage bilaterally with life science stakeholders to advise them and train them on science communication and risk communication in the life

Rank #6 · HORIZON-HLTH-2026-01-TOOL-06

GO

Support to European Research Area (ERA) action on accelerating New Approach Methodologies (NAMs) to advance biomedical research and testing of medicinal products and medical devices

Fit: 60.36 / 100 · Solver: Optimal · CR: 0.0168

Perché questa call ha preso 60/100

La call HORIZON-HLTH-2026-01-TOOL-06 ottiene un fit del 60.4% dopo ottimizzazione LP. Il criterio dominante e' excellence (contributo 35.3/100). Solver: Optimal, CR AHP: 0.0168.

"This topic aims at supporting activities that are enabling or contributing"

*"to one or several expected impacts of destination
"Developing and using new tools,"*

"technologies and digital solutions for a healthy society". To that end, proposals under this"

Gap principali da colmare

- ⚠ Impact non ancora distintivo su outcomes e policy relevance
- ⚠ Implementation debole: piano esecutivo/consortium readiness non sufficiente
- ⚠ Requisito SME non soddisfatto dal profilo corrente

Azioni concrete (prossimi 3-6 mesi)

- ✅ Allineare proposition agli expected outcomes della call con KPI misurabili (60 giorni)
- ✅ Costruire shortlist partner (end-user, integrator, academia) e governance WP (90-120 giorni)
- ✅ Valutare ruolo da partner o struttura legale/cap table compatibile con requisiti SME

Spiegazione Tecnica AHP + LP

Matrice confronto a coppie (AHP)

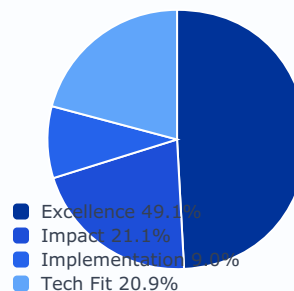
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0.333	1.000	3.000	1.000
0.200	0.333	1.000	0.500
0.500	1.000	2.000	1.000

Pre-LP vs Post-LP

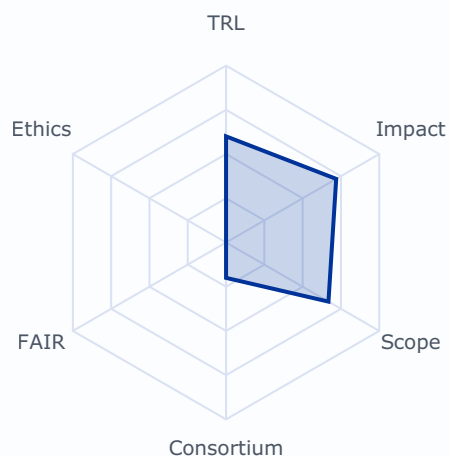
Criterio	Peso AHP	Local pre-LP	Optimal post-LP	Breakdown
excellence	0.4914	0.7191	0.7191	0.3533
impact	0.2105	0.4415	0.4415	0.0929
implementation	0.0896	0.4400	0.2000	0.0179
tech_fit	0.2086	0.6685	0.6685	0.1394

Consistency Ratio: 0.0168 — Matrice consistente

Pesi finali AHP (pie chart)



Spider chart call



Vincoli LP applicati e effetto

- **trl_violation:** False
- **budget_company_available:** 0.0
- **budget_max:** 2900000.0
- **sme_required:** True
- **sme_ok:** False
- **ssh_required:** False
- **gender_balance_required:** False

Expected Outcomes

This topic aims at supporting activities that are enabling or contributing

to one or several expected impacts of destination

“Developing and using new tools, technologies and digital solutions for a healthy society”.

To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- Member States and relevant stakeholders identify priority areas where New Approach Methodologies (NAMs) and infrastructures are most needed and expected to have the highest short- to medium-term impact.

371 This decision is available on the Funding and Tenders Portal, in the reference documents section for Horizon Europe, under ‘Simplified costs decisions’ or through this link:

https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/lis-decision_he_en.pdf

- Member States and other stakeholders jointly support the validation and qualification of a limited set of NAMs that are intended to be accepted and implemented in regulatory testing of medicinal products and medical devices.

- Member States and other stakeholders develop common education and training programmes based on best practices identified in European and non-European countries to better inform researchers and regulators on NAMs and on the application of the 3Rs principles³⁷².

- Member States and other stakeholders implement a harmonised NAM openness and awareness programme that improves open access to NAMs protocols and results of animal experiments. It also provides guidance to harmonise the awareness of NAMs for ethical committee members, reviewers, and regulators, based on best practices in the participating Member States. The programme should propose concrete actions to increase the confidence of regulators in NAMs including a better understanding of the potential and limitations of NAMs.

Scope

This topic aims to coordinate and develop the new European Research Area (ERA)

policy action to accelerate, through an aligned and coordinated approach across Member States and Associated Countries, the development, validation/qualification, acceptance, and uptake of NAMs in biomedical research and regulatory testing of medicinal products and medical devices as part of the ERA Policy Agenda 2025-2027³⁷³.

The ERA action should establish an EU-wide forum that brings together relevant ministries, regulatory agencies, research funding organisations, academia, industry (pharmaceutical and medical technology), Contract Research Organisations (CROs), small and medium-sized enterprises (SMEs) ³⁷⁴, and startups to harmonise policies and strategies for NAMs development and implementation.

The selected proposal should be coordinated by any active participant to the ERA action to ensure consistency with ERA action policy objectives. It should contribute to the implementation of the following themes of the four thematic Working Groups (WGs) of the ERA action:

WG1: Development of NAMs and common European infrastructures. This WG identifies opportunities for the development and integration of NAMs and the establishment of supporting infrastructures. Its focus spans specific disease or biological areas and safety, quality, and efficacy assessment endpoints for medicinal products and medical devices. The WG provides insight and suggests priorities to governments and industry for the further coordinated efforts to leverage promising development of NAMs, taking into consideration

³⁷² Replacement, Reduction, Refinement: <https://nc3rs.org.uk/who-we-are/3rs>

³⁷³ Proposal for a Council Recommendation on the European Research Area Policy Agenda 2025-2027: <https://european-research-area.ec.europa.eu/documents/proposal-council-recommendation-european-research-area-policy-agenda-2025-2027>

³⁷⁴ <https://eur-lex.europa.eu/legal-content/EN/TXT/PDF/?uri=CELEX:32003H0361>

the complementarity of scientific strengths, funding priorities and available expertise in the different Member States and regions.

WG2: Validation, acceptance, and uptake of NAMs.



Rank #7 · HORIZON-HLTH-2026-04-CARE-04

GO

Enhancing and enlarging the European Partnership on Personalised Medicine (EP PerMed) (Top-up)

Fit: 60.18 / 100 · Solver: Optimal · CR: 0.0168

Perché questa call ha preso 60/100

La call HORIZON-HLTH-2026-04-CARE-04 ottiene un fit del 60.2% dopo ottimizzazione LP. Il criterio dominante e' excellence (contributo 31.3/100). Solver: Optimal, CR AHP: 0.0168.

"This topic aims at supporting activities that are enabling or contributing"

*"to one or several expected impacts of destination
"Ensuring equal access to innovative,"*

"sustainable, and high-quality healthcare". To that end, proposals under this topic should aim"

Gap principali da colmare

- ⚠ Impact non ancora distintivo su outcomes e policy relevance
- ⚠ Implementation debole: piano esecutivo/consortium readiness non sufficiente

Azioni concrete (prossimi 3-6 mesi)

- ✅ Allineare proposition agli expected outcomes della call con KPI misurabili (60 giorni)
- ✅ Costruire shortlist partner (end-user, integrator, academia) e governance WP (90-120 giorni)

Spiegazione Tecnica AHP + LP

Matrice confronto a coppie (AHP)

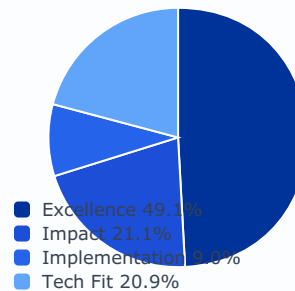
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0.333	1.000	3.000	1.000
0.200	0.333	1.000	0.500
0.500	1.000	2.000	1.000

Pre-LP vs Post-LP

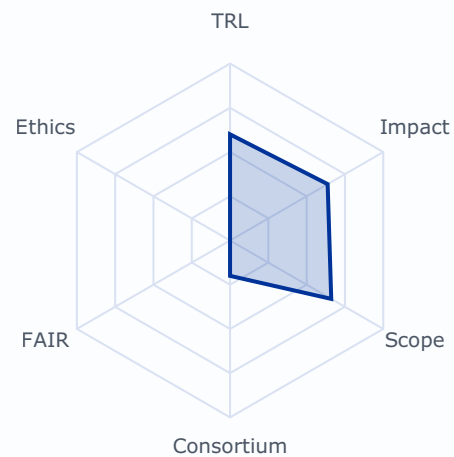
Criterio	Peso AHP	Local pre-LP	Optimal post-LP	Breakdown
excellence	0.4914	0.6372	0.6372	0.3131
impact	0.2105	0.5469	0.5469	0.1151
implementation	0.0896	0.4400	0.4000	0.0358
tech_fit	0.2086	0.6605	0.6605	0.1378

Consistency Ratio: 0.0168 — Matrice consistente

Pesi finali AHP (pie chart)



Spider chart call



Vincoli LP applicati e effetto

- **trl_violation:** False
- **budget_company_available:** 0.0
- **budget_max:** 9800000.0
- **sme_required:** False
- **sme_ok:** False
- **ssh_required:** False
- **gender_balance_required:** False

Expected Outcomes

This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Ensuring equal access to innovative, sustainable, and high-quality healthcare”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following

Scope

This topic targets an action under Article 24(2) HE Regulation aiming to add additional activities to existing grant agreements, together with additional partners that would deliver on those activities. The award of a grant to continue the partnership in accordance with this call should be based on a proposal submitted by the coordinator of the consortium funded under topic HORIZON-HLTH-2023-CARE-08-01: “European Partnership on Personalised Medicine” and the additional activities and additional partners to be funded by the grant should be subject to an evaluation. Taking into account that the present action is a continuation of the topic HORIZON-HLTH-2023-CARE-08-01: “European Partnership on Personalised Medicine” and foresees an amendment to an existing grant agreement, the proposal should present the additional activities (including additional partners) to be covered by the award primarily in terms of grant agreement revisions. Partners from countries recently associated to Horizon Europe from 2024 onwards (2024 included) are particularly welcome. The existing action, the “European Partnership on Personalised Medicine” (EP PerMed) can only reasonably be enhanced and enlarged on the basis of the existing consortium³³⁹, as the co-funded framework established cannot simply be replaced without significant disruption, ³³⁹ Consortium which was awarded the grant under topic HORIZON-HLTH-2023-CARE-08-01: “European Partnership on Personalised Medicine”.

given the top-quality, long-term expertise and wide coverage of the beneficiaries comprising this consortium.

The additional activities to be performed by applicants under this topic should consist of several of the following:

- Organisation of activities or tools according to their expertise and interests, e.g.:
 - ❑ Personalised Medicine (PM) Innovation related activities and tools, business and entrepreneur relations and support, case studies and guides.
 - ❑ PM public health and social care, people’s engagement, activities to support health system’s ability to turn scientific discoveries into new or improved treatments and services, support the scientific community to tackle complex health and social care challenges, international outreach.

Rank #8 · HORIZON-HLTH-2026-01-DISEASE-15

GO

Scaling up innovation in cardiovascular health

Fit: 60.00 / 100 · Solver: Optimal · CR: 0.0168

Perché questa call ha preso 60/100

La call HORIZON-HLTH-2026-01-DISEASE-15 ottiene un fit del 60.0% dopo ottimizzazione LP. Il criterio dominante e' excellence (contributo 32.8/100). Solver: Optimal, CR AHP: 0.0168.

"This topic aims at supporting activities that are enabling or contributing"

*"to one or several expected impacts of destination
"Tackling diseases and reducing disease"*

"burden". To that end, proposals under this topic should aim to deliver results that are directed"

Gap principali da colmare

- ⚠ Implementation debole: piano esecutivo/consortium readiness non sufficiente
- ⚠ La call richiede copertura SSH esplicita

Azioni concrete (prossimi 3-6 mesi)

- ✅ Costruire shortlist partner (end-user, integrator, academia) e governance WP (90-120 giorni)
- ✅ Integrare partner SSH e work package dedicato a impatti sociali/adozione

Spiegazione Tecnica AHP + LP

Matrice confronto a coppie (AHP)

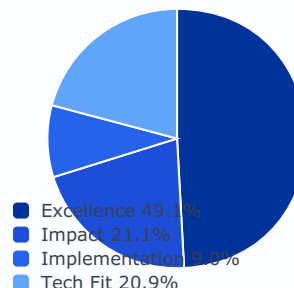
1.000	3.000	5.000	2.000
0.333	1.000	3.000	1.000
0.200	0.333	1.000	0.500
0.500	1.000	2.000	1.000

Pre-LP vs Post-LP

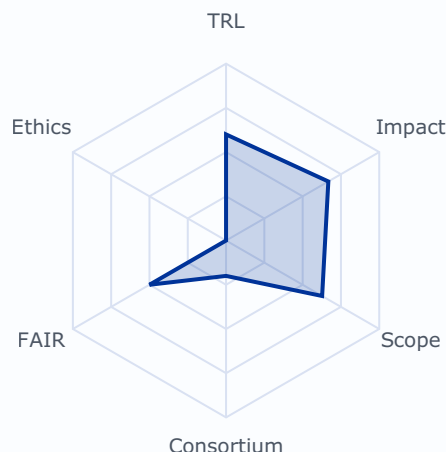
Criterio	Peso AHP	Local pre-LP	Optimal post-LP	Breakdown
excellence	0.4914	0.6678	0.6678	0.3281
impact	0.2105	0.6196	0.5000	0.1052
implementation	0.0896	0.4400	0.4000	0.0358
tech_fit	0.2086	0.6272	0.6272	0.1308

Consistency Ratio: 0.0168 — Matrice consistente

Pesi finali AHP (pie chart)



Spider chart call





Vincoli LP applicati e effetto

- **trl_violation:** False
- **budget_company_available:** 0.0
- **budget_max:** 1900000.0
- **sme_required:** False
- **sme_ok:** False
- **ssh_required:** True
- **gender_balance_required:** False

Expected Outcomes

This topic aims at supporting activities that are enabling or contributing

to one or several expected impacts of destination

“Tackling diseases and reducing disease burden”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- Healthcare providers, policymakers and researchers benefit from an improved knowledge base and collaboration on the key challenges and gaps on cardiovascular health research and a conceptual framework to develop a roadmap for research and innovation is established.
- Health systems gain improved and standardised evidence to better prevent, diagnose or treat cardiovascular diseases (CVDs) and associated comorbidities, based on the research results on prediction, early detection, screening practices and diagnostic methods and tools, including via personalised and digital approaches.
- Medical and non-medical health professionals and technology developers have an increased knowledge, awareness and capacity to uptake and deliver effective and innovative approaches for risk prediction, early detection, screening and health management strategies, such as Virtual Human Twins (VHT)²¹⁸ or Artificial Intelligence (AI)-based applications. This involves supporting, strategic foresight, improving health literacy and cross-sectoral knowledge exchange and collaboration to drive innovation in personalised prevention and cardiovascular risk prediction.
- Healthcare providers and policymakers have an improved knowledge base to inform future strategies for early detection and prevention of CVDs, with specific attention to women and vulnerable groups, through research on personalised risk prediction approaches that consider multiple and interacting risk factors (e.g. genetic predisposition, environmental pollutants, diet, lifestyle habits, multimorbidity, sex and gender).

Scope

CVDs are the main cause of death in the EU, with over 1.7 million deaths annually,

costing about EUR 282 billion, or 11% of the healthcare budget²¹⁹. With projections showing

217 This decision is available on the Funding and Tenders Portal, in the reference documents section for Horizon Europe, under ‘Simplified costs decisions’ or through this link:

https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/lis-decision_he_en.pdf

218 See the European Virtual Human Twins Initiative: [https://digital-](https://digital-strategy.ec.europa.eu/en/policies/virtual-human-twins)

[strategy.ec.europa.eu/en/policies/virtual-human-twins](https://digital-strategy.ec.europa.eu/en/policies/virtual-human-twins)

219 European Union takes action for the cardiovascular health of its 440 million people - EACH: <https://www.cardiovascular-alliance.eu/european-union-takes-action-for-the-cardiovascular-health-of-its-440-million-people>

a rise in CVD prevalence and mortality by 2050 due to an aging population, the Commission is preparing a comprehensive EU Cardiovascular Health (CVH) plan²²⁰ to support Member States in their efforts to reduce the burden of CVDs. The proposal is expected to support prevention, early detection, including via digital and personalised approaches.

Applicants should take stock of research and innovation results to identify gaps and set up a plan for a Strategic Research and Innovation Agenda (SRIA) on CVDs, with the final aim of leveraging research and innovation results to improve risk prediction, early detection and screening practices for CVDs and associated comorbidities, especially obesity and diabetes, across the EU and Associated Countries. This initiative addresses the pressing need to translate existing innovations and promising research results into implementable protocols that enhance prevention, diagnosis, and health outcomes for diverse populations. By fostering collaboration and integrating digital tools and methods, proposals will support the future EU CVH plan, building and aligning with future and ongoing activities, including the European VHT Initiative, the 1+Million Genomes Initiative²²¹, Tech Foresight/Horizon scanning

Rank #9 · HORIZON-HLTH-2026-01-ENVHLTH-05

GO

Support for a multilateral initiative on climate change and health research

Fit: 58.29 / 100 · Solver: Optimal · CR: 0.0168

Perché questa call ha preso 58/100

La call HORIZON-HLTH-2026-01-ENVHLTH-05 ottiene un fit del 58.3% dopo ottimizzazione LP. Il criterio dominante e' excellence (contributo 31.6/100). Solver: Optimal, CR AHP: 0.0168.

"This topic aims at supporting activities that are enabling or contributing"

*"to one or several expected impacts of destination
"Living and working in a health-promoting"*

"environment". To that end, proposals under this topic should aim to deliver results that are"

Gap principali da colmare

- ⚠ Impact non ancora distintivo su outcomes e policy relevance
- ⚠ Implementation debole: piano esecutivo/consortium readiness non sufficiente

Azioni concrete (prossimi 3-6 mesi)

- ✅ Allineare proposition agli expected outcomes della call con KPI misurabili (60 giorni)
- ✅ Costruire shortlist partner (end-user, integrator, academia) e governance WP (90-120 giorni)

Spiegazione Tecnica AHP + LP

Matrice confronto a coppie (AHP)

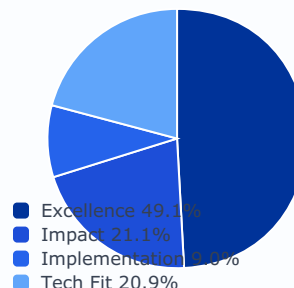
1.000	3.000	5.000	2.000
0.333	1.000	3.000	1.000
0.200	0.333	1.000	0.500
0.500	1.000	2.000	1.000

Pre-LP vs Post-LP

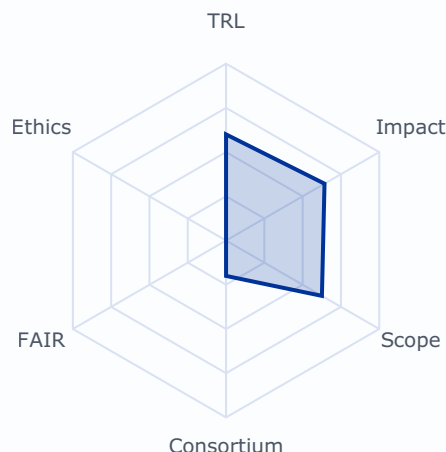
Criterio	Peso AHP	Local pre-LP	Optimal post-LP	Breakdown
excellence	0.4914	0.6428	0.6428	0.3158
impact	0.2105	0.4785	0.4785	0.1007
implementation	0.0896	0.4400	0.4000	0.0358
tech_fit	0.2086	0.6258	0.6258	0.1305

Consistency Ratio: 0.0168 — Matrice consistente

Pesi finali AHP (pie chart)



Spider chart call





Vincoli LP applicati e effetto

- **trl_violation:** False
- **budget_company_available:** 0.0
- **budget_max:** 3000000.0
- **sme_required:** False
- **sme_ok:** False
- **ssh_required:** False
- **gender_balance_required:** False

Expected Outcomes

This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Living and working in a health-promoting environment”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to most of the following expected outcomes:

- An international multilateral initiative is established to facilitate coordination and synergy between different research and innovation funding organisations tackling climate change and health issues with adequate support of a secretariat that works towards the definition of a governance model and forward-looking plan for the implementation of this initiative.
- The international community of research funders working in the climate-health nexus is well connected and supported to improve coordination of activities and alignment of priorities and increase the collective impact of funding streams.
- The research community, civil society groups and policymakers working in the climate-health nexus are well informed about the initiative’s activities and benefit from the knowledge, tools and opportunities created during its implementation.

Scope

Climate-related health challenges are global and complex in nature, which calls for coordinated action bringing together different research disciplines, policy sectors, perspectives and approaches. This requires seamless communication and synergies between different Research and Innovation (R&I) funding instruments.

R&I is key to increasing our understanding of existing and emerging climate-related vulnerabilities experienced by populations and health systems alike, as well as to supporting the development and implementation of effective and sustainable interventions for prevention, adaptation and preparedness against climate-related health threats. They also play a crucial role in supporting the health sector’s transition towards decarbonisation and long-term sustainability while ensuring that quality of care is maintained or improved.

R&I funding schemes and programmes will structure and accelerate the health response needed to meet the severity of the climate crisis in the decades to come. Achieving this requires building on a truly collaborative and impactful network of key players that optimally and efficiently structure and align their research programmes and funding streams.

The Commission, in collaboration with other funders and international organisations, will work on the development of a collaborative interface that brings together research funders 100 This decision is available on the Funding and Tenders Portal, in the reference documents section for Horizon Europe, under ‘Simplified costs decisions’ or through this link:

https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/ls-decision_he_en.pdf

from around the world working at the intersection of health and climate. This platform will establish opportunities for participating organisations to discuss challenges and priorities and share best practices, research strategies and implementation plans. The platform will also serve to identify areas of common interest for collaboration and to build a shared vision to support health and climate research globally. Proposals under this topic should focus on the provision of administrative and technical support to the successful establishment and implementation of a multilateral initiative of

Rank #10 · HORIZON-HLTH-2026-01-IND-03

WATCH

Regulatory science to support translational development of patient-centred health technologies

Fit: 49.50 / 100 · Solver: Optimal · CR: 0.0168

Perché questa call ha preso 50/100

La call HORIZON-HLTH-2026-01-IND-03 ottiene un fit del 49.5% dopo ottimizzazione LP. Il criterio dominante e' tech_fit (contributo 15.7/100). Solver: Optimal, CR AHP: 0.0168.

"This topic aims at supporting activities that are enabling or contributing"

*"to one or several expected impacts of destination
"Maintaining an innovative, sustainable, and"*

"competitive EU health industry". To that end, proposals under this topic should aim to deliver"

Gap principali da colmare

- ⚠ Implementation debole: piano esecutivo/consortium readiness non sufficiente
- ⚠ TRL aziendale inferiore al requisito della call (target: 4)

Azioni concrete (prossimi 3-6 mesi)

- ✅ Costruire shortlist partner (end-user, integrator, academia) e governance WP (90-120 giorni)
- ✅ Eseguire pilot dimostrativo per salire di almeno 1 livello TRL (3-6 mesi)

Spiegazione Tecnica AHP + LP

Matrice confronto a coppie (AHP)

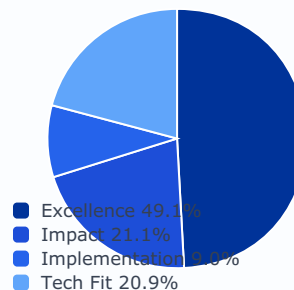
1.000	3.000	5.000	2.000
0.333	1.000	3.000	1.000
0.200	0.333	1.000	0.500
0.500	1.000	2.000	1.000

Pre-LP vs Post-LP

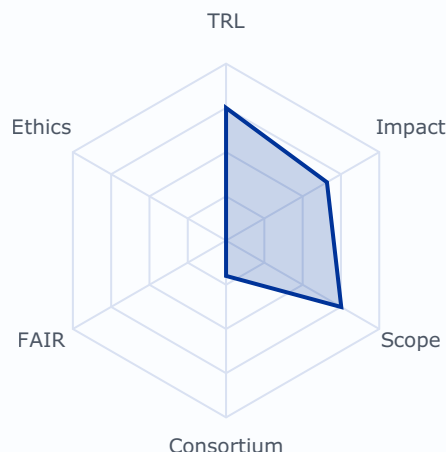
Criterio	Peso AHP	Local pre-LP	Optimal post-LP	Breakdown
excellence	0.4914	0.6587	0.3000	0.1474
impact	0.2105	0.7358	0.7358	0.1549
implementation	0.0896	0.5300	0.4000	0.0358
tech_fit	0.2086	0.7521	0.7521	0.1569

Consistency Ratio: 0.0168 — Matrice consistente

Pesi finali AHP (pie chart)



Spider chart call





Vincoli LP applicati e effetto

- **trl_violation:** True
- **budget_company_available:** 0.0
- **budget_max:** 19600000.0
- **sme_required:** False
- **sme_ok:** False
- **ssh_required:** False
- **gender_balance_required:** False

Expected Outcomes

This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Maintaining an innovative, sustainable, and competitive EU health industry”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to several of the following

Scope

The development, uptake and impact of health technologies typically results from a long product development process that is based on a 'life cycle approach' which typically involves several iterations of defined stages, i.e. from development, assessment to post-market surveillance and post-market clinical follow-up. While health technologies are governed by comprehensive legal frameworks aiming to ensure that health technologies are safe and effective, the regulatory science underlying these legal frameworks needs to be updated. This concerns inter alia i) more precise delineation of specific requirements (e.g. closing existing gaps concerning sufficiency of clinical evidence) and ii) the consideration of novel biomedical approaches, data and digital solutions (e.g. artificial intelligence - AI, virtual human twin, new approach methodologies as well as methods that cut through these domains) which model and predict relevant biological parameters and exploit relevant end-points and novel (bio)markers for clinical diagnostic and prognostic predictions. Such update of the regulatory science of health technologies should aim at supporting an effective adoption and uptake into routine use by health systems and end-users (healthcare providers, citizens), while maintaining guardrails to ensure that innovative health technologies are backed up by evidence of sufficient quality and relevance to the human situation.

Proposals can cover all types of health technologies, aiming to define improved and novel sources of evidence with proven relevance for regulatory decision-making with a focus on safety and performance throughout their lifecycle, i.e. throughout the continuous process of clinical evaluation. To this end, proposals should address either, or a combination of the following: i) the improvement of existing methodologies and their fitness to specific types or classes of health technologies, including methodology for regulatory assessment and ii) explore and examine to which extent novel information sources as indicated above can be considered as evidence that is satisfactory in view of regulatory needs concerning safety and performance.

Proposals should support the update and refinement of regulatory science on health technologies and contribute actionable information that can be used for improved or novel

Rank #11 · HORIZON-HLTH-2027-02-TOOL-01-two-stage

WATCH

Development of predictive biomarkers of disease progression and treatment response by using AI methodologies for chronic non-communicable diseases

Fit: 49.01 / 100 · Solver: Optimal · CR: 0.0168

Perché questa call ha preso 49/100

La call HORIZON-HLTH-2027-02-TOOL-01-two-stage ottiene un fit del 49.0% dopo ottimizzazione LP. Il criterio dominante e' impact (contributo 16.6/100). Solver: Optimal, CR AHP: 0.0168.

"This topic aims at supporting activities that are enabling or contributing"

*"to one or several expected impacts of destination
"Developing and using new tools,"*

"technologies and digital solutions for a healthy society". To that end, proposals under this"

Gap principali da colmare

- ⚠ Implementation debole: piano esecutivo/consortium readiness non sufficiente
- ⚠ TRL aziendale inferiore al requisito della call (target: 4)
- ⚠ Requisito SME non soddisfatto dal profilo corrente

Azioni concrete (prossimi 3-6 mesi)

- ✅ Costruire shortlist partner (end-user, integrator, academia) e governance WP (90-120 giorni)
- ✅ Eseguire pilot dimostrativo per salire di almeno 1 livello TRL (3-6 mesi)
- ✅ Valutare ruolo da partner o struttura legale/cap table compatibile con requisiti SME

Spiegazione Tecnica AHP + LP

Matrice confronto a coppie (AHP)

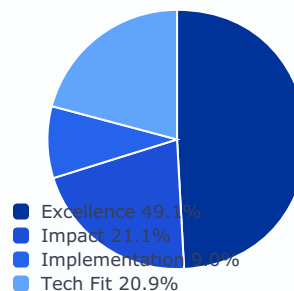
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0.333	1.000	3.000	1.000
0.200	0.333	1.000	0.500
0.500	1.000	2.000	1.000

Pre-LP vs Post-LP

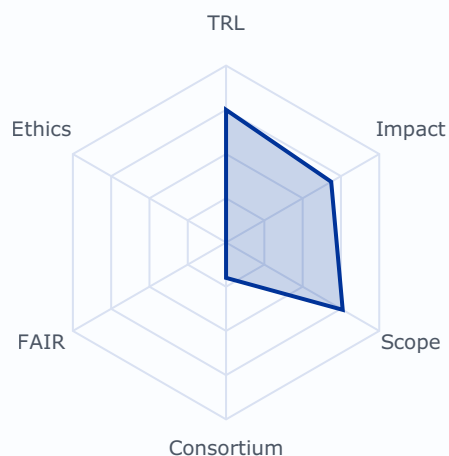
Criterio	Peso AHP	Local pre-LP	Optimal post-LP	Breakdown
excellence	0.4914	0.6864	0.3000	0.1474
impact	0.2105	0.7881	0.7881	0.1659
implementation	0.0896	0.5300	0.2000	0.0179
tech_fit	0.2086	0.7616	0.7616	0.1589

Consistency Ratio: 0.0168 — Matrice consistente

Pesi finali AHP (pie chart)



Spider chart call



Vincoli LP applicati e effetto

- **trl_violation:** True
- **budget_company_available:** 0.0
- **budget_max:** 44200000.0
- **sme_required:** True
- **sme_ok:** False
- **ssh_required:** False
- **gender_balance_required:** False

Expected Outcomes

This topic aims at supporting activities that are enabling or contributing

to one or several expected impacts of destination

“Developing and using new tools, technologies and digital solutions for a healthy society”.

To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- Clinical researchers and developers have access to novel predictive biomarkers to guide a more accurate assessment of disease progression and treatment response and tackle the unmet clinical needs of non-communicable chronic diseases.
- Clinicians and healthcare professionals use clinically validated predictive biomarkers for implementing more effective clinical research and personalised medicine with better health outcomes in chronic non-communicable diseases.
- Key stakeholders have access to trustworthy Artificial Intelligence (AI) tools to guide the development of multimodal predictive biomarkers of higher accuracy and clinical value when compared to the established practice.
- The citizens benefit of better health outcomes thanks to improved clinical guidelines and the implementation of effective biomarker-guided clinical research and personalised healthcare.

Scope

Biomarkers³⁸⁹ are invaluable tools for improving patient outcomes, guiding treatment decisions, accelerating personalised medicine, more effective clinical research and the development of better medicines.

However, despite the scientific discoveries of many clinically relevant biomarkers, estimated on the scale of tens of thousands, only a few biomarkers have been implemented in clinical practice. The traditional ‘one biomarker’ paradigm is insufficient for addressing the unmet clinical needs of chronic, progressive and multifactorial diseases, due to the complexity of the clinical phenotypes characterised by broad inter-and intra-patient heterogeneity. The established biomarkers have limitations in their use as prognostic and predictive indicators, for the assessment of the disease progression and the choices of the optimal therapeutic interventions tailored to the patients’ characteristics.

Therefore, the topic focuses on the clinical development of predictive biomarkers of disease progression and treatment response for chronic non-communicable diseases (excluding cancer) by using established AI methodologies able to combine data of clinically used and candidate biomarkers, with available data from relevant clinical studies, longitudinal and Real-World Data (RWD)³⁹⁰. This topic is expected to support collaborative projects paving the way for future innovations in personalised medicine and enabling more timely and effective therapeutic interventions.

The proposals should address all the following research and innovation activities:

- Set-up a multidisciplinary collaboration to map and evaluate the available information and data on biomarkers currently used in the clinical setting, candidate biomarkers from
- ³⁸⁹ See definition as in the Strategic Research and Innovation Agenda of the Innovative Health Initiative Joint Undertaking:
http://www.ihp.europa.eu/sites/default/files/uploads/Documents/About/IHI_SRIA_ApprovedJan22.pdf
³⁹⁰ EMA definition: “Real-World Data are routinely collected data relating to patient health status or the delivery of healthcare from a variety of sources other than traditional clinical trials (e.g. claims databases, hospital data, electronic health records, registries, mhealth data, etc.)”.

past and ongoing clinical studies, which are scientifically proven as clinically relevant to

Rank #12 · HORIZON-HLTH-2026-01-TOOL-03

WATCH

Integrating New Approach Methodologies (NAMs) to advance biomedical research and regulatory testing

Fit: 46.53 / 100 · Solver: Optimal · CR: 0.0168

Perché questa call ha preso 47/100

La call HORIZON-HLTH-2026-01-TOOL-03 ottiene un fit del 46.5% dopo ottimizzazione LP. Il criterio dominante e' tech_fit (contributo 15.3/100). Solver: Optimal, CR AHP: 0.0168.

"This topic aims at supporting activities that are enabling or contributing"

*"to one or several expected impacts of destination
"Developing and using new tools,""*

"technologies and digital solutions for a healthy society". To that end, proposals under this"

Gap principali da colmare

- ⚠ Implementation debole: piano esecutivo/consortium readiness non sufficiente
- ⚠ TRL aziendale inferiore al requisito della call (target: 4)

Azioni concrete (prossimi 3-6 mesi)

- ✅ Costruire shortlist partner (end-user, integrator, academia) e governance WP (90-120 giorni)
- ✅ Eseguire pilot dimostrativo per salire di almeno 1 livello TRL (3-6 mesi)

Spiegazione Tecnica AHP + LP

Matrice confronto a coppie (AHP)

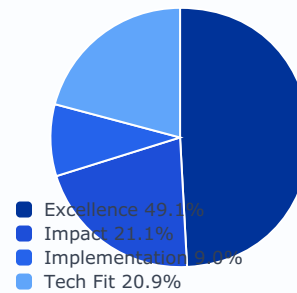
1.000	3.000	5.000	2.000
0.333	1.000	3.000	1.000
0.200	0.333	1.000	0.500
0.500	1.000	2.000	1.000

Pre-LP vs Post-LP

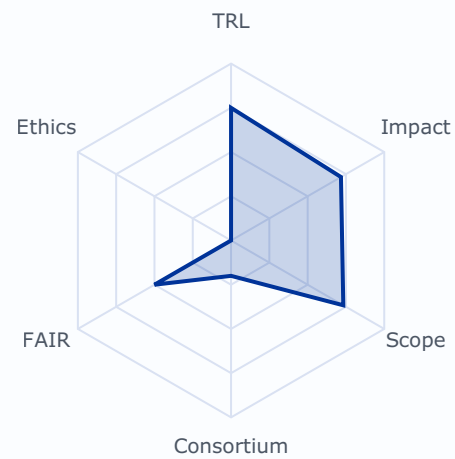
Criterio	Peso AHP	Local pre-LP	Optimal post-LP	Breakdown
excellence	0.4914	0.7176	0.3000	0.1474
impact	0.2105	0.6139	0.6139	0.1292
implementation	0.0896	0.5300	0.4000	0.0358
tech_fit	0.2086	0.7328	0.7328	0.1529

Consistency Ratio: 0.0168 — Matrice consistente

Pesi finali AHP (pie chart)



Spider chart call



Vincoli LP applicati e effetto

- **trl_violation:** True
- **budget_company_available:** 0.0
- **budget_max:** 49000000.0
- **sme_required:** False
- **sme_ok:** False
- **ssh_required:** False
- **gender_balance_required:** False

Expected Outcomes

This topic aims at supporting activities that are enabling or contributing

to one or several expected impacts of destination

“Developing and using new tools, technologies and digital solutions for a healthy society”.

To that end, proposals under this

topic should aim to deliver results that are directed at, tailored towards and contributing to

several of the following expected outcomes:

- Researchers are in possession of improved human-relevant New Approach

Methodologies (NAMs) platforms that capture the

genetic, phenotypic, age-related,

immune, microbiome, and environmental exposure

variability of the human population.

359 This decision is available on the Funding and

Tenders Portal, in the reference documents section for

Horizon Europe, under ‘Simplified costs decisions’ or

through this link:

https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/lis-decision_he_en.pdf

These innovations support more equitable healthcare solutions and personalised

treatment strategies across diverse life stages.

- Industry gets access to platforms that allow a faster pace of innovation for the

development of more cost-effective targeted

therapeutic interventions and improvement

of the safety assessment of chemicals, other medicinal products, and medical devices.

- Patients benefit from innovative platforms and

strategies that improve prediction,

prevention and treatment of diseases, in particular

through enhanced understanding of

disease pathways and mechanisms.

- The general population is better protected through a safer environment, as these

platforms enhance the detection and mitigation of risks

posed by chemicals and other

potentially harmful substances.

- Regulatory bodies gain confidence and trust in NAMs, supporting their integration into

product development, risk assessment, and approval processes.

- Fewer live animals are used in biomedical research and regulatory testing.

Scope

This topic aims to support the ongoing paradigm shift in biomedical research and

safety assessment of chemical compounds by fully

integrating NAMs across the entire

research and regulatory spectrum, from basic

discovery phase to clinical application, and

regulatory testing of medicinal products and medical

devices, and/or industrial and

environmental chemicals.

NAMs include a wide range of innovative and human-relevant technologies such as in-vitro

or human ex-vivo assays, organoids, Organ-on-Chip

(OoC) systems, human tissue models,

induced Pluripotent Stem Cell (iPSC) applications,

virtual twin tools, in-silico methods, and

Artificial Intelligence (AI)-driven modelling.

Although the Commission and several Member States

have supported the development of

NAMs for over two decades, primarily in the context of

chemical risk assessment, regulatory

uptake remains limited. There is a need to address this

situation by delivering validated

NAMs solutions that can be adopted by industry and

accepted by the regulators for the safety

assessment of chemicals. In parallel, there is a growing

readiness to expand the development

and application of NAMs across the entire biomedical

research spectrum, from early

discovery through to clinical translation and regulatory

testing of medicinal products and

medical devices.

Proposals should bring together stakeholders from

academia, health-related infrastructures,

SMEs³⁶⁰, industry, and regulators to develop new

NAMs platforms or improve existing ones

that could be used for biomedical applications and/or

regulatory testing. For biomedical

applications, these platforms should enhance disease

modelling precision, especially in areas

where current animal models are of limited human

relevance, and where NAMs could

³⁶⁰ [https://eur-lex.europa.eu/legal-content/EN/TXT/](https://eur-lex.europa.eu/legal-content/EN/TXT/PDF/?uri=CELEX:32003H0361)

[PDF/?uri=CELEX:32003H0361](https://eur-lex.europa.eu/legal-content/EN/TXT/PDF/?uri=CELEX:32003H0361)

effectively complement or replace animal studies. For proposals addressing regulatory use, in

particular the safety assessment of chemicals, other

medicinal products and medical devices,

the intended context(s) of use should be clearly

defined, with validation strategies and

methodologies aligned with current OECD and/or

European Medicines Agency (EMA)

guidance. Early, proactive, and sustained engagement

Rank #13 · HORIZON-HLTH-2027-02-IND-02-two-stage

WATCH

Portable and versatile Point-of-care diagnostics

Fit: 45.87 / 100 · Solver: Optimal · CR: 0.0168

Perché questa call ha preso 46/100

La call HORIZON-HLTH-2027-02-IND-02-two-stage ottiene un fit del 45.9% dopo ottimizzazione LP. Il criterio dominante e' impact (contributo 15.6/100). Solver: Optimal, CR AHP: 0.0168.

"This topic aims at supporting activities that are enabling or contributing"

*"to one or several expected impacts of destination
"Maintaining an innovative, sustainable, and"*

"competitive EU health industry". To that end, proposals under this topic should aim to deliver"

Gap principali da colmare

- ⚠ Implementation debole: piano esecutivo/consortium readiness non sufficiente
- ⚠ TRL aziendale inferiore al requisito della call (target: 6)
- ⚠ Requisito SME non soddisfatto dal profilo corrente

Azioni concrete (prossimi 3-6 mesi)

- ✅ Costruire shortlist partner (end-user, integrator, academia) e governance WP (90-120 giorni)
- ✅ Eseguire pilot dimostrativo per salire di almeno 1 livello TRL (3-6 mesi)
- ✅ Valutare ruolo da partner o struttura legale/cap table compatibile con requisiti SME

Spiegazione Tecnica AHP + LP

Matrice confronto a coppie (AHP)

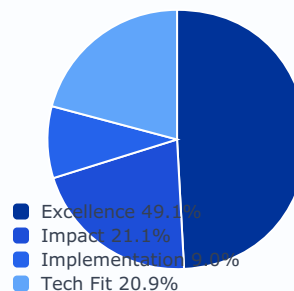
1.000	3.000	5.000	2.000
0.333	1.000	3.000	1.000
0.200	0.333	1.000	0.500
0.500	1.000	2.000	1.000

Pre-LP vs Post-LP

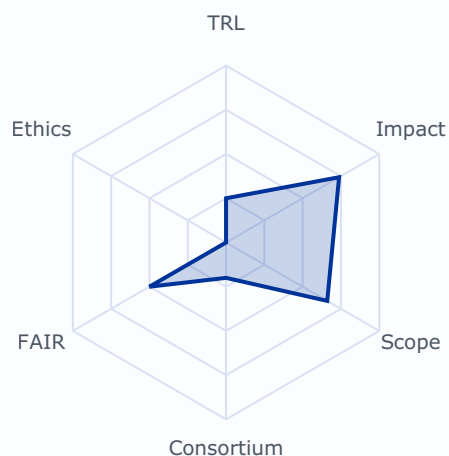
Criterio	Peso AHP	Local pre-LP	Optimal post-LP	Breakdown
excellence	0.4914	0.7379	0.3000	0.1474
impact	0.2105	0.7393	0.7393	0.1556
implementation	0.0896	0.2300	0.2000	0.0179
tech_fit	0.2086	0.6605	0.6605	0.1378

Consistency Ratio: 0.0168 — Matrice consistente

Pesi finali AHP (pie chart)



Spider chart call



Vincoli LP applicati e effetto

- **trl_violation:** True
- **budget_company_available:** 0.0
- **budget_max:** 39300000.0
- **sme_required:** True
- **sme_ok:** False
- **ssh_required:** False
- **gender_balance_required:** False

Expected Outcomes

This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Maintaining an innovative, sustainable, and competitive EU health industry”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- Healthcare professionals dispose of diagnostic tools at the point of care that accelerate therapeutic decision making.
- Patients benefit from fast and accurate diagnosis leading to improved health outcomes.
- Thanks to more efficient diagnosis, health systems will get better evidence for disease control and prevention strategies.

Scope

Point-of-Care (PoC) medical testing has made great technical progress (e.g. improved extraction, microfluidics, miniaturisation, and data processing techniques) with PoC test accuracies nearly matching those of lab-based tests. PoC tests may thus be an alternative to laboratory testing methods, enabling faster diagnostic results and therapeutic decision making.

448 This decision is available on the Funding and Tenders Portal, in the reference documents section for Horizon Europe, under ‘Simplified costs decisions’ or through this link:

https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/lis-decision_he_en.pdf

However, PoC testing is not always achieving a completely accurate diagnosis and one of the major issues with PoC diagnostics is the occurrence of false results during testing, another one is the often-cumbersome sample preparation. Hence there is a need for PoC diagnostics that are more sensitive, selective and easy-to-use allowing for improved clinical practice.

The World Health Organization (WHO) has defined a set of criteria for PoC diagnostics in primary care which, in the advent of digital technologies, has been completed with two additional features and is represented by the acronym REASSURED: Real-time connectivity, Ease of specimen collection and environmental friendliness, Affordable, Sensitive, Specific, User-friendly, Rapid and robust, Equipment-free (or equipment-modest) and Deliverable to end users. To these criteria adds the feature of “sample-to-answer” (sometimes also called “sample-to-result”) and more challenges like: Miniaturisation, power supply, versatility (nature and origin of the human sample), biocompatibility of the used materials and their suitability for mass production, readiness for high-throughput testing, quality control, regulatory compliance, environmental footprint and, last but not least, cost, which is of particular concern in resource-limited settings. All these challenges are not only valid for PoC diagnostics developed for infectious diseases, they equally apply to those that are designed to detect non-communicable diseases as well as their continuous monitoring on patients. Mobile technologies are playing an important role, especially since around 70% of the globally 7.4 billion cell phone users live in developing countries,

Rank #14 · HORIZON-HLTH-2026-01-TOOL-05

WATCH

Pilot actions for follow-on funding: Leveraging EU-funded collaborative research in regenerative medicine

Fit: 45.76 / 100 · Solver: Optimal · CR: 0.0168

Perché questa call ha preso 46/100

La call HORIZON-HLTH-2026-01-TOOL-05 ottiene un fit del 45.8% dopo ottimizzazione LP. Il criterio dominante e' excellence (contributo 14.7/100). Solver: Optimal, CR AHP: 0.0168.

"This topic aims at supporting activities that are enabling or contributing"

*"to one or several expected impacts of destination
"Developing and using new tools,""*

"technologies and digital solutions for a healthy society". To that end, proposals under this"

Gap principali da colmare

- ⚠ Implementation debole: piano esecutivo/consortium readiness non sufficiente
- ⚠ TRL aziendale inferiore al requisito della call (target: 5)

Azioni concrete (prossimi 3-6 mesi)

- ✅ Costruire shortlist partner (end-user, integrator, academia) e governance WP (90-120 giorni)
- ✅ Eseguire pilot dimostrativo per salire di almeno 1 livello TRL (3-6 mesi)

Spiegazione Tecnica AHP + LP

Matrice confronto a coppie (AHP)

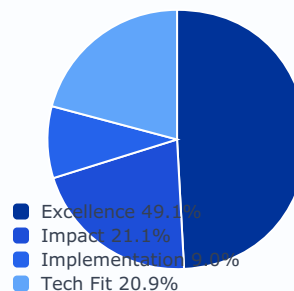
1.000	3.000	5.000	2.000
0.333	1.000	3.000	1.000
0.200	0.333	1.000	0.500
0.500	1.000	2.000	1.000

Pre-LP vs Post-LP

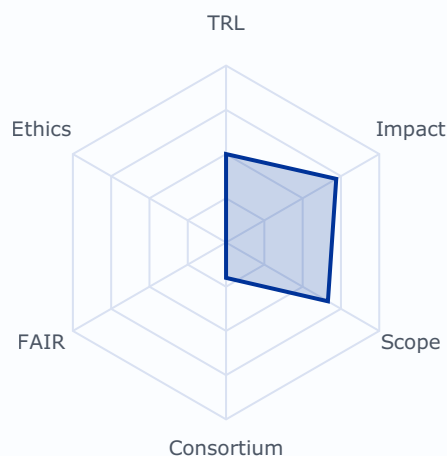
Criterio	Peso AHP	Local pre-LP	Optimal post-LP	Breakdown
excellence	0.4914	0.7193	0.3000	0.1474
impact	0.2105	0.6528	0.6528	0.1374
implementation	0.0896	0.3800	0.3800	0.0340
tech_fit	0.2086	0.6650	0.6650	0.1387

Consistency Ratio: 0.0168 — Matrice consistente

Pesi finali AHP (pie chart)



Spider chart call



Vincoli LP applicati e effetto

- **trl_violation:** True
- **budget_company_available:** 0.0
- **budget_max:** 29500000.0
- **sme_required:** False
- **sme_ok:** False
- **ssh_required:** False
- **gender_balance_required:** False

Expected Outcomes

This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Developing and using new tools, technologies and digital solutions for a healthy society”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- The overall competitiveness of the EU biotechnology sector is strengthened through the further development of closer-to-deployment health innovations.
- The EU benefits from greater impact of the EU's Research and Innovation (R&I) Framework Programmes through successful leveraging of previous EU funding in the field of regenerative medicine.
- EU innovators secure further funding to finalise the last stages of development.
- Patients benefit faster from solutions that improve their health and wellbeing.

367 This decision is available on the Funding and Tenders Portal, in the reference documents section for Horizon Europe, under ‘Simplified costs decisions’ or through this link:

https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/lis-decision_he_en.pdf

Scope

R&I is essential for economic growth and boosting the competitiveness of the EU's life sciences sector. Through the Horizon 2020 and Horizon Europe Framework Programmes, the EU has supported projects that significantly impact our health by fostering scientific discoveries and developing new solutions. Transformational health innovations, such as mRNA vaccines, highlight the importance of collaboration among businesses, research institutions, and healthcare providers. Furthermore, sustained funding throughout the entire value chain is crucial for maximising impact and ensuring more products reach patients faster. The main aim of this topic is to pilot a follow-on funding mechanism, supporting the stepwise development of biotech innovations through collaboration, resulting from previously supported EU R&I actions in the field of health. This topic contributes to strengthening the R&I ecosystem within the EU and supports the implementation of the “Strategy for European Life Sciences”³⁶⁸. Given the importance of biotechnology as a critical technology³⁶⁹, this topic aims to ensure that promising research results are efficiently taken further along the value chain, speeding the time to market or patient through stepwise funding and increasing the EU's competitiveness. The chosen area of focus is regenerative medicine as it has the potential to heal or replace tissues and organs damaged by age, disease, or trauma, as well as to normalise congenital defects. Proposals should focus on prototyping, demonstrating and validating health innovations from TRL 5, moving beyond early-stage research to clinical development, testing, or eventual large-scale manufacturing. The previously funded EU research on which the proposal is build should be applicable to the field of regenerative medicine and should have clear exploitation potential and/or socio-economic benefits for the patients.

Applicants are expected to:

- Demonstrate in their proposal that the health product, therapy or service, has been successfully validated at preclinical level in the prior EU funded project and provide justification of the innovation potential with qualitative and quantitative data (e.g. publications, patent/trademark/design applications, spin-out/start-up track record, regulatory procedures, venture capital pitches, funds

Rank #15 · HORIZON-HLTH-2027-03-TOOL-02

WATCH

Advancing bio-printing of living cells for regenerative medicine

Fit: 43.42 / 100 · Solver: Optimal · CR: 0.0168

Perché questa call ha preso 43/100

La call HORIZON-HLTH-2027-03-TOOL-02 ottiene un fit del 43.4% dopo ottimizzazione LP. Il criterio dominante e' excellence (contributo 14.7/100). Solver: Optimal, CR AHP: 0.0168.

"This topic aims at supporting activities that are enabling or contributing"

*"to one or several expected impacts of destination
"Developing and using new tools,"*

"technologies and digital solutions for a healthy society". To that end, proposals under this"

Gap principali da colmare

- ⚠ Implementation debole: piano esecutivo/consortium readiness non sufficiente
- ⚠ TRL aziendale inferiore al requisito della call (target: 4)

Azioni concrete (prossimi 3-6 mesi)

- ✅ Costruire shortlist partner (end-user, integrator, academia) e governance WP (90-120 giorni)
- ✅ Eseguire pilot dimostrativo per salire di almeno 1 livello TRL (3-6 mesi)

Spiegazione Tecnica AHP + LP

Matrice confronto a coppie (AHP)

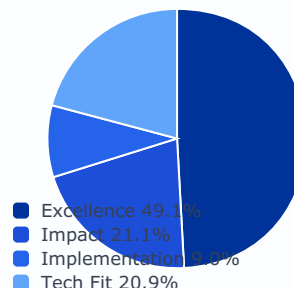
1.000	3.000	5.000	2.000
0.333	1.000	3.000	1.000
0.200	0.333	1.000	0.500
0.500	1.000	2.000	1.000

Pre-LP vs Post-LP

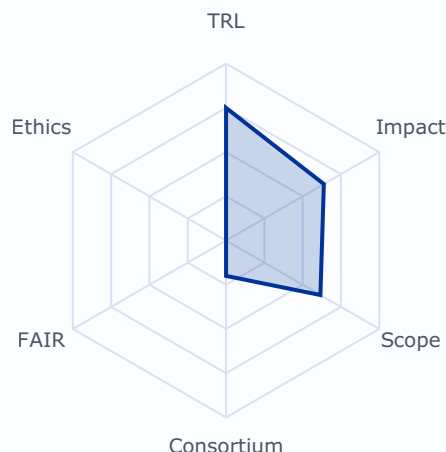
Criterio	Peso AHP	Local pre-LP	Optimal post-LP	Breakdown
excellence	0.4914	0.6385	0.3000	0.1474
impact	0.2105	0.5818	0.5818	0.1225
implementation	0.0896	0.5300	0.4000	0.0358
tech_fit	0.2086	0.6160	0.6160	0.1285

Consistency Ratio: 0.0168 — Matrice consistente

Pesi finali AHP (pie chart)



Spider chart call





Vincoli LP applicati e effetto

- **trl_violation:** True
- **budget_company_available:** 0.0
- **budget_max:** 39300000.0
- **sme_required:** False
- **sme_ok:** False
- **ssh_required:** False
- **gender_balance_required:** False

Expected Outcomes

This topic aims at supporting activities that are enabling or contributing

to one or several expected impacts of destination

“Developing and using new tools, technologies and digital solutions for a healthy society”.

To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to most of the following expected outcomes:

- Biomedical scientists from academia and industry will gain access to entire bio-printing units designed to regenerate human tissue.
- Healthcare professionals acquire information on the safe and effective use of equipment enabling advanced therapies with bio-printed human tissue.
- Healthcare providers dispose of tools enabling them to treat conditions of unmet medical need.
- Individual patients will benefit from a personalised approach to their respective medical condition thanks to the bio-printed regenerative medicine solution.

Scope

Tissue-specific functional 3D bio-printing of living cells has made significant progress as a new approach for transplantation applications in regenerative medicine. There are currently several types of bio-printing technologies under development for the repair of different targeted tissues or organs. To fully unleash the potential of bio-printed cell

constructs for regenerative medicine several bottlenecks still need to be overcome. Various studies in pre-clinical models have shown that bio-printed cell constructs or tissues hold great promise for regenerative medicine, by allowing autologous tissue grafts being printed thus avoiding adverse graft-host reactions. However, translation of such approaches into clinical settings (i.e. humans) and their application to internal organs still needs to be investigated and demonstrated. Dependent on the actual target site (i.e. the defect tissue or organ in the human body) different bio-printing approaches may be preferable. For printing complex tissues and especially entire organs an in-vitro approach followed by transplantation is the preferred way.

“In-situ” bio-printing, sometimes also referred to as “in-vivo”, or as “intraoperative”, reflects a bioprinting process performed on a live subject in a surgical setting and has in certain instances (e.g. tissue repair) advantages over an in-vitro bio-printing technique followed by transplantation. In-situ bio-printing involves direct patterning of bio-inks onto a patient’s body at the target site, allowing for precise construction of a site-matching tissue-structure within the actual physiological location where regeneration or repair is needed. As such, in-situ bio-printing allows for high adaptability, reduced risk of contamination, improved cell viability, function and host integration. The high cell densities present in the human vital organs underscore the importance of bio-inks which contain less additional biomaterials as matrix. Hence the bio-printing of cell constructs that comprise native tissue-like cell densities may facilitate repair and/or regeneration of defective complex tissues or internal organs. For such approaches meticulous engineering of the bio-printing equipment is necessary, involving sophisticated micro-surgical instrumentation and medical imaging platforms. To achieve the desired function and to mimic the natural cues in native tissues for in-vitro printed bio-constructs, the use of additional stimuli is needed,

Rank #16 · HORIZON-HLTH-2027-01-CARE-02

WATCH

Personalised approaches to reduce risks from Adverse Drug Reactions due to administration of multiple medications

Fit: 42.50 / 100 · Solver: Optimal · CR: 0.0168

Perché questa call ha preso 42/100

La call HORIZON-HLTH-2027-01-CARE-02 ottiene un fit del 42.5% dopo ottimizzazione LP. Il criterio dominante e' excellence (contributo 14.7/100). Solver: Optimal, CR AHP: 0.0168.

"This topic aims at supporting activities that are enabling or contributing"

*"to one or several expected impacts of destination
"Ensuring equal access to innovative,"*

"sustainable, and high-quality healthcare". To that end, proposals under this topic should aim"

Gap principali da colmare

- ⚠ Implementation debole: piano esecutivo/consortium readiness non sufficiente
- ⚠ TRL aziendale inferiore al requisito della call (target: 4)
- ⚠ Requisito SME non soddisfatto dal profilo corrente

Azioni concrete (prossimi 3-6 mesi)

- ✅ Costruire shortlist partner (end-user, integrator, academia) e governance WP (90-120 giorni)
- ✅ Eseguire pilot dimostrativo per salire di almeno 1 livello TRL (3-6 mesi)
- ✅ Valutare ruolo da partner o struttura legale/cap table compatibile con requisiti SME

Spiegazione Tecnica AHP + LP

Matrice confronto a coppie (AHP)

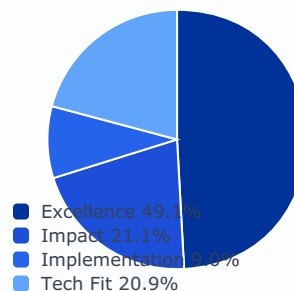
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0.333	1.000	3.000	1.000
0.200	0.333	1.000	0.500
0.500	1.000	2.000	1.000

Pre-LP vs Post-LP

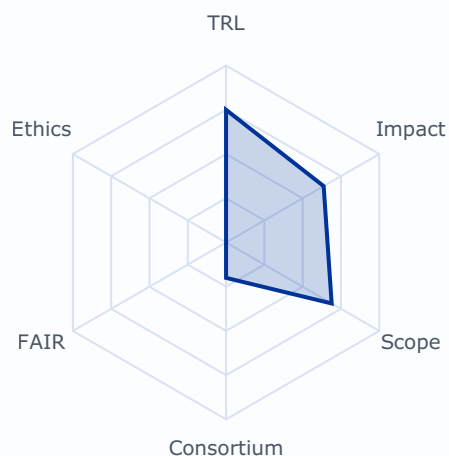
Criterio	Peso AHP	Local pre-LP	Optimal post-LP	Breakdown
excellence	0.4914	0.6372	0.3000	0.1474
impact	0.2105	0.5503	0.5503	0.1158
implementation	0.0896	0.5300	0.2000	0.0179
tech_fit	0.2086	0.6894	0.6894	0.1438

Consistency Ratio: 0.0168 — Matrice consistente

Pesi finali AHP (pie chart)



Spider chart call



Vincoli LP applicati e effetto

- **trl_violation:** True
- **budget_company_available:** 0.0
- **budget_max:** 38000000.0
- **sme_required:** True
- **sme_ok:** False
- **ssh_required:** False
- **gender_balance_required:** False

Expected Outcomes

This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Ensuring equal access to innovative, sustainable, and high-quality healthcare”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following

Scope

While medicinal products contribute considerably to the health of EU citizens, they can also have adverse reactions. It is estimated that around 5% of all hospital deaths are due to an adverse drug reaction. On average, 16% of hospitalised older³⁴¹ patients experience significant ADRs, varying in severity and mostly preventable, with commonly prescribed drug classes (such as diuretics, anti-bacterials, antithrombotic agents, analgesics, antineoplastics, etc.) accounting for most ADRs³⁴². Overall, ADRs increase morbidity, mortality, hospitalisations, and healthcare costs. ADRs from multiple medications contribute significantly to healthcare costs due to increased hospitalisations and treatments, making this an area of focus to achieve cost efficiency. Initial failure to recognise ADRs can generate inappropriate prescription cascades, in which the side effects of drugs are misdiagnosed as symptoms of new problems, resulting in further prescriptions and further side effects that tend to accumulate, confusing and complicating the diagnostic while aggravating the evolution. Therefore, there is a distinct need for research to help identify and prevent such prescription cascades, possibly by maximising the use of technology, as well as to improve multiple drug management in order to reduce patient harm. Furthermore, it is also possible that aside from the ADRs specific to individual drugs taken in combination, new ADRs can emerge as results from the drug combinations themselves. Research activities under this topic should make use of the constantly improving health technologies and data analytics that provide new opportunities to address these issues more effectively, by better integrating medication management into healthcare practices, including into Electronic Health Records (EHR) and decision support systems. Identifying and validating relevant biomarkers for better patient stratification can contribute to significantly decreasing the risk of adverse drug reactions. Biomarkers can also help to detect adverse drug reactions early before occurrence of clinical symptoms and enable early countermeasures. Generating knowledge on the interaction and complexity of biochemical pathways can improve the understanding of patients' response to ADRs and thus provide better tailored treatments and early responses to adverse reactions.

Rank #17 · HORIZON-HLTH-2027-03-TOOL-04

WATCH

Virtual Human Twins (VHTs) for integrated clinical decision support in prevention and diagnosis

Fit: 42.20 / 100 · Solver: Optimal · CR: 0.0168

Perché questa call ha preso 42/100

La call HORIZON-HLTH-2027-03-TOOL-04 ottiene un fit del 42.2% dopo ottimizzazione LP. Il criterio dominante e' tech_fit (contributo 15.1/100). Solver: Optimal, CR AHP: 0.0168.

"This topic aims at supporting activities that are enabling or contributing"

*"to one or several expected impacts of destination
"Developing and using new tools,""*

"technologies and digital solutions for a healthy society". To that end, proposals under this"

Gap principali da colmare

- ⚠ Implementation debole: piano esecutivo/consortium readiness non sufficiente
- ⚠ TRL aziendale inferiore al requisito della call (target: 4)
- ⚠ Requisito SME non soddisfatto dal profilo corrente
- ⚠ La call richiede copertura SSH esplicita

Azioni concrete (prossimi 3-6 mesi)

- ✅ Costruire shortlist partner (end-user, integrator, academia) e governance WP (90-120 giorni)
- ✅ Eseguire pilot dimostrativo per salire di almeno 1 livello TRL (3-6 mesi)
- ✅ Valutare ruolo da partner o struttura legale/cap table compatibile con requisiti SME
- ✅ Integrare partner SSH e work package dedicato a impatti sociali/adozione

Spiegazione Tecnica AHP + LP

Matrice confronto a coppie (AHP)

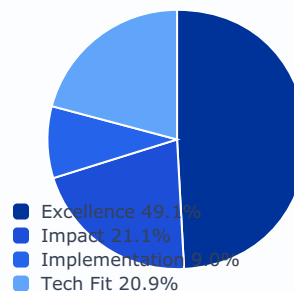
1.000	3.000	5.000	2.000
0.333	1.000	3.000	1.000
0.200	0.333	1.000	0.500
0.500	1.000	2.000	1.000

Pre-LP vs Post-LP

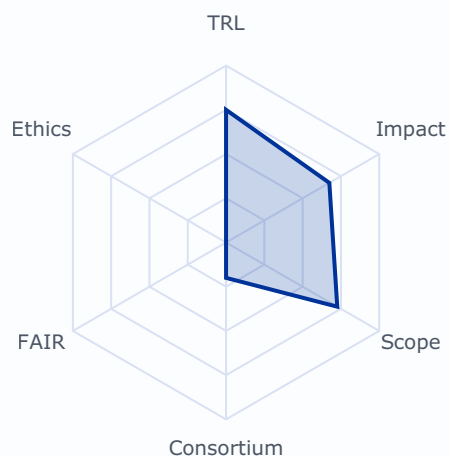
Criterio	Peso AHP	Local pre-LP	Optimal post-LP	Breakdown
excellence	0.4914	0.6742	0.3000	0.1474
impact	0.2105	0.7773	0.5000	0.1052
implementation	0.0896	0.5300	0.2000	0.0179
tech_fit	0.2086	0.7260	0.7260	0.1514

Consistency Ratio: 0.0168 — Matrice consistente

Pesi finali AHP (pie chart)



Spider chart call



Vincoli LP applicati e effetto

- **trl_violation:** True
- **budget_company_available:** 0.0
- **budget_max:** 39300000.0
- **sme_required:** True
- **sme_ok:** False
- **ssh_required:** True
- **gender_balance_required:** False

Expected Outcomes

This topic aims at supporting activities that are enabling or contributing

to one or several expected impacts of destination

“Developing and using new tools, technologies and digital solutions for a healthy society”.

To that end, proposals under this

topic should aim to deliver results that are directed at, tailored towards and contributing to all

the following expected outcomes:

- Healthcare professionals have access to multi-scale⁴⁰¹, multi-organ models of individual patients that aim to improve prevention and diagnosis in high disease burden areas.

- Health professionals benefit from enhanced knowledge of complex diseases and co-morbidities by recourse to multi-scale, multi-organ models.

⁴⁰¹ In the context of this topic, multi-scale refers to modelling at different levels of human anatomy, e.g. at (sub-)cellular, tissue, organ or organ system level.

- Patients with diverse characteristics (e.g. of any sex, age group, racial or ethnic origin⁴⁰²)

benefit from improved, integrated and personalised prevention and diagnostics tools.

- Health professionals and patients benefit from the use of “Virtual Human Twin” (VHT) models which enable integration of other preventive and diagnostic tools and modalities.

Scope

VHTs are digital representations and in-silico models of an individual's health and

disease state at different levels of anatomy. Multi-scale, multi-organ VHT solutions have a

potential for tailored prevention and diagnosis,

particularly in areas of high disease burden,

and can significantly benefit citizens' health and the efficiency of EU health systems.

Proposals should take into account the work of projects funded under topic HORIZON-

HLTH-2023-TOOL-05-03: “Integrated, multi-scale computational models of patient patho-physiology (‘virtual twins’) for personalised disease management”, which had a predominant

focus on disease management, and focus on high-potential multi-disciplinary approaches at

greater complexity (multiscale, multiorgan,

longitudinal), strengthening their deployment in

health and care, including the integration into care pathways and links with other decision

support tools.

The proposals should address all the following activities:

- Select clinical use cases to deliver multi-disciplinary, high impact solutions requiring multi-organ, multi-scale approaches to modelling complex pathophysiology over time, as a basis from where prevention and diagnosis of diseases with high morbidity and mortality could be enhanced. Proposals can put forward use cases in any areas of high disease burden; examples include co-morbidities, chronic cardiovascular conditions, infection and (auto)immunity, inflammation and cancer, diabetes and related conditions, rare diseases, degenerative diseases (including their interaction with mental health conditions), the exposome and its impact on human health and disease.

- Building on current approaches, standards, data repositories (e.g. biobanks, environmental data, others) and modelling assets (e.g. those of the EDITH CSA⁴⁰³ and the Platform for Advanced VHT Models⁴⁰⁴), and new data if relevant, design, develop, extend and validate multi-organ, multi-scale, dynamic computational models that accurately simulate a person's health and disease states, as necessary.

- Evaluate, select, extend and validate diverse modelling methodologies, resulting in integrated, advanced, interoperable, patient-specific VHT models that can integrate

Rank #18 · HORIZON-HLTH-2026-01-STAYHLTH-02

WATCH

Behavioural interventions as primary prevention for Non-Communicable Diseases (NCDs) among young people

Fit: 42.10 / 100 · Solver: Optimal · CR: 0.0168

Perché questa call ha preso 42/100

La call HORIZON-HLTH-2026-01-STAYHLTH-02 ottiene un fit del 42.1% dopo ottimizzazione LP. Il criterio dominante e' excellence (contributo 14.7/100). Solver: Optimal, CR AHP: 0.0168.

"This topic aims at supporting activities that are enabling or contributing"

*"to one or several expected impacts of destination
"Staying healthy in a rapidly changing"*

"society". To that end, proposals under this topic should aim to deliver results that are directed"

Gap principali da colmare

- ⚠ Implementation debole: piano esecutivo/consortium readiness non sufficiente
- ⚠ TRL aziendale inferiore al requisito della call (target: 4)
- ⚠ La call richiede copertura SSH esplicita

Azioni concrete (prossimi 3-6 mesi)

- ✅ Costruire shortlist partner (end-user, integrator, academia) e governance WP (90-120 giorni)
- ✅ Eseguire pilot dimostrativo per salire di almeno 1 livello TRL (3-6 mesi)
- ✅ Integrare partner SSH e work package dedicato a impatti sociali/adozione

Spiegazione Tecnica AHP + LP

Matrice confronto a coppie (AHP)

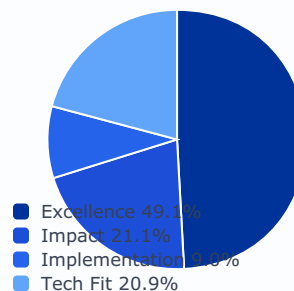
1.000	3.000	5.000	2.000
0.333	1.000	3.000	1.000
0.200	0.333	1.000	0.500
0.500	1.000	2.000	1.000

Pre-LP vs Post-LP

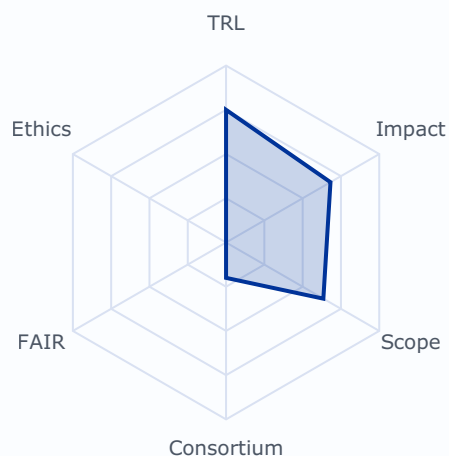
Criterio	Peso AHP	Local pre-LP	Optimal post-LP	Breakdown
excellence	0.4914	0.6817	0.3000	0.1474
impact	0.2105	0.7118	0.5000	0.1052
implementation	0.0896	0.5300	0.4000	0.0358
tech_fit	0.2086	0.6352	0.6352	0.1325

Consistency Ratio: 0.0168 — Matrice consistente

Pesi finali AHP (pie chart)



Spider chart call



Vincoli LP applicati e effetto

- **trl_violation:** True
- **budget_company_available:** 0.0
- **budget_max:** 20600000.0
- **sme_required:** False
- **sme_ok:** False
- **ssh_required:** True
- **gender_balance_required:** False

Expected Outcomes

This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Staying healthy in a rapidly changing society”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to most of the following expected outcomes:

- Healthcare professionals have access to behavioural interventions that can be used to establish and reinforce healthy habits and sustain behavioural changes.
- Health professionals and educators have access to evidence-based strategies to mitigate risks of Non-Communicable Diseases (NCDs) for youth, with clear metrics that can be used to assess health outcomes.
- Youth have increased individual responsibility through targeted education, digital services, including easily accessible tools for self-monitoring, and community-based

support, stemming from increased collaboration between healthcare professionals, educators and families.

- Researchers have access to Real-World Data (RWD) 59, existing health data infrastructure and digital tools, including Artificial Intelligence (AI), which can contribute to the sustained success of behavioural health interventions.
- Policymakers at local, regional, national and EU levels have new knowledge on behavioural interventions on NCDs among youth, which they can use to improve interventions in diverse European contexts.

Scope

The topic is focused on behavioural interventions for youth, defined as 12 to 25 years old, for the primary prevention of the top NCDs later in life, where “top NCDs” refers to the most prevalent NCDs⁶⁰. For the purpose of this call, NCDs explicitly exclude cancer.

Implementation research should be conducted to implement existing behavioural interventions. These interventions should be evidence-based and have an emphasis on empowerment and self-management (e.g. health literacy, health education, health promotion). As self-monitoring is an essential element of self-management, proposals should include user-friendly hardware and software for efficient self-monitoring (i.e. wearables and point-of-care devices for measuring various physiological parameters and other predictors and other biomarkers and the corresponding apps for easy readout and tracking, possibly also including gamification elements). Hardware and software should be interoperable in line with internationally accepted standards in order to avoid lock-in effects and assure scalability.

Proposals should also include most of the following aspects:

- Ensure that gender-sensitive and intersectional approaches are integrated, addressing potential gender-specific barriers for groups at risk of discrimination, as well as cultural and socioeconomic backgrounds, and should also outline how digital tools, including AI and RWD and biomarkers (e.g. genomic data, wearables, etc.) or existing relevant administrative dataset, will be integrated to enhance the scalability, personalisation, and effectiveness of interventions in the long-term.
- Present a clear, evidence-based strategy showing how the interventions will be tailored, deployed, and assessed at individual, family, community, and societal levels, while considering social inequalities and lifestyle factors (i.e. nutrition, sleep rhythm) and ensuring a robust methodological framework for evaluating the effectiveness of interventions (e.g. randomised controlled trials, quasi-experimental designs, etc.), with clearly defined indicators of success of the intervention (e.g. biometric markers, psychosocial wellbeing metrics, physical activity change, etc.). Applicants should evaluate unintended consequences for all interventions.

Rank #19 · HORIZON-HLTH-2026-01-ENVHLTH-04

WATCH

Towards climate resilient, prepared and carbon neutral populations and healthcare systems

Fit: 41.76 / 100 · Solver: Optimal · CR: 0.0168

Perché questa call ha preso 42/100

La call HORIZON-HLTH-2026-01-ENVHLTH-04 ottiene un fit del 41.8% dopo ottimizzazione LP. Il criterio dominante e' excellence (contributo 14.7/100). Solver: Optimal, CR AHP: 0.0168.

"This topic aims at supporting activities that are enabling or contributing"

*"to one or several expected impacts of destination
"Living and working in a health-promoting"*

"environment". To that end, proposals under this topic should aim to deliver results that are"

Gap principali da colmare

- ⚠ Impact non ancora distintivo su outcomes e policy relevance
- ⚠ Implementation debole: piano esecutivo/consortium readiness non sufficiente
- ⚠ TRL aziendale inferiore al requisito della call (target: 4)
- ⚠ La call richiede copertura SSH esplicita

Azioni concrete (prossimi 3-6 mesi)

- ✓ Allineare proposition agli expected outcomes della call con KPI misurabili (60 giorni)
- ✓ Costruire shortlist partner (end-user, integrator, academia) e governance WP (90-120 giorni)
- ✓ Eseguire pilot dimostrativo per salire di almeno 1 livello TRL (3-6 mesi)
- ✓ Integrare partner SSH e work package dedicato a impatti sociali/adozione

Spiegazione Tecnica AHP + LP

Matrice confronto a coppie (AHP)

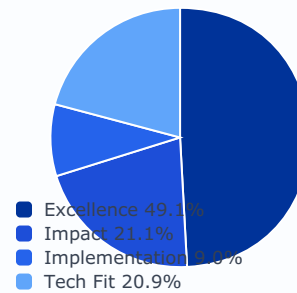
1.000	3.000	5.000	2.000
0.333	1.000	3.000	1.000
0.200	0.333	1.000	0.500
0.500	1.000	2.000	1.000

Pre-LP vs Post-LP

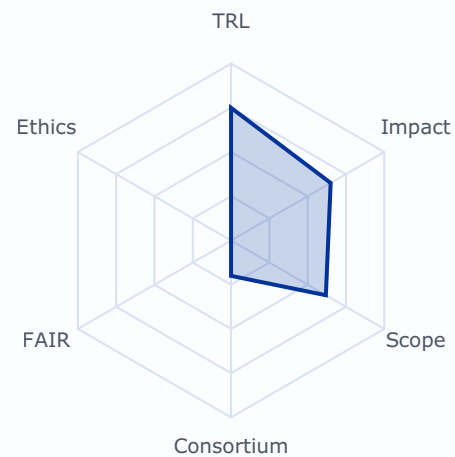
Criterio	Peso AHP	Local pre-LP	Optimal post-LP	Breakdown
excellence	0.4914	0.6503	0.3000	0.1474
impact	0.2105	0.5205	0.5000	0.1052
implementation	0.0896	0.5300	0.4000	0.0358
tech_fit	0.2086	0.6189	0.6189	0.1291

Consistency Ratio: 0.0168 — Matrice consistente

Pesi finali AHP (pie chart)



Spider chart call



Vincoli LP applicati e effetto

- **trl_violation:** True
- **budget_company_available:** 0.0
- **budget_max:** 45000000.0
- **sme_required:** False
- **sme_ok:** False
- **ssh_required:** True
- **gender_balance_required:** False

Expected Outcomes

This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Living and working in a health-promoting environment”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to most of the following expected outcomes:

- The healthcare sector is supported with new technological developments and frameworks for greening, decarbonising and adapting to climate change, thus also contributing to protect biodiversity and ecosystem services.
- Governments, public health authorities, healthcare providers and practitioners, social care services and civil society have access to the best available evidence on the health costs and benefits (including co-benefits) of climate adaptation and mitigation actions and interventions.
- Policymakers and public authorities develop environment, climate change and health policies and interventions based on robust frameworks and incorporating innovative, inclusive and accessible solutions and technologies.
- Governments and public health authorities are supported in their adoption of robust frameworks and interventions to tackle societal challenges linked to the health impacts of climatic and environmental factors.
- Populations are empowered and equipped with knowledge, tools and resources to adopt health-protective behaviours and adapt to health-related climate risks.

94 This decision is available on the Funding and Tenders Portal, in the reference documents section for Horizon Europe, under ‘Simplified costs decisions’ or through this link:

https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/lis-decision_he_en.pdf

Scope

The health sector accounts for nearly 5% of global greenhouse gas (GHG) emissions and generates significant demands for energy and materials, as well as dangerous polluting streams. Proactive mitigation efforts in the health sector can significantly reduce GHG emissions and pollution, saving many lives and contributing to relieve pressures on biodiversity and ecosystem services. However, specific mechanisms for emission reductions in the health sector remain less defined compared to those in other sectors.

At the same time, the climate crisis subjects healthcare systems to unprecedented pressures (e.g. on infrastructure, workforce, overall systems) while simultaneously having to respond to increasing healthcare needs. To reduce pressure in healthcare systems and generally improve public health, it is crucial to design inclusive and accessible interventions that prevent the health impacts of climate change and related environmental degradation, increase resilience and preparedness of individuals and communities and foster the adoption of health-protective behaviours.

Research activities under this topic should generate evidence on the opportunities and health co-benefits of mitigation in the health sector as well as foster the development of low-carbon medical technologies and digital solutions for the sector. Proposals should also support the design of effective, scalable, cost-effective and transferable interventions and frameworks that can be applied across a wide range of healthcare settings and/or in population, community and societal contexts and involving, when relevant, public and patient engagement. Proposals can consider both living and working environments. More specifically research actions under this topic should include some of the following

Rank #20 · HORIZON-HLTH-2026-01-CARE-03

WATCH

Identifying and addressing low-value care in health and care systems

Fit: 41.61 / 100 · Solver: Optimal · CR: 0.0168

Perché questa call ha preso 42/100

La call HORIZON-HLTH-2026-01-CARE-03 ottiene un fit del 41.6% dopo ottimizzazione LP. Il criterio dominante e' excellence (contributo 14.7/100). Solver: Optimal, CR AHP: 0.0168.

"This topic aims at supporting activities that are enabling or contributing"

*"to one or several expected impacts of destination
"Ensuring equal access to innovative,"*

"sustainable, and high-quality healthcare". To that end, proposals under this topic should aim"

Gap principali da colmare

- ⚠ Impact non ancora distintivo su outcomes e policy relevance
- ⚠ Implementation debole: piano esecutivo/consortium readiness non sufficiente
- ⚠ TRL aziendale inferiore al requisito della call (target: 4)

Azioni concrete (prossimi 3-6 mesi)

- ✅ Allineare proposition agli expected outcomes della call con KPI misurabili (60 giorni)
- ✅ Costruire shortlist partner (end-user, integrator, academia) e governance WP (90-120 giorni)
- ✅ Eseguire pilot dimostrativo per salire di almeno 1 livello TRL (3-6 mesi)

Spiegazione Tecnica AHP + LP

Matrice confronto a coppie (AHP)

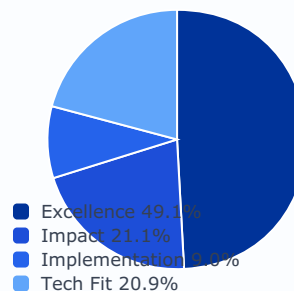
1.000	3.000	5.000	2.000
0.333	1.000	3.000	1.000
0.200	0.333	1.000	0.500
0.500	1.000	2.000	1.000

Pre-LP vs Post-LP

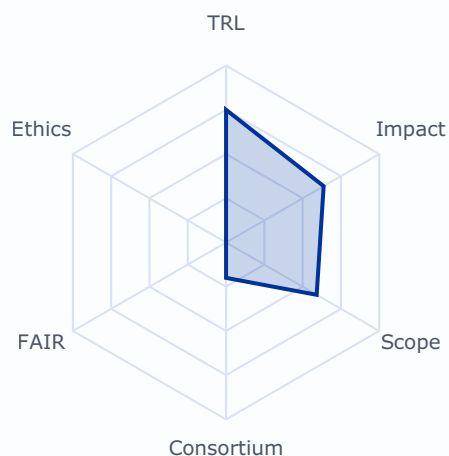
Criterio	Peso AHP	Local pre-LP	Optimal post-LP	Breakdown
excellence	0.4914	0.6372	0.3000	0.1474
impact	0.2105	0.5200	0.5200	0.1095
implementation	0.0896	0.5300	0.4000	0.0358
tech_fit	0.2086	0.5916	0.5916	0.1234

Consistency Ratio: 0.0168 — Matrice consistente

Pesi finali AHP (pie chart)



Spider chart call



Vincoli LP applicati e effetto

- **trl_violation:** True
- **budget_company_available:** 0.0
- **budget_max:** 38000000.0
- **sme_required:** False
- **sme_ok:** False
- **ssh_required:** False
- **gender_balance_required:** False

Expected Outcomes

This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Ensuring equal access to innovative, sustainable, and high-quality healthcare”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following

Scope

Low-value care, as defined in the footnote, can have widespread negative consequences for patients, caregivers, healthcare professionals, the health and care system, and the broader environment. A 2017 OECD report³³⁰ estimated that “wasteful healthcare spending is common” and that “up to one-fifth of healthcare spending could be redirected towards better uses”. Low-value care represents a significant challenge, contributing to waste, costs, misuse of resources, and inefficiencies. Addressing low-value care can free up and allow reallocation of valuable healthcare resources to other areas of need, thereby maximising health outcomes, improving health and care systems resilience, and reducing their environmental impact. In this context, a recent report³³¹ by the Expert Group on Health Systems Performance Assessment: “From a health system perspective, low-value care encompasses overuse, misuse and underuse of healthcare services (for example, prevention, diagnostics, treatment, medication). Overuse and/or misuse comprise the delivery of harmful, ineffective, inappropriate, or not cost-effective healthcare services. Underuse refers to healthcare services not provided or used despite being necessary. Low-value care can lead to negative consequences for patients, their caregivers, the healthcare workforce, the health system as a whole and the wider environment.”

³³⁰ OECD (2017), Tackling Wasteful Spending on Health, OECD Publishing, Paris.
<http://dx.doi.org/10.1787/9789264266414-en>

³³¹ Report by the Expert Group on Health Systems Performance Assessment: “Identifying, Measuring And Reducing Low-Value Care In The Context Of Health System Performance Assessment”.

Systems Performance Assessment (HSPA)³³² establishes the methodological basis and metrics to identify, measure and reduce low-value care. Research activities under this topic should adopt a patient-centred approach that considers the needs and preferences of patients and citizens. They should promote socially acceptable solutions, taking into account relevant ethical, social and legal aspects and foster dialogue and collaboration between policymakers, healthcare providers, healthcare professionals, and patients/citizens. Proposals should engage citizens and

Rank #21 · HORIZON-HLTH-2026-01-DISEASE-09

WATCH

Multisectoral approach to tackle chronic non-communicable diseases: implementation research maximising collaboration and coordination with sectors and in settings beyond the healthcare system (GACD)

Fit: 41.49 / 100 · Solver: Optimal · CR: 0.0168

Perché questa call ha preso 41/100

La call HORIZON-HLTH-2026-01-DISEASE-09 ottiene un fit del 41.5% dopo ottimizzazione LP. Il criterio dominante e' excellence (contributo 14.7/100). Solver: Optimal, CR AHP: 0.0168.

"This topic aims at supporting activities that are enabling or contributing"

*"to one or several expected impacts of destination
"Tackling diseases and reducing disease"*

"burden". To that end, proposals under this topic should aim to deliver results that are directed"

Gap principali da colmare

- ⚠ Implementation debole: piano esecutivo/consortium readiness non sufficiente
- ⚠ TRL aziendale inferiore al requisito della call (target: 4)
- ⚠ La call richiede copertura SSH esplicita

Azioni concrete (prossimi 3-6 mesi)

- ✅ Costruire shortlist partner (end-user, integrator, academia) e governance WP (90-120 giorni)
- ✅ Eseguire pilot dimostrativo per salire di almeno 1 livello TRL (3-6 mesi)
- ✅ Integrare partner SSH e work package dedicato a impatti sociali/adozione

Spiegazione Tecnica AHP + LP

Matrice confronto a coppie (AHP)

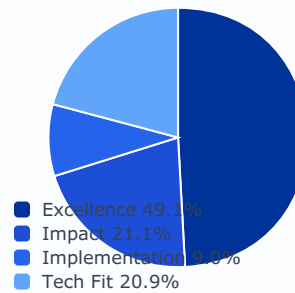
1.000	3.000	5.000	2.000
0.333	1.000	3.000	1.000
0.200	0.333	1.000	0.500
0.500	1.000	2.000	1.000

Pre-LP vs Post-LP

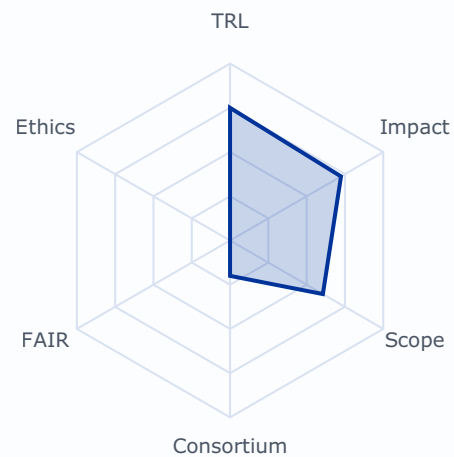
Criterio	Peso AHP	Local pre-LP	Optimal post-LP	Breakdown
excellence	0.4914	0.7243	0.3000	0.1474
impact	0.2105	0.6550	0.5000	0.1052
implementation	0.0896	0.5300	0.4000	0.0358
tech_fit	0.2086	0.6060	0.6060	0.1264

Consistency Ratio: 0.0168 — Matrice consistente

Pesi finali AHP (pie chart)



Spider chart call



Vincoli LP applicati e effetto

- **trl_violation:** True
- **budget_company_available:** 0.0
- **budget_max:** 9800000.0
- **sme_required:** False
- **sme_ok:** False
- **ssh_required:** True
- **gender_balance_required:** False

Expected Outcomes

This topic aims at supporting activities that are enabling or contributing

to one or several expected impacts of destination “Tackling diseases and reducing disease burden”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to some of the following expected outcomes:

- Researchers, policymakers, healthcare- and non-healthcare-related stakeholders and authorities in low- and middle-income countries (LMICs)¹⁷⁰ and/or those in high-income countries (HICs) serving disadvantaged populations have access to improved insights and evidence on how to maximise collaboration and coordination with sectors and in settings beyond the healthcare system in the context of Non-Communicable Diseases (NCDs).

- Researchers, policymakers, healthcare- and non-healthcare-related stakeholders and authorities have an improved understanding how the proposed interventions draw on collaborative multisectoral engagement and utilise the different settings in which people are educated, work and live, to expand efforts to reduce risks, prevent, manage and control NCDs.

¹⁶⁹ This decision is available on the Funding and Tenders Portal, in the reference documents section for Horizon Europe, under ‘Simplified costs decisions’ or through this link:

https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/lis-decision_he_en.pdf

¹⁷⁰ As defined by the World Bank, <https://www.worldbank.org>

- Communities, relevant stakeholders from different sectors and authorities are fully engaged in implementing and taking up interventions that tackle the growing burden of NCDs through actions in sectors and settings outside the traditional health system and its facilities health-related outcomes, improve quality of life across the life course and extend healthy life expectancy.

Scope

The Commission is a member of the Global Alliance for Chronic Diseases (GACD)¹⁷¹.

The GACD specifically addresses NCDs and supports implementation research¹⁷² to improve health outcomes. This topic is launched in concertation with the other GACD members (international funding agencies) and aligned with the 11th GACD call.

Besides health-related determinants, the burden of NCDs is also driven by structural and social inequities, population ageing, the effects of globalisation on marketing and trade, diet and activity, commercial and economic determinants of health, rapid urbanisation and climate change, factors over which a conventional healthcare-oriented system has little sway. There is a need for a comprehensive approach, involving sectors outside of health, to meet the global targets that governments have agreed upon to protect people from chronic NCDs. Tackling chronic NCDs most effectively therefore requires engagement and coordinated policy development within and across many government departments, including education, workplace, environment, social systems, housing, transportation, agriculture, food industry and nutrition, leisure and culture.

The aim of this topic is to fund implementation research focused on strategies to tackle the growing burden of NCDs through actions in sectors and settings outside the traditional health system¹⁷³ and its facilities (with or without the involvement of the healthcare system) to attain equitable health-related outcomes or influence health determinants for people living in LMICs, and/or underserved populations in HICs. Proposals can focus on more than one setting and/or include cross-sectoral approaches, involving both health and non-health settings to expand efforts to reduce risks, prevent, manage and control NCDs. Safety is a major concern in non-health settings, and proposals should ensure any risks and safety considerations are addressed.

The choice of intervention(s)¹⁷⁴ and provision of existing evidence of the intervention’s effectiveness, cost-effectiveness, sustainability, scalability and potential for long-term health and other impacts should be justified (and in what context this evidence has been generated).

¹⁷¹ <https://www.gacd.org>

¹⁷² https://iris.who.int/bitstream/handle/10665/91758/9789241506212_eng.pdf

Rank #22 · HORIZON-HLTH-2026-01-CARE-01

WATCH

Public procurement of innovative solutions for improving citizens' access to healthcare through integrated or personalised approaches

Fit: 41.32 / 100 · Solver: Optimal · CR: 0.0168

Perché questa call ha preso 41/100

La call HORIZON-HLTH-2026-01-CARE-01 ottiene un fit del 41.3% dopo ottimizzazione LP. Il criterio dominante e' excellence (contributo 14.7/100). Solver: Optimal, CR AHP: 0.0168.

"This topic aims at supporting activities that are enabling or contributing"

*"to one or several expected impacts of destination
"Ensuring equal access to innovative,"*

"sustainable, and high-quality healthcare". To that end, proposals under this topic should aim"

Gap principali da colmare

- ⚠ Implementation debole: piano esecutivo/consortium readiness non sufficiente
- ⚠ TRL aziendale inferiore al requisito della call (target: 7)

Azioni concrete (prossimi 3-6 mesi)

- ✅ Costruire shortlist partner (end-user, integrator, academia) e governance WP (90-120 giorni)
- ✅ Eseguire pilot dimostrativo per salire di almeno 1 livello TRL (3-6 mesi)

Spiegazione Tecnica AHP + LP

Matrice confronto a coppie (AHP)

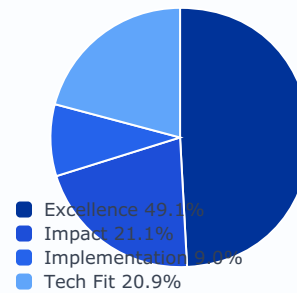
1.000	3.000	5.000	2.000
0.333	1.000	3.000	1.000
0.200	0.333	1.000	0.500
0.500	1.000	2.000	1.000

Pre-LP vs Post-LP

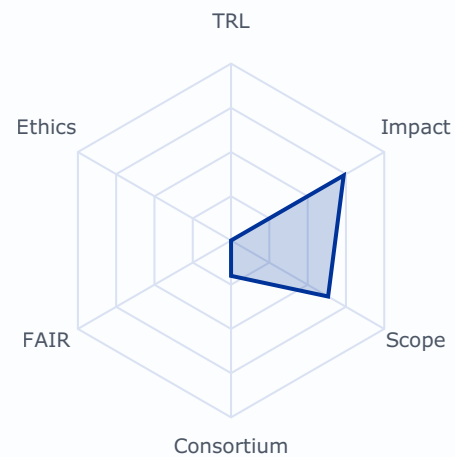
Criterio	Peso AHP	Local pre-LP	Optimal post-LP	Breakdown
excellence	0.4914	0.7354	0.3000	0.1474
impact	0.2105	0.6011	0.6011	0.1265
implementation	0.0896	0.0800	0.0800	0.0072
tech_fit	0.2086	0.6335	0.6335	0.1321

Consistency Ratio: 0.0168 — Matrice consistente

Pesi finali AHP (pie chart)



Spider chart call



Vincoli LP applicati e effetto

- **trl_violation:** True
- **budget_company_available:** 0.0
- **budget_max:** 24500000.0
- **sme_required:** False
- **sme_ok:** False
- **ssh_required:** False
- **gender_balance_required:** False

Expected Outcomes

This topic aims at supporting activities that are enabling or contributing

to one or several expected impacts of destination

“Ensuring equal access to innovative, sustainable, and high-quality healthcare”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to several of the following expected outcomes:

- Patients and their carers, health authorities and health professionals will benefit from the deployment of innovative solutions, designed around actual clinical needs, that facilitate identification, integration or coordination of care, allowing for personalised, more accessible, inclusive and higher quality of health and care.
- Patients will benefit from personalised approaches, improved care experiences and health outcomes or are more engaged in their care and better equipped to make informed decisions on their health, in collaboration with health professionals.
- Health professionals will be better equipped with, and thus benefit from, improved means for diagnosis, care delivery and/or coordination, with multi-disciplinary approaches and closer patient engagement, thanks to new technologies.
- Health systems will improve their accessibility, coordination mechanisms, effectiveness, inclusivity and resilience, thanks to innovative solutions, with a better use of resources, thus stimulating organisational innovation, cultural transformation within hospitals, and European-level collaboration.

Scope

Public Procurement of Innovative Solutions (PPI)³¹⁵ can boost the wider market uptake of high impact innovations in health systems, while enhancing the tools available to ³¹⁵ <https://research-and-innovation.ec.europa.eu/strategy/support-policy-making/shaping-eu-research-and-innovation-policy/new-european-innovation-agenda/innovation-procurement/horizon-europe-funding->

providers and improving access to healthcare for citizens. This supports enhancement of social rights ³¹⁶ and the European economic competitiveness by providing business opportunities and thus incentives to innovate. By acting as early adopters of innovative solutions, procurers can open up new growth markets for the European industry and small and medium-sized enterprises (SMEs)³¹⁷. Joint/ collaborative demand-driven initiatives can help create economies of scale and facilitate the wider adoption of innovations in the health sector for the benefits of patients in need.

PPI actions target consortia of procurers with a similar need that want to procure together the deployment of innovative solutions for supporting integration of care or diagnostics for personalised medicine. This topic does not provide direct funding to developers, industry or research organisations to perform research and development. They will be able to respond to the call for tenders launched by consortia of procurers funded under this topic.

Proposals should specify which segment of the patient population they target, the specific organisational and/or technological innovations to be procured, and why the proposed innovative solutions would be fit for purpose adhering, when relevant, to the principles of integrated care³¹⁸ or personalised medicine³¹⁹.

Examples of target groups that could be covered by this action are: patients at risk of vulnerability such as children and older/frail people with complex needs for health and social care; people with multi-morbidities or non-communicable diseases of high burden; people with both physical and mental health conditions; people living with rare diseases or cancer; persons with disabilities; other groups of patients in need of highly integrated and coordinated care. Proposals should pay attention to how gender and intersectional factors (e.g. caregiving responsibilities, work-related health disparities etc.)

Rank #23 · HORIZON-HLTH-2027-01-ENVHLTH-MISSCLIMA-03

WATCH

Tools and technologies to support health adaptation to climate change

Fit: 41.14 / 100 · Solver: Optimal · CR: 0.0168

Perché questa call ha preso 41/100

La call HORIZON-HLTH-2027-01-ENVHLTH-MISSCLIMA-03 ottiene un fit del 41.1% dopo ottimizzazione LP. Il criterio dominante e' excellence (contributo 14.7/100). Solver: Optimal, CR AHP: 0.0168.

"This topic aims at supporting activities that are enabling or contributing"

*"to one or several expected impacts of destination
"Living and working in a health-promoting"*

"environment" and the EU Mission on Adaptation to Climate Change. To that end, proposals"

Gap principali da colmare

- ⚠ Implementation debole: piano esecutivo/consortium readiness non sufficiente
- ⚠ TRL aziendale inferiore al requisito della call (target: 6)

Azioni concrete (prossimi 3-6 mesi)

- ✅ Costruire shortlist partner (end-user, integrator, academia) e governance WP (90-120 giorni)
- ✅ Eseguire pilot dimostrativo per salire di almeno 1 livello TRL (3-6 mesi)

Spiegazione Tecnica AHP + LP

Matrice confronto a coppie (AHP)

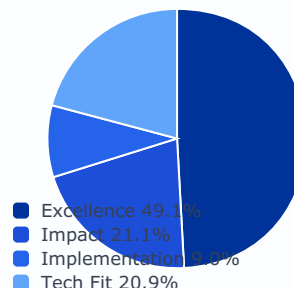
1.000	3.000	5.000	2.000
0.333	1.000	3.000	1.000
0.200	0.333	1.000	0.500
0.500	1.000	2.000	1.000

Pre-LP vs Post-LP

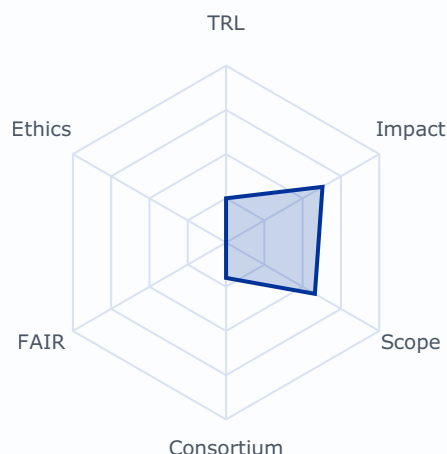
Criterio	Peso AHP	Local pre-LP	Optimal post-LP	Breakdown
excellence	0.4914	0.6296	0.3000	0.1474
impact	0.2105	0.5815	0.5815	0.1224
implementation	0.0896	0.2300	0.2300	0.0206
tech_fit	0.2086	0.5799	0.5799	0.1210

Consistency Ratio: 0.0168 — Matrice consistente

Pesi finali AHP (pie chart)



Spider chart call





Vincoli LP applicati e effetto

- **trl_violation:** True
- **budget_company_available:** 0.0
- **budget_max:** 20000000.0
- **sme_required:** False
- **sme_ok:** False
- **ssh_required:** False
- **gender_balance_required:** False

Expected Outcomes

This topic aims at supporting activities that are enabling or contributing

to one or several expected impacts of destination

“Living and working in a health-promoting environment” and the EU Mission on Adaptation to Climate Change. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to most of the following expected outcomes:

- Populations, public authorities and healthcare systems benefit from innovative solutions and technologies to increase surveillance and prevention and reduce climatic and environmental health risks.
- Policymakers and public authorities develop and implement environment, climate change and health policies and interventions supported by nearly fit-for-use solutions that can be further upscaled and deployed.

Scope

Enhancing the adaptive capacity and resilience of healthcare systems and communities is crucial to prevent and reduce the health impacts of climate change. However, many of the urgently needed technologies, tools, systems and solutions are still at an early developmental stage, relying on further support for development and testing. Proposals under this topic are expected to close this gap and build on innovations being developed in the field, supported through, among others, EU Research and Innovation (R&I) funding. In this context, Pre-commercial Procurement (PCP) projects can drive innovation and speed up the development of technologies for health adaptation to climate change by supporting the research and development of solutions to increase the resilience and preparedness of healthcare systems, communities and individuals against climate change. By focusing on early-stage solutions, PCP fosters collaboration between public sector buyers (e.g. public authorities, local authorities, health organisations) and private developers to create climate adaptation technologies, systems and solutions in the context of human health. These solutions will accelerate the transition to more climate-resilient healthcare systems and societies. PCP actions target consortia of procurers with similar needs that want to jointly procure the development of innovative solutions for supporting adaptation efforts. This topic does not

provide direct funding to developers, industry or research organisations to perform Research and Development (R&D). They will be able to respond to the call for tenders launched by consortia of procurers funded under this topic. Proposals under this topic should support the development of innovative solutions, tools and models to enhance surveillance, prediction, prevention, risk management and diagnosis (e.g. testing), supporting the adaptation, resilience, and preparedness of healthcare systems and populations to climatic and climate-related environmental health risks.

More specifically proposals can support any of the areas listed below:

- Geospatial technologies and decision-support frameworks that help local authorities and healthcare providers track at “high resolution” and better manage direct and indirect health risks related to climate change.

Rank #24 · HORIZON-HLTH-2027-02-DISEASE-01-two-stage

WATCH

Innovative healthcare interventions for non-communicable diseases

Fit: 41.14 / 100 · Solver: Optimal · CR: 0.0168

Perché questa call ha preso 41/100

La call HORIZON-HLTH-2027-02-DISEASE-01-two-stage ottiene un fit del 41.1% dopo ottimizzazione LP. Il criterio dominante e' excellence (contributo 14.7/100). Solver: Optimal, CR AHP: 0.0168.

"This topic aims at supporting activities that are enabling or contributing"

*"to one or several expected impacts of destination
"Tackling diseases and reducing disease"*

"burden". To that end, proposals under this topic should aim to deliver results that are directed"

Gap principali da colmare

- ⚠ Implementation debole: piano esecutivo/consortium readiness non sufficiente
- ⚠ TRL aziendale inferiore al requisito della call (target: 4)
- ⚠ Requisito SME non soddisfatto dal profilo corrente
- ⚠ La call richiede copertura SSH esplicita

Azioni concrete (prossimi 3-6 mesi)

- ✅ Costruire shortlist partner (end-user, integrator, academia) e governance WP (90-120 giorni)
- ✅ Eseguire pilot dimostrativo per salire di almeno 1 livello TRL (3-6 mesi)
- ✅ Valutare ruolo da partner o struttura legale/cap table compatibile con requisiti SME
- ✅ Integrare partner SSH e work package dedicato a impatti sociali/adozione

Spiegazione Tecnica AHP + LP

Matrice confronto a coppie (AHP)

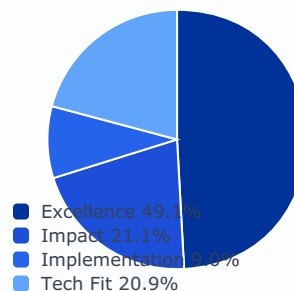
1.000	3.000	5.000	2.000
0.333	1.000	3.000	1.000
0.200	0.333	1.000	0.500
0.500	1.000	2.000	1.000

Pre-LP vs Post-LP

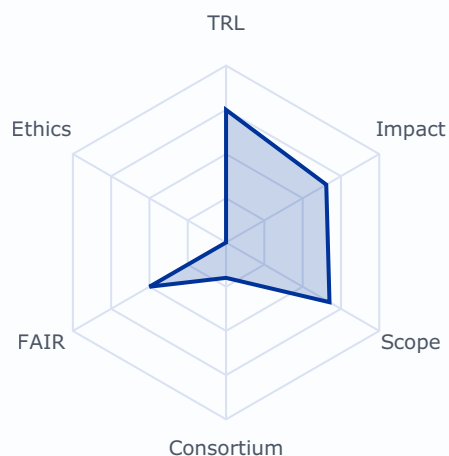
Criterio	Peso AHP	Local pre-LP	Optimal post-LP	Breakdown
excellence	0.4914	0.6542	0.3000	0.1474
impact	0.2105	0.6242	0.5000	0.1052
implementation	0.0896	0.5300	0.2000	0.0179
tech_fit	0.2086	0.6753	0.6753	0.1409

Consistency Ratio: 0.0168 — Matrice consistente

Pesi finali AHP (pie chart)



Spider chart call



Vincoli LP applicati e effetto

- **trl_violation:** True
- **budget_company_available:** 0.0
- **budget_max:** 63800000.0
- **sme_required:** True
- **sme_ok:** False
- **ssh_required:** True
- **gender_balance_required:** False

Expected Outcomes

This topic aims at supporting activities that are enabling or contributing

to one or several expected impacts of destination

“Tackling diseases and reducing disease burden”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- Researchers, developers and clinical practitioners have access to state-of-the-art knowledge, data, technologies, tools, methods, best practices, and trainings to develop innovative healthcare interventions aimed at reducing burden of the following specific Non-Communicable Diseases (NCDs): cardiovascular diseases, diabetes, chronic respiratory diseases or chronic kidney diseases.
- Scientific and clinical communities can use innovative healthcare interventions to generate meaningful advances in clinical practice and care for patients with NCDs following validation in late-stage clinical trials.
- Scientific and clinical communities make wide use of relevant databases and/or integrate them with existing infrastructures for storage and sharing of collected data according to FAIR²²⁸ principles, thereby encouraging further use of the data.
- Policymakers, scientific and clinical communities, developers, patient organisations, regulators, and other relevant bodies are informed of the research advances made and the requirements for a widespread implementation of the innovative therapeutic interventions and complementary approaches.
- Patients and caregivers are constructively engaged with the research, ensuring that their needs are catered for, with the aim of tangibly benefitting from the interventions.

Scope

NCDs represent over 80% of the disease burden in Europe and the leading cause of avoidable premature deaths. Innovative and effective healthcare interventions are required to provide treatment and disease management solutions and assure best quality of care for patients suffering from NCDs when prevention strategies have failed.

Proposals should address all the following aspects:

227 This decision is available on the Funding and Tenders Portal, in the reference documents section for Horizon Europe, under ‘Simplified costs decisions’ or through this link:

https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/ls-decision_he_en.pdf

228 See definition of FAIR data in the introduction to this Work Programme part.

- Perform rigorous early stage²²⁹ clinical trial(s) to validate novel or refined healthcare interventions²³⁰ for treatment and/or disease management solutions for patients suffering from the following specific NCDs: cardiovascular diseases, diabetes, chronic respiratory diseases or chronic kidney diseases ²³¹. Whenever relevant, existing co- and multimorbidities should be addressed in the trial design.
- Clinical trial(s) should be supported by completed proof-of-concept²³² of clinical safety and efficacy.
- Both preclinical research and the draft clinical trial protocol should be completed at the time of submission of the proposal. Proposals should also demonstrate evidence of preliminary consultations with ethics and regulatory authorities at the time of submission.
- A sound feasibility assessment, including an appropriate patient selection and realistic recruitment plans, justified by publications or preliminary results should be provided.
- Take into account sex and gender differences in all relevant aspects throughout the research process, and consider stratification criteria such as age, disability, racial or ethnic origin²³³, socio-economic status, genetic and epigenetic variations, etc., where relevant.
- Use and/or develop technologies, including digital ones (e.g. (generative) Artificial Intelligence, wearable technologies) to help implement

Rank #25 · HORIZON-HLTH-2026-01-ENVHLTH-01

WATCH

Towards a better understanding and anticipation of the impacts of climate change on health

Fit: 41.12 / 100 · Solver: Optimal · CR: 0.0168

Perché questa call ha preso 41/100

La call HORIZON-HLTH-2026-01-ENVHLTH-01 ottiene un fit del 41.1% dopo ottimizzazione LP. Il criterio dominante e' excellence (contributo 14.7/100). Solver: Optimal, CR AHP: 0.0168.

"This topic aims at supporting activities that are enabling or contributing"

*"to one or several expected impacts of destination
"Living and working in a health-promoting"*

"environment". To that end, proposals under this topic should aim to deliver results that are"

Gap principali da colmare

- ⚠ Implementation debole: piano esecutivo/consortium readiness non sufficiente
- ⚠ TRL aziendale inferiore al requisito della call (target: 4)
- ⚠ La call richiede copertura SSH esplicita

Azioni concrete (prossimi 3-6 mesi)

- ✅ Costruire shortlist partner (end-user, integrator, academia) e governance WP (90-120 giorni)
- ✅ Eseguire pilot dimostrativo per salire di almeno 1 livello TRL (3-6 mesi)
- ✅ Integrare partner SSH e work package dedicato a impatti sociali/adozione

Spiegazione Tecnica AHP + LP

Matrice confronto a coppie (AHP)

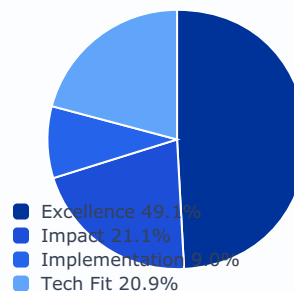
1.000	3.000	5.000	2.000
0.333	1.000	3.000	1.000
0.200	0.333	1.000	0.500
0.500	1.000	2.000	1.000

Pre-LP vs Post-LP

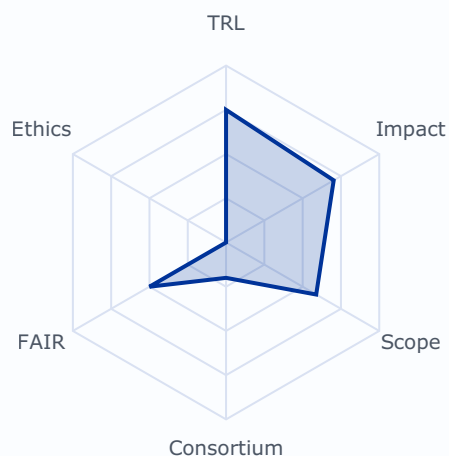
Criterio	Peso AHP	Local pre-LP	Optimal post-LP	Breakdown
excellence	0.4914	0.7028	0.3000	0.1474
impact	0.2105	0.6356	0.5000	0.1052
implementation	0.0896	0.5300	0.4000	0.0358
tech_fit	0.2086	0.5884	0.5884	0.1227

Consistency Ratio: 0.0168 — Matrice consistente

Pesi finali AHP (pie chart)



Spider chart call



Vincoli LP applicati e effetto

- **trl_violation:** True
- **budget_company_available:** 0.0
- **budget_max:** 55000000.0
- **sme_required:** False
- **sme_ok:** False
- **ssh_required:** True
- **gender_balance_required:** False

Expected Outcomes

This topic aims at supporting activities that are enabling or contributing

to one or several expected impacts of destination

“Living and working in a health-promoting environment”. To that end, proposals under this topic

should aim to deliver results that are

directed at, tailored towards and contributing to most of the following expected outcomes:

- Citizens, patients, public authorities, social care services, healthcare practitioners and policymakers have a better understanding of the climatic health risks and determinants of
- 84 This decision is available on the Funding and Tenders Portal, in the reference documents section for Horizon Europe, under ‘Simplified costs decisions’ or through this link:

https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/lis-decision_he_en.pdf

disease and are better equipped to address health outcomes through enhanced and inclusive prevention, resilience, adaptation, preparedness and response, including better diagnosis and treatment.

- Governments, public health authorities, researchers and civil society organisations are supported to tackle societal challenges linked to the health impacts of climatic factors.
- Public authorities, organisations and the research community can rely on data collection and sharing according to FAIR85 principles and leveraging of data availability and quality.
- Policymakers and public authorities develop evidence-based climate change and health policies and interventions that are nature positive, inclusive and responsive to diverse population needs.

Scope

The climate crisis poses an existential challenge to planetary and human health with

larger effects on populations, groups and regions in a vulnerable situation. Climate change

increases the incidence of non-communicable diseases and the prevalence of mental health

conditions and facilitates the emergence and spread of infectious diseases. Climate change can

act as a risk multiplier and exacerbates existing health conditions and vulnerabilities.

Applicants should explicitly state in their proposal which of the following broad focus areas is

targeted and the proposed work should address only this specific broad focus area:

- Non-Communicable Diseases (NCDs) and/or individual safety (e.g. injuries or fatalities), excluding mental health aspects: proposals should explore evidence on the complex interactions between climate change (e.g. changes in the frequency and intensity of extreme weather events) and NCDs and individual safety, which often involve multiple climate exposure pathways and compound and cascading climatic events.
- Mental health, considering interactions with brain health if relevant: in the broad focus area of mental health and psychosocial well-being, proposals should increase the evidence on the acute and long-term impacts of climate change and the understanding of new syndromes related to climate stress.
- Infectious diseases, including vector-borne and non-vector-borne: proposals should increase the understanding of the factors driving climate-related burden from infectious diseases.

In general, proposals should develop approaches to prevent and reduce the impacts of climate

factors in the studied health outcomes and increase population and workforce resilience. A

One Health approach should also be applied where relevant.

More specifically, research actions under this topic should include several of the following

Rank #26 · HORIZON-HLTH-2026-01-DISEASE-02

WATCH

Innovative interventions to prevent the harmful effects of using digital technologies on the mental health of children and young adults

Fit: 41.10 / 100 · Solver: Optimal · CR: 0.0168

Perché questa call ha preso 41/100

La call HORIZON-HLTH-2026-01-DISEASE-02 ottiene un fit del 41.1% dopo ottimizzazione LP. Il criterio dominante e' excellence (contributo 14.7/100). Solver: Optimal, CR AHP: 0.0168.

"This topic aims at supporting activities that are enabling or contributing"

*"to one or several expected impacts of destination
"Tackling diseases and reducing disease"*

"burden". To that end, proposals under this topic should aim to deliver results that are directed"

Gap principali da colmare

- ⚠ Implementation debole: piano esecutivo/consortium readiness non sufficiente
- ⚠ TRL aziendale inferiore al requisito della call (target: 4)
- ⚠ Requisito SME non soddisfatto dal profilo corrente
- ⚠ La call richiede copertura SSH esplicita

Azioni concrete (prossimi 3-6 mesi)

- ✅ Costruire shortlist partner (end-user, integrator, academia) e governance WP (90-120 giorni)
- ✅ Eseguire pilot dimostrativo per salire di almeno 1 livello TRL (3-6 mesi)
- ✅ Valutare ruolo da partner o struttura legale/cap table compatibile con requisiti SME
- ✅ Integrare partner SSH e work package dedicato a impatti sociali/adozione

Spiegazione Tecnica AHP + LP

Matrice confronto a coppie (AHP)

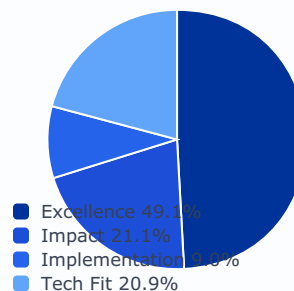
1.000	3.000	5.000	2.000
0.333	1.000	3.000	1.000
0.200	0.333	1.000	0.500
0.500	1.000	2.000	1.000

Pre-LP vs Post-LP

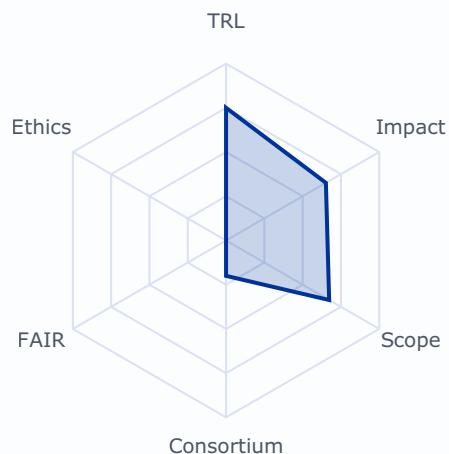
Criterio	Peso AHP	Local pre-LP	Optimal post-LP	Breakdown
excellence	0.4914	0.6519	0.3000	0.1474
impact	0.2105	0.7111	0.5000	0.1052
implementation	0.0896	0.5300	0.2000	0.0179
tech_fit	0.2086	0.6731	0.6731	0.1404

Consistency Ratio: 0.0168 — Matrice consistente

Pesi finali AHP (pie chart)



Spider chart call



Vincoli LP applicati e effetto

- **trl_violation:** True
- **budget_company_available:** 0.0
- **budget_max:** 44200000.0
- **sme_required:** True
- **sme_ok:** False
- **ssh_required:** True
- **gender_balance_required:** False

Expected Outcomes

This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Tackling diseases and reducing disease burden”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- Researchers and healthcare professionals have an improved understanding of the neuro-biological and cognitive/behavioural evidence base on the correlation and impact of digital technologies on mental health, including brain development.
- Policymakers and digital technology and content developers are provided with a robust evidence base on the impact (positive or negative) of digital technologies on mental health in children and young adults¹³⁸.
- Policymakers, digital technology developers, and educational institutions amongst others make use (e.g. developing guidelines) of the evidence base and widely implement the newly developed interventions aimed at promoting children and young adults’ mental health while mitigating any negative impacts of digital technology use.
- Children, young adults, families, guardians, educators, and carers have access to the newly developed interventions designed to prevent harm and promote the positive use of digital technologies.
- Children and young adults are empowered and develop resilience, including digital literacy, enabling them to engage in a healthy and positive way with digital technologies.

Scope

Already before the COVID-19 pandemic, 1 in 6 people in the EU suffered from mental health issues. The economic costs of it are estimated at 4% of the Gross Domestic Product (GDP)¹³⁹ and since then these figures worsened¹⁴⁰ in particular among vulnerable groups such as children and adolescents or those at risk of discrimination. Digital technologies have the potential to enhance mental health for instance by providing access to information, support networks and therapy services¹⁴¹. However, there are indications that the excessive or misguided use of digital technologies, particularly among children and young adults, can negatively affect mental health and exacerbate mental disorders. There is an urgent need for more robust data to foster a safer, responsible and healthier use of digital technologies among children and young adults, prioritising the protection of their mental health.

Therefore, proposals should aim at generating robust scientific evidence on the impact of digital technologies, as well as developing and testing context-specific digital interventions that promote the positive and responsible use of them to improve mental health, avoiding the development or exacerbation of mental disorders. These innovative digital interventions should leverage multi-source data (e.g. sleep patterns, heart rate, stress levels, screen-time analytics, social media use, biological data, clinical data), and could include the use of Artificial Intelligence (AI). When handling data and indicators, sex and gender identity-

¹³⁸ There is no universal definition of youth and young adults. For the purpose of this topic, we follow the WHO definition of young adult a person aged 15-24: <https://www.who.int/southeastasia/health-topics/adolescent-health>

¹³⁹ https://health.ec.europa.eu/system/files/2020-02/2018_healthatglance_rep_en_0.pdf

¹⁴⁰ [https://www.europarl.europa.eu/RegData/etudes/BRIE/2023/751416/EPRS_BRI\(2023\)751416_EN.pdf](https://www.europarl.europa.eu/RegData/etudes/BRIE/2023/751416/EPRS_BRI(2023)751416_EN.pdf)

¹⁴¹ <https://eur-lex.europa.eu/legal-content/EN/TXT/PDF/?uri=CELEX:52023DC0298>

disaggregated data should be collected and analysed, incorporating intersectional factors where feasible.

The applicants should address all the following aspects:

- Generate the neuro-biological and cognitive/

Rank #27 · HORIZON-HLTH-2027-01-DISEASE-10

WATCH

Prevention and management of chronic non- communicable diseases in children and young people (GACD)

Fit: 41.07 / 100 · Solver: Optimal · CR: 0.0168

Perché questa call ha preso 41/100

La call HORIZON-HLTH-2027-01-DISEASE-10 ottiene un fit del 41.1% dopo ottimizzazione LP. Il criterio dominante e' excellence (contributo 14.7/100). Solver: Optimal, CR AHP: 0.0168.

"This topic aims at supporting activities that are enabling or contributing"

*"to one or several expected impacts of destination
"Tackling diseases and reducing disease"*

"burden". To that end, proposals under this topic should aim to deliver results that are directed"

Gap principali da colmare

- ⚠ Implementation debole: piano esecutivo/consortium readiness non sufficiente
- ⚠ TRL aziendale inferiore al requisito della call (target: 4)
- ⚠ La call richiede copertura SSH esplicita

Azioni concrete (prossimi 3-6 mesi)

- ✅ Costruire shortlist partner (end-user, integrator, academia) e governance WP (90-120 giorni)
- ✅ Eseguire pilot dimostrativo per salire di almeno 1 livello TRL (3-6 mesi)
- ✅ Integrare partner SSH e work package dedicato a impatti sociali/adozione

Spiegazione Tecnica AHP + LP

Matrice confronto a coppie (AHP)

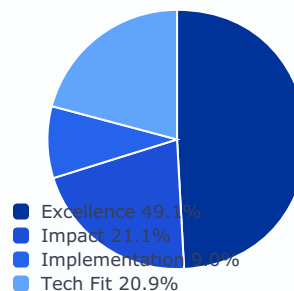
1.000	3.000	5.000	2.000
0.333	1.000	3.000	1.000
0.200	0.333	1.000	0.500
0.500	1.000	2.000	1.000

Pre-LP vs Post-LP

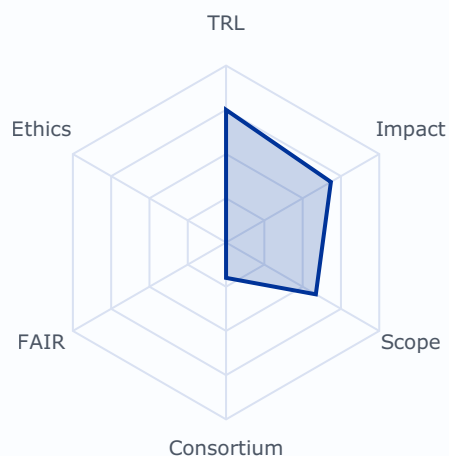
Criterio	Peso AHP	Local pre-LP	Optimal post-LP	Breakdown
excellence	0.4914	0.6845	0.3000	0.1474
impact	0.2105	0.6144	0.5000	0.1052
implementation	0.0896	0.5300	0.4000	0.0358
tech_fit	0.2086	0.5861	0.5861	0.1223

Consistency Ratio: 0.0168 — Matrice consistente

Pesi finali AHP (pie chart)



Spider chart call



Vincoli LP applicati e effetto

- **trl_violation:** True
- **budget_company_available:** 0.0
- **budget_max:** 11800000.0
- **sme_required:** False
- **sme_ok:** False
- **ssh_required:** True
- **gender_balance_required:** False

Expected Outcomes

This topic aims at supporting activities that are enabling or contributing

to one or several expected impacts of destination “Tackling diseases and reducing disease burden”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to some of the following expected outcomes:

- Researchers, healthcare practitioners and providers in low- and middle-income countries (LMICs) 274 and/or those in high-income countries (HICs) serving disadvantaged populations have access to improved insights and evidence on how to equitably promote the early prevention, risk reduction, and timely diagnosis of Non-Communicable Diseases (NCDs) in children and/or young people.
- Policymakers, public health managers and authorities, parents and their children, and young adults have access to evidence and recommendations for national programmes and policies to improve quality of life in children and/or young people and extend healthy life expectancy.
- Researchers, clinicians, policymakers, public health managers and authorities have an improved understanding how to effectively adapt and/or scale up interventions for

273 This decision is available on the Funding and Tenders Portal, in the reference documents section for Horizon Europe, under ‘Simplified costs decisions’ or through this link:

https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/lis-decision_he_en.pdf

274 As defined by the World Bank, <https://www.worldbank.org>

prevention and management of chronic NCDs in children and/or young people at local, regional, and national levels.

- Communities, parents and their children, young adults, local stakeholders and authorities are fully engaged in implementing and taking up interventions that tackle NCDs in children and/or young people.

Scope

The Commission is a member of the Global Alliance for Chronic Diseases (GACD)²⁷⁵.

The GACD specifically addresses NCDs and supports implementation research²⁷⁶ to improve health outcomes. This topic is launched in concertation with the other GACD members (international funding agencies) and aligned with the 12th GACD call.

Chronic NCDs that begin in childhood have an impact on both quality of life and life expectancy. Onset of many NCDs diseases occurs at younger ages in LMICs, and this is further accompanied by a longer duration of disease and a higher rate of complications, including multimorbidity. The conditions in which people are born, grow and live (the social determinants of health) including access to good nutrition, education, housing, and healthcare are major contributors to health and ill health²⁷⁷. Up to 70% of preventable adult deaths from NCDs are linked to risk factors originating in childhood and adolescence²⁷⁸, and interventions that can successfully control or prevent chronic disease in young people can dramatically improve health outcomes later in life.

Childhood and adolescence are critical periods, when behaviours associated with NCD risk are adopted including tobacco use, alcohol use, substance abuse, unhealthy diets and sedentary lifestyles and children and young people are often targeted by commercial marketing of unhealthy products.

The aim of this topic is to fund implementation research, exploring strategies, evidence-based program and policy interventions across prevention, diagnosis, screening and management of chronic NCDs, centred on the critical life stages spanning early childhood to young adulthood (1-24 years of age) living in LMICs, and/or underserved populations in HICs.

In this regard, proposals focused on implementation research should explore implementation strategies on evidence-based interventions, adaptations of interventions and tailored interventions, or initiatives including (though not limited to) those focussed on one or more of the following:

- Policy evaluation to tackle childhood- and/or youth-relevant social, economic, political, structural or commercial determinants of chronic NCD conditions.

275 <https://www.gacd.org>

276 <https://iris.who.int/bitstream/handle/>

Rank #28 · HORIZON-HLTH-2027-01-STAYHLTH-01

WATCH

Addressing disabilities through the life course to support independent living and inclusion

Fit: 40.77 / 100 · Solver: Optimal · CR: 0.0168

Perché questa call ha preso 41/100

La call HORIZON-HLTH-2027-01-STAYHLTH-01 ottiene un fit del 40.8% dopo ottimizzazione LP. Il criterio dominante e' excellence (contributo 14.7/100). Solver: Optimal, CR AHP: 0.0168.

"This topic aims at supporting activities that are enabling or contributing"

*"to one or several expected impacts of destination
"Staying healthy in a rapidly changing"*

"society". To that end, proposals under this topic should aim to deliver results that are directed"

Gap principali da colmare

- ⚠ Impact non ancora distintivo su outcomes e policy relevance
- ⚠ Implementation debole: piano esecutivo/consortium readiness non sufficiente
- ⚠ TRL aziendale inferiore al requisito della call (target: 4)
- ⚠ La call richiede copertura SSH esplicita

Azioni concrete (prossimi 3-6 mesi)

- ✅ Allineare proposition agli expected outcomes della call con KPI misurabili (60 giorni)
- ✅ Costruire shortlist partner (end-user, integrator, academia) e governance WP (90-120 giorni)
- ✅ Eseguire pilot dimostrativo per salire di almeno 1 livello TRL (3-6 mesi)
- ✅ Integrare partner SSH e work package dedicato a impatti sociali/adozione

Spiegazione Tecnica AHP + LP

Matrice confronto a coppie (AHP)

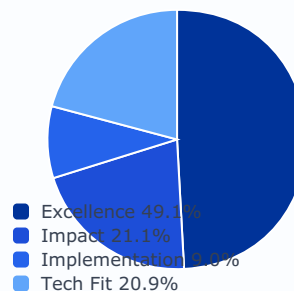
1.000	3.000	5.000	2.000
0.333	1.000	3.000	1.000
0.200	0.333	1.000	0.500
0.500	1.000	2.000	1.000

Pre-LP vs Post-LP

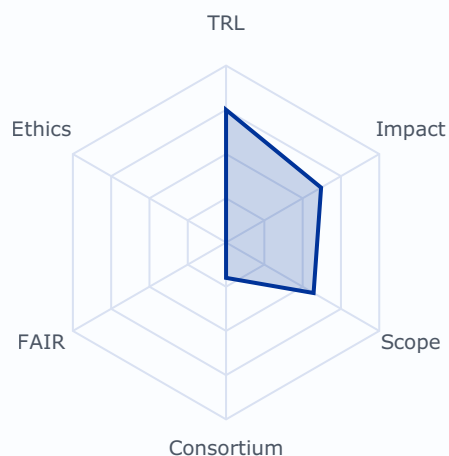
Criterio	Peso AHP	Local pre-LP	Optimal post-LP	Breakdown
excellence	0.4914	0.6204	0.3000	0.1474
impact	0.2105	0.5442	0.5000	0.1052
implementation	0.0896	0.5300	0.4000	0.0358
tech_fit	0.2086	0.5715	0.5715	0.1192

Consistency Ratio: 0.0168 — Matrice consistente

Pesi finali AHP (pie chart)



Spider chart call



Vincoli LP applicati e effetto

- **trl_violation:** True
- **budget_company_available:** 0.0
- **budget_max:** 39300000.0
- **sme_required:** False
- **sme_ok:** False
- **ssh_required:** True
- **gender_balance_required:** False

Expected Outcomes

This topic aims at supporting activities that are enabling or contributing

to one or several expected impacts of destination “Staying healthy in a rapidly changing society”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- Persons with disabilities are empowered and can enjoy their rights to live independently, participate in society and be included in the community on an equal basis with others.
- The scientific community develops innovative solutions with a focus on removing barriers faced by persons with disabilities to live independently and they are provided with community support services where they live in the community.
- Policymakers, health and care services, social and service providers, disability organisations, funders, the scientific community, and other relevant bodies are informed of the research advances and best practices addressing the health and needs of persons with disabilities to support them living independently and being included in society.

Scope

The focus of this topic is human-centred on persons with long-term disabilities⁷³ -

physical, mental, intellectual or sensory- aiming at supporting independent living across the life-course from a health perspective. Persons with disabilities have an equal right to live independently and be included in the community.

Independent living requires a differentiated landscape of quality, accessible, person-centred and affordable, community- and family-based services comprising personal assistance, medical and social care and interventions by social

⁷² This decision is available on the Funding and Tenders Portal, in the reference documents section for Horizon Europe, under ‘Simplified costs decisions’ or through this link:

https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/lis-decision_he_en.pdf

⁷³ Persons with disabilities include those who have long-term physical, mental, intellectual or sensory impairments which in interaction with various barriers may hinder their full and effective participation in society on an equal basis with others (Art. 1 of the Convention on the Rights of Persons with Disabilities - <https://www.ohchr.org/en/instruments-mechanisms/instruments/convention-rights-persons-disabilities>).

workers, thereby facilitating everyday activities and providing choice to persons with disabilities and their families⁷⁴.

The objective of this topic is to explore new ways to promote independent living and inclusion in society of persons with disabilities, reducing to the maximum possible the impact of barriers faced in their daily lives, and supporting the transition from institutions to living in the community while addressing all-encompassing aspects of personal support, such as community transformation, service provision, assistive and accessible technologies and environments.

Research actions under this topic should address at least three of the following areas:

- Health related research addressing disabilities that stem from health conditions and health conditions associated to disabilities. Thus, research may look into finding the causes of the disease(s) leading to the disability and/or disease treatment with the purpose of supporting independent living. Innovative solutions could also include among

Rank #29 · HORIZON-HLTH-2027-01-ENVHLTH-02

WATCH

Integrating climate-related exposures into the human exposome and characterising its changes in response to climate change

Fit: 40.47 / 100 · Solver: Optimal · CR: 0.0168

Perché questa call ha preso 40/100

La call HORIZON-HLTH-2027-01-ENVHLTH-02 ottiene un fit del 40.5% dopo ottimizzazione LP. Il criterio dominante e' excellence (contributo 14.7/100). Solver: Optimal, CR AHP: 0.0168.

"This topic aims at supporting activities that are enabling or contributing"

*"to one or several expected impacts of destination
"Living and working in a health-promoting"*

"environment". To that end, proposals under this topic should aim to deliver results that are"

Gap principali da colmare

- ⚠ Implementation debole: piano esecutivo/consortium readiness non sufficiente
- ⚠ TRL aziendale inferiore al requisito della call (target: 4)
- ⚠ La call richiede copertura SSH esplicita

Azioni concrete (prossimi 3-6 mesi)

- ✅ Costruire shortlist partner (end-user, integrator, academia) e governance WP (90-120 giorni)
- ✅ Eseguire pilot dimostrativo per salire di almeno 1 livello TRL (3-6 mesi)
- ✅ Integrare partner SSH e work package dedicato a impatti sociali/adozione

Spiegazione Tecnica AHP + LP

Matrice confronto a coppie (AHP)

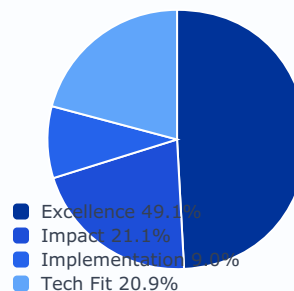
1.000	3.000	5.000	2.000
0.333	1.000	3.000	1.000
0.200	0.333	1.000	0.500
0.500	1.000	2.000	1.000

Pre-LP vs Post-LP

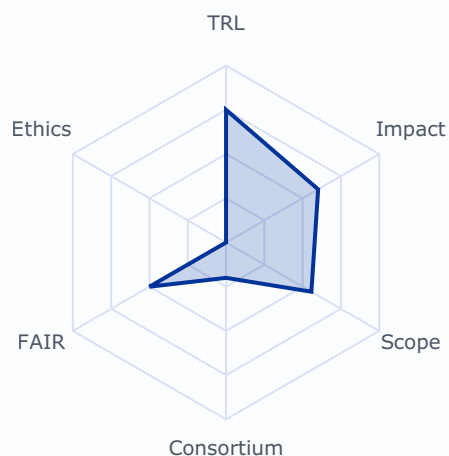
Criterio	Peso AHP	Local pre-LP	Optimal post-LP	Breakdown
excellence	0.4914	0.6011	0.3000	0.1474
impact	0.2105	0.6607	0.5000	0.1052
implementation	0.0896	0.5300	0.4000	0.0358
tech_fit	0.2086	0.5571	0.5571	0.1162

Consistency Ratio: 0.0168 — Matrice consistente

Pesi finali AHP (pie chart)



Spider chart call



Vincoli LP applicati e effetto

- **trl_violation:** True
- **budget_company_available:** 0.0
- **budget_max:** 45000000.0
- **sme_required:** False
- **sme_ok:** False
- **ssh_required:** True
- **gender_balance_required:** False

Expected Outcomes

This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Living and working in a health-promoting environment”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to most of the following expected outcomes:

- Researchers, policymakers, healthcare practitioners and the public have a more comprehensive understanding of the human exposome and the interactions between climatic, environmental and socio-behavioural factors, supported by FAIR102 data linking these exposures to disease and health outcomes.
- Researchers, governments, policymakers, social care services and healthcare practitioners have improved knowledge on the links between the climatic, social, lifestyle and environmental factors of the exposome and global health burden, supporting their efforts to adopt the exposome approach to identify and address relevant health impacts.
- The public has access to the latest information on the influence of global environmental exposures on health, enabling the adoption of health-promoting, climate-resilient and nature-positive behaviours.

Scope

The exposome is the totality of exposures (and their interactions) experienced by an individual throughout their lifetime, including chemical, physical, biological, nutritional and psychosocial factors, from conception onwards. Many of these factors originate in the environment, including climate-related exposures such as extreme heat, heightened air pollution or drought. Climate change may amplify or interact synergistically with other better-established exposures, dynamically altering the human exposome and its health implications. Despite this, climate factors remain underrepresented in large-scale human exposome studies. 102 See definition of FAIR data in the introduction to this Work Programme part.

Research activities under this topic should strengthen the use of the exposome approach to study global exposures and generate evidence on their health implications. Proposals should focus on integrating climate-related factors into exposome research and understanding how the exposome changes in response to direct and indirect climate exposures. Moreover, research activities should be multiscale and multidisciplinary and account for the complexity and multifactorial nature of health determinants and the most pressing unmet medical needs in relation to environmental degradation and disrupted ecosystems. Proposals should include climate-relevant social determinants of health as part of their proposed activities.

More specifically, research actions under this topic should include all the following activities:

- Incorporate multiple climate exposures into exposomics studies and provide insights on their influence on disease burden, through interactions with other exposome factors.
- Predict, identify and monitor changes in the exposome (including environmental, social and occupational exposures) resulting from climate-related pressures and study their health implications to identify emerging health risks and potential benefits of climate change.
- Advance data generation, analysis, integration and interpretation in human exposomics, developing methodologies and integrating novel approaches (e.g. AI technologies and machine learning) for advanced data analytics, including for Real-World Data (RWD)¹⁰³. In addition, research actions should include several of

Rank #30 · HORIZON-HLTH-2026-01-DISEASE-03

WATCH

Advancing research on the prevention, diagnosis, and management of post-infection long-term conditions

Fit: 40.08 / 100 · Solver: Optimal · CR: 0.0168

Perché questa call ha preso 40/100

La call HORIZON-HLTH-2026-01-DISEASE-03 ottiene un fit del 40.1% dopo ottimizzazione LP. Il criterio dominante e' excellence (contributo 14.7/100). Solver: Optimal, CR AHP: 0.0168.

"This topic aims at supporting activities that are enabling or contributing"

*"to one or several expected impacts of destination
"Tackling diseases and reducing disease"*

"burden". To that end, proposals under this topic should aim to deliver results that are directed"

Gap principali da colmare

- ⚠ Impact non ancora distintivo su outcomes e policy relevance
- ⚠ Implementation debole: piano esecutivo/consortium readiness non sufficiente
- ⚠ TRL aziendale inferiore al requisito della call (target: 4)
- ⚠ Requisito SME non soddisfatto dal profilo corrente
- ⚠ La call richiede copertura SSH esplicita

Azioni concrete (prossimi 3-6 mesi)

- ✅ Allineare proposition agli expected outcomes della call con KPI misurabili (60 giorni)
- ✅ Costruire shortlist partner (end-user, integrator, academia) e governance WP (90-120 giorni)
- ✅ Eseguire pilot dimostrativo per salire di almeno 1 livello TRL (3-6 mesi)
- ✅ Valutare ruolo da partner o struttura legale/cap table compatibile con requisiti SME
- ✅ Integrare partner SSH e work package dedicato a impatti sociali/adozione

Spiegazione Tecnica AHP + LP

Matrice confronto a coppie (AHP)

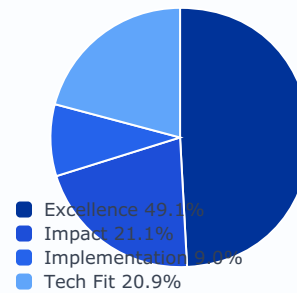
1.000	3.000	5.000	2.000
0.333	1.000	3.000	1.000
0.200	0.333	1.000	0.500
0.500	1.000	2.000	1.000

Pre-LP vs Post-LP

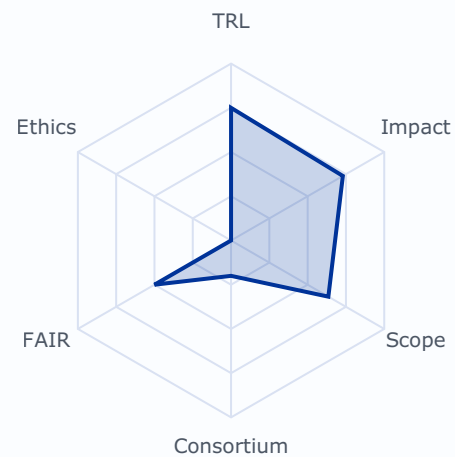
Criterio	Peso AHP	Local pre-LP	Optimal post-LP	Breakdown
excellence	0.4914	0.7299	0.3000	0.1474
impact	0.2105	0.4891	0.4891	0.1030
implementation	0.0896	0.5300	0.2000	0.0179
tech_fit	0.2086	0.6353	0.6353	0.1325

Consistency Ratio: 0.0168 — Matrice consistente

Pesi finali AHP (pie chart)



Spider chart call



Vincoli LP applicati e effetto

- **trl_violation:** True
- **budget_company_available:** 0.0
- **budget_max:** 39300000.0
- **sme_required:** True
- **sme_ok:** False
- **ssh_required:** True
- **gender_balance_required:** False

Expected Outcomes

This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Tackling diseases and reducing disease burden”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- All players along the healthcare value chain have access to evidence-based treatment and management strategies for post-infection conditions and improve patient recovery and quality of life across diverse populations.
- Public health authorities and healthcare practitioners have access to effective prevention, diagnostic and treatment tools, ensuring better allocation of healthcare resources.
- Healthcare systems improve their efficiency and reduce long-term economic burdens by streamlining post-infectious disease care and addressing disparities in healthcare access.

151 This decision is available on the Funding and Tenders Portal, in the reference documents section for Horizon Europe, under ‘Simplified costs decisions’ or through this link:

https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/lis-decision_he_en.pdf

- Public health authorities have access to evidence-based information to integrate research findings into policy for improved public health preparedness and resilience, including training of healthcare staff and enhanced long-term disease management guidelines.

Scope

Microbial infections can lead to long-lasting consequences on patients’ quality of life, leading to long-term conditions characterised by persistent inflammation, organ damage, and impaired functional capacity, which pose a growing public health and economic challenge.

These conditions are insufficiently understood, underdiagnosed, and lack effective treatments.

Advancing research into their prevention, treatment and management is essential to improving patient outcomes, reducing healthcare burdens, and strengthening workforce productivity.

The topic is open to long-term conditions resulting from infections by any type of microorganism (including viruses, bacteria, parasites, and fungi), which persist after the initial infection has been resolved. Research linked to cancer is excluded as it will be covered by the Cancer Mission.

Proposals should aim to develop innovative approaches for the prevention, diagnosis, and management of post-infection conditions. Proposals should address most of the following research areas:

- Identify protective and risk factors associated with the development of post-infection conditions to inform targeted prevention strategies, by integrating relevant information such as genetics, epigenetics, immune or inflammatory responses, and/or other relevant factors.
- Increase understanding of the pathophysiology of post-infection conditions (including inflammatory aspects) to identify biomarkers and develop clinically validated diagnostic approaches for early detection, disease progression and/or treatment optimisation.
- Develop and validate preventive and/or therapeutic interventions, including targeted pharmacological treatments, repurposing of existing drugs or precision medicine approaches, through early-stage clinical trials¹⁵² that demonstrate clinical safety and efficacy.
- Identify effective supportive rehabilitation approaches, including physical therapy, cognitive interventions, and psychological support, to enhance patient recovery, mental health and quality of life and evaluate their effectiveness.
- Examine best practices for integrating post-infectious disease management into primary and specialised healthcare settings, improving

Rank #31 · HORIZON-HLTH-2027-02-DISEASE-14-two-stage

WATCH

Clinical trials for advancing innovative interventions for neurodegenerative diseases

Fit: 39.85 / 100 · Solver: Optimal · CR: 0.0168

Perché questa call ha preso 40/100

La call HORIZON-HLTH-2027-02-DISEASE-14-two-stage ottiene un fit del 39.9% dopo ottimizzazione LP. Il criterio dominante e' excellence (contributo 14.7/100). Solver: Optimal, CR AHP: 0.0168.

"This topic aims at supporting activities that are enabling or contributing"

*"to one or several expected impacts of destination
"Tackling diseases and reducing disease"*

"burden". To that end, proposals under this topic should aim to deliver results that are directed"

Gap principali da colmare

- ⚠ Implementation debole: piano esecutivo/consortium readiness non sufficiente
- ⚠ TRL aziendale inferiore al requisito della call (target: 4)
- ⚠ Requisito SME non soddisfatto dal profilo corrente
- ⚠ La call richiede copertura SSH esplicita

Azioni concrete (prossimi 3-6 mesi)

- ✅ Costruire shortlist partner (end-user, integrator, academia) e governance WP (90-120 giorni)
- ✅ Eseguire pilot dimostrativo per salire di almeno 1 livello TRL (3-6 mesi)
- ✅ Valutare ruolo da partner o struttura legale/cap table compatibile con requisiti SME
- ✅ Integrare partner SSH e work package dedicato a impatti sociali/adozione

Spiegazione Tecnica AHP + LP

Matrice confronto a coppie (AHP)

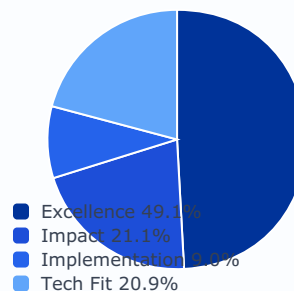
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0.333	1.000	3.000	1.000
0.200	0.333	1.000	0.500
0.500	1.000	2.000	1.000

Pre-LP vs Post-LP

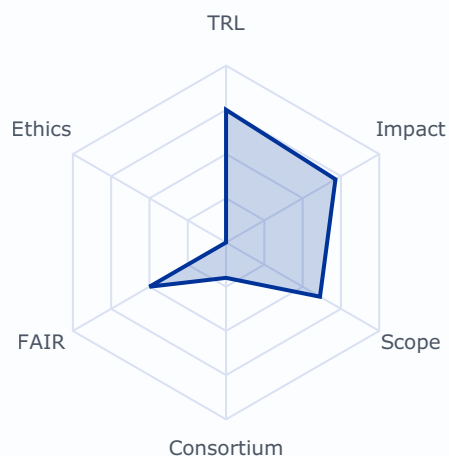
Criterio	Peso AHP	Local pre-LP	Optimal post-LP	Breakdown
excellence	0.4914	0.7143	0.3000	0.1474
impact	0.2105	0.8319	0.5000	0.1052
implementation	0.0896	0.5300	0.2000	0.0179
tech_fit	0.2086	0.6132	0.6132	0.1279

Consistency Ratio: 0.0168 — Matrice consistente

Pesi finali AHP (pie chart)



Spider chart call



Vincoli LP applicati e effetto

- **trl_violation:** True
- **budget_company_available:** 0.0
- **budget_max:** 39300000.0
- **sme_required:** True
- **sme_ok:** False
- **ssh_required:** True
- **gender_balance_required:** False

Expected Outcomes

This topic aims at supporting activities that are enabling or contributing

to one or several expected impacts of destination

“Tackling diseases and reducing disease burden”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- The scientific and clinical communities make effective use of state-of-the-art knowledge, data, technologies, tools, methods, best practices, and trainings to underpin and complement the development of innovative interventions aimed at more effective treatments for neurodegenerative diseases.
- The scientific and clinical communities benefit from the exchange of data, knowledge and best practices, thereby strengthening their collaboration in the EU, the Associated Countries and beyond.
- The scientific and clinical communities make wide use of relevant databases and/or integrate them with existing infrastructures for storage and sharing of collected data according to FAIR²⁸⁶ principles, thereby encouraging further use of the data.
- Policymakers, funders, scientific and clinical communities, patient organisations, regulators, and other relevant bodies are informed of the research advances made and the requirements for a widespread implementation of the innovative therapeutic interventions and complementary approaches.
- Patients and caregivers are constructively engaged with the research, ensuring that their needs are catered for, with the aim of tangibly benefitting from the interventions.

Scope

Neurodegenerative diseases are a high burden for patients, caregivers, health systems and society. Given the limitations with current therapeutic solutions, including that they primarily address symptoms rather than underlying causes and can have serious side effects, together with the increasing prevalence of neurodegenerative diseases in an aging population, there is a huge need to develop more innovative, safer and more effective therapeutic solutions for these diseases. To further enhance their safety and effectiveness, the therapeutic solution based on an active substance should be combined/complemented with another multidisciplinary approach (e.g. lifestyle changes, cognitive training, rehabilitation therapies).

Together this innovative intervention should lead to an improved quality of life and reduce the societal impact of these diseases.

Rare neurodegenerative diseases are excluded²⁸⁷.

²⁸⁶ See definition of FAIR data in the introduction to this Work Programme part.

Proposals should address most of the following aspects:

- Perform rigorous early-stage²⁸⁸ clinical trials into the safety and efficacy of the innovative interventions and their mode of administration, ensuring adequate cohorts/sample sizes with adequate representation of the patient population, including in terms of age, sex and ethnicity.
- Through the clinical trials and to the extent possible of additional studies, gain further insight into the potentially novel mechanism(s) of action of the innovative therapies and complementary approaches. This could entail analyses of imaging (e.g. MRI, ultrasound, nuclear imaging), as well as physiological, molecular, biochemical or omics signatures revealing potential perturbations prior to the intervention and recovery/improvement thereafter, and it could lead to the development of surrogate endpoints. This insight should open the path to more personalised interventions and approaches.
- Use and/or develop technologies, including digital ones (e.g. (generative) Artificial Intelligence - AI²⁸⁹, wearable technologies) to help implement and monitor the long-term efficacy of the intervention(s), as well as manage the disorder and/or monitor their progression (e.g. with unobtrusive technologies

Rank #32 · HORIZON-HLTH-2027-01-DISEASE-08

WATCH

Development of innovative antimicrobials against pathogens resistant to antimicrobials

Fit: 39.11 / 100 · Solver: Optimal · CR: 0.0168

Perché questa call ha preso 39/100

La call HORIZON-HLTH-2027-01-DISEASE-08 ottiene un fit del 39.1% dopo ottimizzazione LP. Il criterio dominante e' excellence (contributo 14.7/100). Solver: Optimal, CR AHP: 0.0168.

"This topic aims at supporting activities that are enabling or contributing"

*"to one or several expected impacts of destination
"Tackling diseases and reducing disease"*

"burden". To that end, proposals under this topic should aim to deliver results that are directed"

Gap principali da colmare

- ⚠ Impact non ancora distintivo su outcomes e policy relevance
- ⚠ Implementation debole: piano esecutivo/consortium readiness non sufficiente
- ⚠ TRL aziendale inferiore al requisito della call (target: 4)
- ⚠ Requisito SME non soddisfatto dal profilo corrente

Azioni concrete (prossimi 3-6 mesi)

- ✓ Allineare proposition agli expected outcomes della call con KPI misurabili (60 giorni)
- ✓ Costruire shortlist partner (end-user, integrator, academia) e governance WP (90-120 giorni)
- ✓ Eseguire pilot dimostrativo per salire di almeno 1 livello TRL (3-6 mesi)
- ✓ Valutare ruolo da partner o struttura legale/cap table compatibile con requisiti SME

Spiegazione Tecnica AHP + LP

Matrice confronto a coppie (AHP)

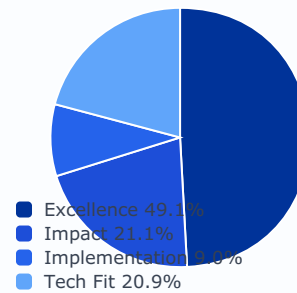
1.000	3.000	5.000	2.000
0.333	1.000	3.000	1.000
0.200	0.333	1.000	0.500
0.500	1.000	2.000	1.000

Pre-LP vs Post-LP

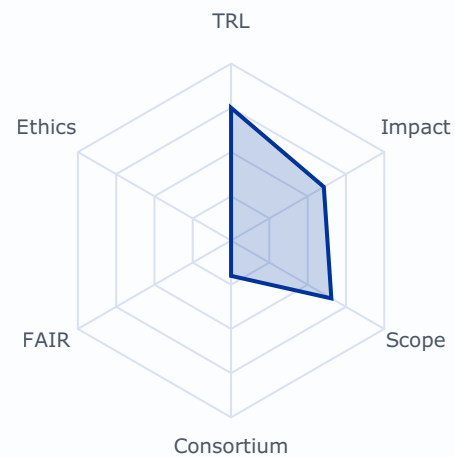
Criterio	Peso AHP	Local pre-LP	Optimal post-LP	Breakdown
excellence	0.4914	0.6056	0.3000	0.1474
impact	0.2105	0.4235	0.4235	0.0891
implementation	0.0896	0.5300	0.2000	0.0179
tech_fit	0.2086	0.6548	0.6548	0.1366

Consistency Ratio: 0.0168 — Matrice consistente

Pesi finali AHP (pie chart)



Spider chart call



Vincoli LP applicati e effetto

- **trl_violation:** True
- **budget_company_available:** 0.0
- **budget_max:** 44200000.0
- **sme_required:** True
- **sme_ok:** False
- **ssh_required:** False
- **gender_balance_required:** False

Expected Outcomes

This topic aims at supporting activities that are enabling or contributing

to one or several expected impacts of destination

“Tackling diseases and reducing disease

burden”. To that end, proposals under this topic should

aim to deliver results that are directed

at, tailored towards and contributing to all the following

expected outcomes:

- The scientific and clinical communities have a better understanding of and access to new and innovative products for the treatment of antimicrobial resistant bacteria and fungi, as well as for further clinical investigation.
- Candidate therapies are available to treat patients for antimicrobial resistant bacteria and fungi, increasing therapeutic options for clinical deployment in the fight against Antimicrobial Resistance (AMR).

Scope

The rapid rise of AMR presents a formidable threat to public health, challenging our ability to treat infections that were once easily managed with standard antimicrobials. As pathogens continually adapt and develop resistance to existing drugs, the efficacy of these treatments diminishes, leading to more severe and prolonged illnesses, increased healthcare costs and productivity losses, and higher mortality rates. This escalating crisis underscores an urgent need for viable therapeutic alternatives required to reduce the burden of diseases caused by antibiotic resistance. Innovative solutions are crucial to maintaining effective disease management and safeguarding public health. Proposals should pursue the development of innovative and effective antibacterial and antifungal agents, including antibody-based therapies, which meet at least one of the four 265 Pathogen i to vi, as given in the scope of this topic.

World Health Organization (WHO) innovation criteria²⁶⁶, namely: i) new chemical class, ii) new target, iii) new mode of action and iv) no evidence of cross-resistance.

This topic contributes to strengthening the Research and Innovation ecosystem within the EU and supports the implementation of the European Medical Countermeasures Strategy²⁶⁷ and the forthcoming Critical Medicines Act²⁶⁸.

Proposals under this topic should not pursue the development of phage-therapies.

Applicants should explicitly state in their proposal which of the following pathogens is targeted and the proposed work should address only this specific pathogen. The proposed work should pursue the development of existing therapeutic candidates targeting exclusively one of the following pathogens:

- Carbapenem resistant *Acinetobacter baumannii* (CRAB)
- Carbapenem-resistant Enterobacterales (CRE) and third-generation cephalosporin-resistant Enterobacterales (C3GRE)
- Carbapenem resistant *Pseudomonas aeruginosa*
- Methicillin-Resistant *Staphylococcus aureus* (MRSA)
- (Drug)-resistant *Aspergillus fumigatus*
- (Drug)-resistant *Candida* spp

Identifying a specific pathogen does not preclude the exploration of these candidates' effects on other bacteria or fungi. Proposals should thus aim to accelerate testing of novel candidates in human trials, diversify and accelerate the global

Rank #33 · HORIZON-HLTH-2027-01-IND-01

WATCH

Development of cell-free protein synthesis platforms for discovery and/or production of biologicals

Fit: 39.09 / 100 · Solver: Optimal · CR: 0.0168

Perché questa call ha preso 39/100

La call HORIZON-HLTH-2027-01-IND-01 ottiene un fit del 39.1% dopo ottimizzazione LP. Il criterio dominante e' excellence (contributo 14.7/100). Solver: Optimal, CR AHP: 0.0168.

"This topic aims at supporting activities that are enabling or contributing"

*"to one or several expected impacts of destination
"Maintaining an innovative, sustainable, and"*

"competitive EU health industry". To that end, proposals under this topic should aim to deliver"

Gap principali da colmare

- ⚠ Impact non ancora distintivo su outcomes e policy relevance
- ⚠ Implementation debole: piano esecutivo/consortium readiness non sufficiente
- ⚠ TRL aziendale inferiore al requisito della call (target: 4)
- ⚠ Requisito SME non soddisfatto dal profilo corrente

Azioni concrete (prossimi 3-6 mesi)

- ✅ Allineare proposition agli expected outcomes della call con KPI misurabili (60 giorni)
- ✅ Costruire shortlist partner (end-user, integrator, academia) e governance WP (90-120 giorni)
- ✅ Eseguire pilot dimostrativo per salire di almeno 1 livello TRL (3-6 mesi)
- ✅ Valutare ruolo da partner o struttura legale/cap table compatibile con requisiti SME

Spiegazione Tecnica AHP + LP

Matrice confronto a coppie (AHP)

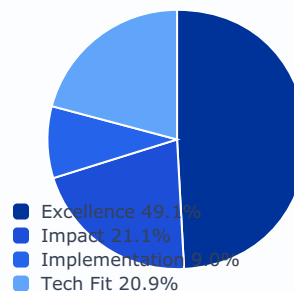
1.000	3.000	5.000	2.000
0.333	1.000	3.000	1.000
0.200	0.333	1.000	0.500
0.500	1.000	2.000	1.000

Pre-LP vs Post-LP

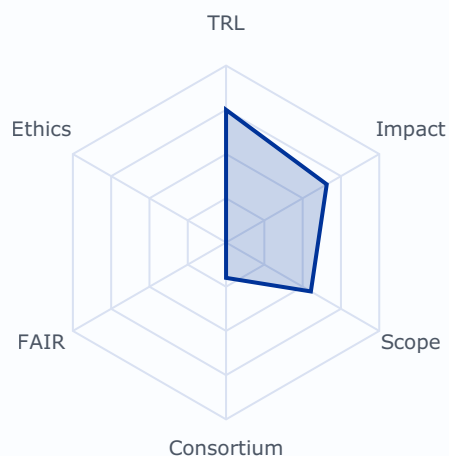
Criterio	Peso AHP	Local pre-LP	Optimal post-LP	Breakdown
excellence	0.4914	0.6573	0.3000	0.1474
impact	0.2105	0.5228	0.5228	0.1100
implementation	0.0896	0.5300	0.2000	0.0179
tech_fit	0.2086	0.5537	0.5537	0.1155

Consistency Ratio: 0.0168 — Matrice consistente

Pesi finali AHP (pie chart)



Spider chart call



Vincoli LP applicati e effetto

- **trl_violation:** True
- **budget_company_available:** 0.0
- **budget_max:** 24500000.0
- **sme_required:** True
- **sme_ok:** False
- **ssh_required:** False
- **gender_balance_required:** False

Expected Outcomes

This topic aims at supporting activities that are enabling or contributing

to one or several expected impacts of destination

“Maintaining an innovative, sustainable, and competitive EU health industry”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- Biopharmaceutical industries get access to streamlined development and production processes for peptide- or protein-based biologicals.
- Health systems benefit from the availability of enhanced or decentralised production systems for innovative health technologies that involve peptides or proteins, and which improve health and care.
- Citizens and patients will benefit from better access, availability and affordability of pharmaceuticals based on biologicals.

Scope

Cell-Free Protein Synthesis (CFPS) has been employed in fundamental biological research for decades, however, interest for the approach as a viable means for drug development and production has only emerged in recent years. The advantages that CFPS provides in terms of efficiency, simplicity, flexibility, cost- and time savings outweigh the hurdles that are still to be overcome for CFPS to become a routine manufacturing system for peptide- or protein-based biologicals. Currently, there are several CFPS systems used that are either based on prokaryotic or eukaryotic cell lysates (including mammalian) or fully synthetic systems consisting of all the molecular machinery necessary to create functional proteins. The choice of a specific lysate is dictated by the target protein and the end-use application. Proteins that require post-translational modification are generally produced using lysates of mammalian cells. Hence systems based on mammalian cells are of particular interest as they combine properties inherent to eukaryotic cells and their ability to produce human-like glycosylated proteins with the advantages of cell-free synthesis. These proteins include antibody fragments, antigens, virus-like particles, cytokines, enzymes, antimicrobial peptides and proteins containing non-natural amino acids. The benefits of CFPS are manifold, from ease of handling and scalability, on-demand launch of production, ability to rapidly switch products, simplified purification to facilitated standardisation and quality control. CFPS needs less energy

resources, the manufacturing footprint is less complex and smaller than in cell cultivation and it enables production of proteins that have toxic effects on cells. In addition, CFPS has the potential as an enabling technology for personalised medicines and is amenable to decentralised manufacturing. CFPS has gained even more interest in the recent past owing to advances in synthetic biology and thanks to the rise of Machine-Learning/Artificial Intelligence (ML/AI). The use of generative deep learning and artificial intelligence has high potential in the de-novo design of biomolecules with specific properties of therapeutic and/or preventive nature. CFPS offers here great opportunities to increase the throughput in screening of the de-novo created biomolecules.

Rank #34 · HORIZON-HLTH-2027-01-DISEASE-07

WATCH

Development of monoclonal antibodies to prevent and treat infections from Filo-, Nairo-, Phenui-, Picorna- and Toga viruses

Fit: 38.73 / 100 · Solver: Optimal · CR: 0.0168

Perché questa call ha preso 39/100

La call HORIZON-HLTH-2027-01-DISEASE-07 ottiene un fit del 38.7% dopo ottimizzazione LP. Il criterio dominante e' excellence (contributo 14.7/100). Solver: Optimal, CR AHP: 0.0168.

"This topic aims at supporting activities that are enabling or contributing"

*"to one or several expected impacts of destination
"Tackling diseases and reducing disease"*

"burden". To that end, proposals under this topic should aim to deliver results that are directed"

Gap principali da colmare

- ⚠ Impact non ancora distintivo su outcomes e policy relevance
- ⚠ Implementation debole: piano esecutivo/consortium readiness non sufficiente
- ⚠ TRL aziendale inferiore al requisito della call (target: 4)
- ⚠ Requisito SME non soddisfatto dal profilo corrente

Azioni concrete (prossimi 3-6 mesi)

- ✅ Allineare proposition agli expected outcomes della call con KPI misurabili (60 giorni)
- ✅ Costruire shortlist partner (end-user, integrator, academia) e governance WP (90-120 giorni)
- ✅ Eseguire pilot dimostrativo per salire di almeno 1 livello TRL (3-6 mesi)
- ✅ Valutare ruolo da partner o struttura legale/cap table compatibile con requisiti SME

Spiegazione Tecnica AHP + LP

Matrice confronto a coppie (AHP)

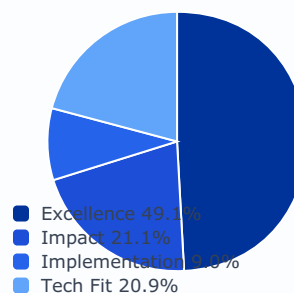
1.000	3.000	5.000	2.000
0.333	1.000	3.000	1.000
0.200	0.333	1.000	0.500
0.500	1.000	2.000	1.000

Pre-LP vs Post-LP

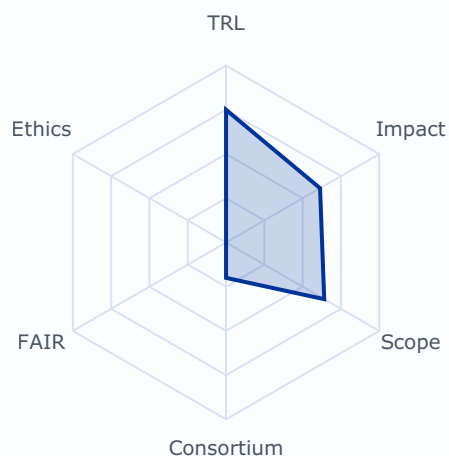
Criterio	Peso AHP	Local pre-LP	Optimal post-LP	Breakdown
excellence	0.4914	0.6137	0.3000	0.1474
impact	0.2105	0.4193	0.4193	0.0883
implementation	0.0896	0.5300	0.2000	0.0179
tech_fit	0.2086	0.6409	0.6409	0.1337

Consistency Ratio: 0.0168 — Matrice consistente

Pesi finali AHP (pie chart)



Spider chart call



Vincoli LP applicati e effetto

- **trl_violation:** True
- **budget_company_available:** 0.0
- **budget_max:** 37300000.0
- **sme_required:** True
- **sme_ok:** False
- **ssh_required:** False
- **gender_balance_required:** False

Expected Outcomes

This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Tackling diseases and reducing disease burden”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- The scientific and clinical communities have a better understanding of and access to experimental monoclonal antibodies for the prevention and treatment of emerging or re-emerging viral infections, as well as for further clinical investigation.
- Candidate monoclonal antibody therapies are available to treat patients for emerging and re-emerging viral infections, increasing therapeutic options for clinical deployment in case of an epidemic or pandemic.

Scope

Infectious diseases remain a major threat to health and health security in the EU and globally. Viral disease emergence is already being accelerated by climate change, and thus a proactive approach to the development of antiviral prophylactics and therapeutics in preparedness for future infectious disease outbreaks is needed. The capacity to produce antibodies that can target new variants and rapidly increase production would serve as an essential preparedness strategy against future health threats, whether from infectious disease epidemics or pandemics.

This topic contributes to strengthening the Research and Innovation ecosystem within the EU and supports the implementation of the European Medical Countermeasures Strategy²⁵⁹.

Applicants should explicitly state in their proposal which of the following viruses is targeted and the proposed work should address only this specific virus. The proposed work should aim to advance the development of existing prophylactic and therapeutic monoclonal antibody candidates targeting exclusively one of the following viruses:

258 Virus i to vi, as given in the scope of this topic.

259 https://health.ec.europa.eu/health-emergency-preparedness-and-response-hera/preparedness/medical-countermeasures-strategy_en

- Ebola Virus
- Marburg Virus
- Crimean-Congo Hemorrhagic Fever Virus
- Rift Valley Fever Virus
- Enterovirus D68
- Chikungunya Virus

Proposals should focus on antibodies produced or derived from a single cell clone through recombinant expression, such as B-cell derived antibodies, hybridoma derived antibodies and nanobodies.

Proposals should thus aim to diversify and accelerate the global prophylactic and therapeutic research and development portfolio for emerging and re-emerging viral infections, and to strengthen the leading role of the EU in prophylactic and therapeutic research and development.

Proposals should address all the following research areas:

- If necessary, finalisation of the in-vitro characterisation of existing monoclonal antibody

Rank #35 · HORIZON-HLTH-2027-01-DISEASE-06

WATCH

Development of monoclonal antibodies to prevent and treat infections from Flaviviruses

Fit: 38.66 / 100 · Solver: Optimal · CR: 0.0168

Perché questa call ha preso 39/100

La call HORIZON-HLTH-2027-01-DISEASE-06 ottiene un fit del 38.7% dopo ottimizzazione LP. Il criterio dominante e' excellence (contributo 14.7/100). Solver: Optimal, CR AHP: 0.0168.

"This topic aims at supporting activities that are enabling or contributing"

*"to one or several expected impacts of destination
"Tackling diseases and reducing disease"*

"burden". To that end, proposals under this topic should aim to deliver results that are directed"

Gap principali da colmare

- ⚠ Impact non ancora distintivo su outcomes e policy relevance
- ⚠ Implementation debole: piano esecutivo/consortium readiness non sufficiente
- ⚠ TRL aziendale inferiore al requisito della call (target: 4)
- ⚠ Requisito SME non soddisfatto dal profilo corrente

Azioni concrete (prossimi 3-6 mesi)

- ✓ Allineare proposition agli expected outcomes della call con KPI misurabili (60 giorni)
- ✓ Costruire shortlist partner (end-user, integrator, academia) e governance WP (90-120 giorni)
- ✓ Eseguire pilot dimostrativo per salire di almeno 1 livello TRL (3-6 mesi)
- ✓ Valutare ruolo da partner o struttura legale/cap table compatibile con requisiti SME

Spiegazione Tecnica AHP + LP

Matrice confronto a coppie (AHP)

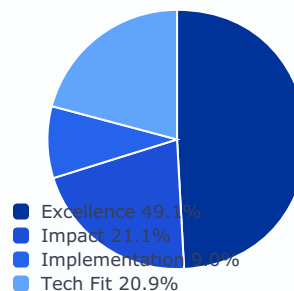
1.000	3.000	5.000	2.000
0.333	1.000	3.000	1.000
0.200	0.333	1.000	0.500
0.500	1.000	2.000	1.000

Pre-LP vs Post-LP

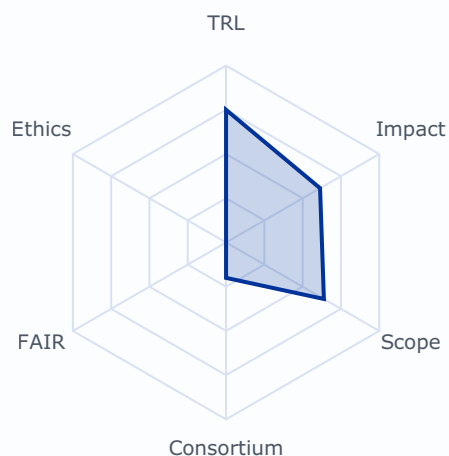
Criterio	Peso AHP	Local pre-LP	Optimal post-LP	Breakdown
excellence	0.4914	0.6137	0.3000	0.1474
impact	0.2105	0.4185	0.4185	0.0881
implementation	0.0896	0.5300	0.2000	0.0179
tech_fit	0.2086	0.6387	0.6387	0.1332

Consistency Ratio: 0.0168 — Matrice consistente

Pesi finali AHP (pie chart)



Spider chart call



Vincoli LP applicati e effetto

- **trl_violation:** True
- **budget_company_available:** 0.0
- **budget_max:** 37300000.0
- **sme_required:** True
- **sme_ok:** False
- **ssh_required:** False
- **gender_balance_required:** False

Expected Outcomes

This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Tackling diseases and reducing disease burden”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- The scientific and clinical communities have a better understanding of and access to experimental monoclonal antibodies for the prevention and treatment of emerging or re-emerging viral infections, as well as for further clinical investigation.
- Candidate monoclonal antibody therapies are available to treat patients for emerging and re-emerging viral infections, increasing therapeutic options for clinical deployment in case of an epidemic or pandemic.

Scope

Infectious diseases remain a major threat to health and health security in the EU and globally. Viral disease emergence is already being accelerated by climate change, and thus a proactive approach to the development of antiviral prophylactics and therapeutics in preparedness for future infectious disease outbreaks is needed. The capacity to produce antibodies that can target new variants and rapidly increase production would serve as an essential preparedness strategy against future health threats, whether from infectious disease epidemics or pandemics.

This topic contributes to strengthening the Research and Innovation ecosystem within the EU and supports the implementation of the European Medical Countermeasures Strategy²⁵².

Applicants should explicitly state in their proposal which of the following Flaviviruses is targeted and the proposed work should address only this specific Flavivirus. The proposed work should aim to advance the development of existing prophylactic and therapeutic monoclonal antibody candidates targeting exclusively one of the following Flaviviruses:

i. Dengue Virus

ii. Tick-borne Encephalitis Virus

251 Flavivirus i to vi, as given in the scope of this topic.

252 https://health.ec.europa.eu/health-emergency-preparedness-and-response-hera/preparedness/medical-countermeasures-strategy_en

iii. Japanese Encephalitis Virus

iv. West Nile Fever Virus

v. Yellow Fever Virus

vi. Zika Virus

Proposals should focus on antibodies produced or derived from a single cell clone through recombinant expression, such as B-cell derived antibodies, hybridoma derived antibodies and nanobodies.

Proposals should thus aim to diversify and accelerate the global prophylactic and therapeutic research and development portfolio for emerging and re-emerging viral infections, and to strengthen the leading role of the EU in prophylactic and therapeutic research and development.

Proposals should address all the following research areas:

- If necessary, finalisation of the in-vitro characterisation of existing monoclonal antibody

Rank #36 · HORIZON-HLTH-2026-01-DISEASE-04

WATCH

Development of novel vaccines for viral pathogens with epidemic potential

Fit: 38.42 / 100 · Solver: Optimal · CR: 0.0168

Perché questa call ha preso 38/100

La call HORIZON-HLTH-2026-01-DISEASE-04 ottiene un fit del 38.4% dopo ottimizzazione LP. Il criterio dominante e' excellence (contributo 14.7/100). Solver: Optimal, CR AHP: 0.0168.

"This topic aims at supporting activities that are enabling or contributing"

*"to one or several expected impacts of destination
"Tackling diseases and reducing disease"*

"burden". To that end, proposals under this topic should aim to deliver results that are directed"

Gap principali da colmare

- ⚠ Impact non ancora distintivo su outcomes e policy relevance
- ⚠ Implementation debole: piano esecutivo/consortium readiness non sufficiente
- ⚠ TRL aziendale inferiore al requisito della call (target: 4)
- ⚠ Requisito SME non soddisfatto dal profilo corrente

Azioni concrete (prossimi 3-6 mesi)

- ✓ Allineare proposition agli expected outcomes della call con KPI misurabili (60 giorni)
- ✓ Costruire shortlist partner (end-user, integrator, academia) e governance WP (90-120 giorni)
- ✓ Eseguire pilot dimostrativo per salire di almeno 1 livello TRL (3-6 mesi)
- ✓ Valutare ruolo da partner o struttura legale/cap table compatibile con requisiti SME

Spiegazione Tecnica AHP + LP

Matrice confronto a coppie (AHP)

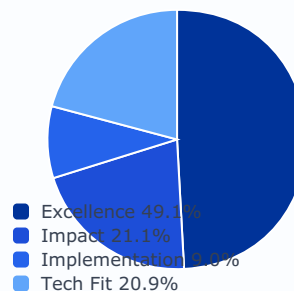
1.000	3.000	5.000	2.000
0.333	1.000	3.000	1.000
0.200	0.333	1.000	0.500
0.500	1.000	2.000	1.000

Pre-LP vs Post-LP

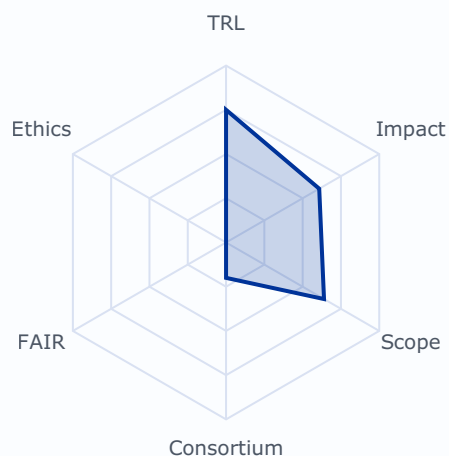
Criterio	Peso AHP	Local pre-LP	Optimal post-LP	Breakdown
excellence	0.4914	0.6081	0.3000	0.1474
impact	0.2105	0.4060	0.4060	0.0855
implementation	0.0896	0.5300	0.2000	0.0179
tech_fit	0.2086	0.6396	0.6396	0.1334

Consistency Ratio: 0.0168 — Matrice consistente

Pesi finali AHP (pie chart)



Spider chart call



Vincoli LP applicati e effetto

- **trl_violation:** True
- **budget_company_available:** 0.0
- **budget_max:** 44200000.0
- **sme_required:** True
- **sme_ok:** False
- **ssh_required:** False
- **gender_balance_required:** False

Expected Outcomes

This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Tackling diseases and reducing disease burden”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- The scientific and clinical communities have a better understanding of and access to experimental vaccines for the prevention and treatment of emerging or re-emerging viral infections, as well as for further clinical investigation, including better understanding of biological sex and social determinants that influence immune response, vaccine efficacy, safety, and uptake.
- Candidate vaccines are available for emerging and re-emerging viral infections, increasing therapeutic options for clinical deployment in case of an epidemic or pandemic.

Scope

Infectious diseases remain a major threat to health and health security in the EU and globally. Viral disease emergence is already being accelerated by climate change, and thus a proactive approach to the development of vaccine-based antiviral prophylactics and therapeutics in preparedness for future infectious disease outbreaks is needed. The availability of vaccines that can be adjusted to variants would provide a critical preparedness measure against future health threats, due to infectious disease epidemics or pandemics.

This topic contributes to strengthening the Research and Innovation ecosystem within the EU and supports the implementation of the European Medical Countermeasures Strategy¹⁶³.

162 Virus i to viii, as given in the scope of this topic.

163 https://health.ec.europa.eu/health-emergency-preparedness-and-response-hera/preparedness/medical-countermeasures-strategy_en

Applicants should explicitly state in their proposal which of the following viruses is targeted and the proposed work should address only this specific virus. The proposed work should aim to advance the development of existing prophylactic and therapeutic vaccine candidates targeting exclusively one of the following viruses:

- Junin mammarenavirus
- Lassa mammarenavirus
- Andes virus,
- Hantaan virus
- Sin Nombre virus
- Hendra virus
- Enterovirus D68
- Venezuelan equine encephalitis virus

Proposals should thus aim to diversify and accelerate the global prophylactic and therapeutic research and development portfolio for emerging and re-emerging viral infections, and to strengthen the leading role of the EU in prophylactic and therapeutic research and development.

Proposals should address all the following research areas:

- If necessary, finalisation of the in-vitro characterisation of existing vaccine candidates with regard to target specificity, epitope recognised, and their ability to impair or inactivate viral functions.
- In-vivo tests in at least one animal model or, if available in humanised immune system

Rank #37 · HORIZON-HLTH-2026-01-DISEASE-11

WATCH

Understanding of sex and/or gender-specific mechanisms of cardiovascular diseases: determinants, risk factors and pathways

Fit: 38.17 / 100 · Solver: Optimal · CR: 0.0168

Perché questa call ha preso 38/100

La call HORIZON-HLTH-2026-01-DISEASE-11 ottiene un fit del 38.2% dopo ottimizzazione LP. Il criterio dominante e' excellence (contributo 14.7/100). Solver: Optimal, CR AHP: 0.0168.

"This topic aims at supporting activities that are enabling or contributing"

*"to one or several expected impacts of destination
"Tackling diseases and reducing disease"*

"burden". To that end, proposals under this topic should aim to deliver results that are directed"

Gap principali da colmare

- ⚠ Implementation debole: piano esecutivo/consortium readiness non sufficiente
- ⚠ TRL aziendale inferiore al requisito della call (target: 4)
- ⚠ Requisito SME non soddisfatto dal profilo corrente
- ⚠ La call richiede copertura SSH esplicita

Azioni concrete (prossimi 3-6 mesi)

- ✅ Costruire shortlist partner (end-user, integrator, academia) e governance WP (90-120 giorni)
- ✅ Eseguire pilot dimostrativo per salire di almeno 1 livello TRL (3-6 mesi)
- ✅ Valutare ruolo da partner o struttura legale/cap table compatibile con requisiti SME
- ✅ Integrare partner SSH e work package dedicato a impatti sociali/adozione

Spiegazione Tecnica AHP + LP

Matrice confronto a coppie (AHP)

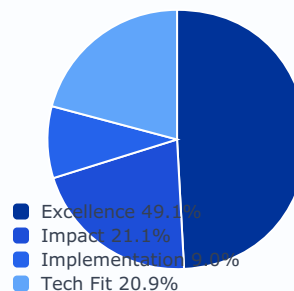
1.000	3.000	5.000	2.000
0.333	1.000	3.000	1.000
0.200	0.333	1.000	0.500
0.500	1.000	2.000	1.000

Pre-LP vs Post-LP

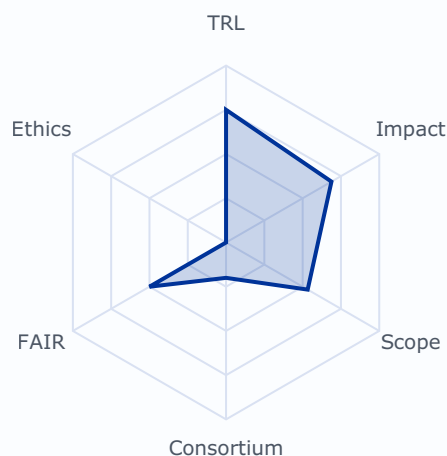
Criterio	Peso AHP	Local pre-LP	Optimal post-LP	Breakdown
excellence	0.4914	0.6881	0.3000	0.1474
impact	0.2105	0.5570	0.5000	0.1052
implementation	0.0896	0.5300	0.2000	0.0179
tech_fit	0.2086	0.5328	0.5328	0.1111

Consistency Ratio: 0.0168 — Matrice consistente

Pesi finali AHP (pie chart)



Spider chart call



Vincoli LP applicati e effetto

- **trl_violation:** True
- **budget_company_available:** 0.0
- **budget_max:** 39300000.0
- **sme_required:** True
- **sme_ok:** False
- **ssh_required:** True
- **gender_balance_required:** False

Expected Outcomes

This topic aims at supporting activities that are enabling or contributing

to one or several expected impacts of destination

“Tackling diseases and reducing disease burden”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- Researchers, developers of medical interventions, and healthcare professionals have a better understanding of biological sex and/or gender-specific health determinants, risk factors and pathways for cardiovascular diseases.
 - Researchers, developers of medical interventions, and healthcare professionals have access and use sex and/or gender-specific or tailored risk models for better prevention, detection and diagnostic and treatment strategies.
- 182 This decision is available on the Funding and Tenders Portal, in the reference documents section for Horizon Europe, under ‘Simplified costs decisions’ or through this link:
https://ec.europa.eu/info/funding-tenders/opportunities/docs/2021-2027/horizon/guidance/lis-decision_he_en.pdf
- Healthcare systems benefit from novel sex and/or gender-specific strategies for prevention, detection, diagnosis and treatment options, resulting in reduced burden of cardiovascular diseases.

Scope

Cardiovascular diseases (CVDs) are the leading cause of premature deaths in the EU and account for 32% of all deaths in 2021 (over 1.7 million deaths)¹⁸³.

Biological sex and gender play a specific role both in the incidence and the prevalence of certain diseases, including CVDs. Sex and gender disparities in CVDs are influenced by biological, behavioural, and sociocultural factors, affecting symptoms, prevalence, treatment, and outcomes. Hormonal influences, genetic predispositions, and/or physiological differences contribute to variations in how CVD presents and progresses in men and women. Risk factors such as diabetes, cholesterol, smoking, and age have different impacts across genders, highlighting the need for customised treatment strategies. Unique gender-specific conditions in women, such as menopause, pregnancy complications like preeclampsia and certain autoimmune diseases, also increase the risk for CVDs¹⁸⁴.

Mainstreaming a gender perspective into the research, prevention and control of CVDs is thus crucial to understanding and addressing the health risks and needs of women and men of all ages¹⁸⁵.

Although the significant progress has been done in investigating sex and/or gender-specific pathophysiological mechanisms of cardiovascular diseases, more research is needed to translate basic discoveries into the development of innovative prevention, detection, diagnosis, and treatment options.

Proposals should address most of the following aspects:

- Contribute to further the understanding on the structural, hormonal, and/or biological distinctions between sexes/genders to improve diagnostics and therapeutics for CVDs.
- Develop sex and/or gender-specific tailored risk models in a view of better prevention, detection and diagnostic, and treatment strategies.
- Identify and/or validate novel or existing sex and/or gender-specific health determinants, risk factors and pathways for cardiovascular disease(s) through the generation, integration and validation of data derived from relevant disciplines (e.g. molecular biology, behavioural science, nutrition, clinical, social and environmental epidemiology; exposure sciences; genetics and epigenetics, etc.).

183 European Union takes action for the

Rank #38 · HORIZON-HLTH-2027-01-DISEASE-05

WATCH

Development of novel small molecule antiviral therapeutics for pathogens with epidemic potential

Fit: 37.79 / 100 · Solver: Optimal · CR: 0.0168

Perché questa call ha preso 38/100

La call HORIZON-HLTH-2027-01-DISEASE-05 ottiene un fit del 37.8% dopo ottimizzazione LP. Il criterio dominante e' excellence (contributo 14.7/100). Solver: Optimal, CR AHP: 0.0168.

"This topic aims at supporting activities that are enabling or contributing"

*"to one or several expected impacts of destination
"Tackling diseases and reducing disease"*

"burden". To that end, proposals under this topic should aim to deliver results that are directed"

Gap principali da colmare

- ⚠ Impact non ancora distintivo su outcomes e policy relevance
- ⚠ Implementation debole: piano esecutivo/consortium readiness non sufficiente
- ⚠ TRL aziendale inferiore al requisito della call (target: 4)
- ⚠ Requisito SME non soddisfatto dal profilo corrente

Azioni concrete (prossimi 3-6 mesi)

- ✅ Allineare proposition agli expected outcomes della call con KPI misurabili (60 giorni)
- ✅ Costruire shortlist partner (end-user, integrator, academia) e governance WP (90-120 giorni)
- ✅ Eseguire pilot dimostrativo per salire di almeno 1 livello TRL (3-6 mesi)
- ✅ Valutare ruolo da partner o struttura legale/cap table compatibile con requisiti SME

Spiegazione Tecnica AHP + LP

Matrice confronto a coppie (AHP)

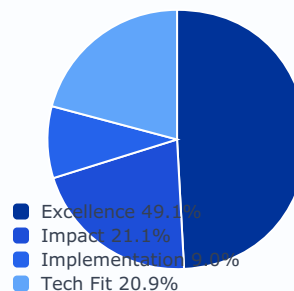
1.000	3.000	5.000	2.000
0.333	1.000	3.000	1.000
0.200	0.333	1.000	0.500
0.500	1.000	2.000	1.000

Pre-LP vs Post-LP

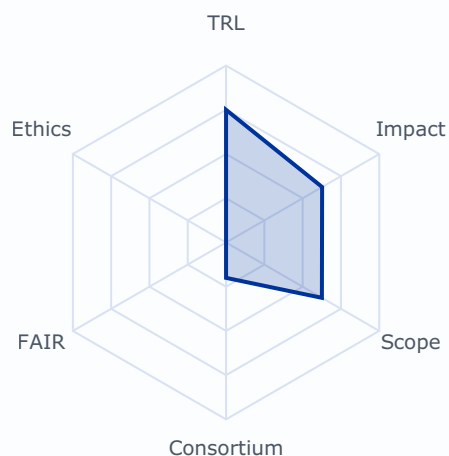
Criterio	Peso AHP	Local pre-LP	Optimal post-LP	Breakdown
excellence	0.4914	0.6272	0.3000	0.1474
impact	0.2105	0.3898	0.3898	0.0820
implementation	0.0896	0.5300	0.2000	0.0179
tech_fit	0.2086	0.6257	0.6257	0.1305

Consistency Ratio: 0.0168 — Matrice consistente

Pesi finali AHP (pie chart)



Spider chart call



Vincoli LP applicati e effetto

- **trl_violation:** True
- **budget_company_available:** 0.0
- **budget_max:** 44200000.0
- **sme_required:** True
- **sme_ok:** False
- **ssh_required:** False
- **gender_balance_required:** False

Expected Outcomes

This topic aims at supporting activities that are enabling or contributing to one or several expected impacts of destination “Tackling diseases and reducing disease burden”. To that end, proposals under this topic should aim to deliver results that are directed at, tailored towards and contributing to all the following expected outcomes:

- The scientific and clinical communities have a better understanding of and access to experimental antivirals for the prevention and treatment of emerging or re-emerging viral infections, as well as for further clinical investigation.
- Candidate antiviral therapies are available to treat patients for emerging and re-emerging viral infections, increasing therapeutic options for clinical deployment in case of an epidemic or pandemic.

Scope

Infectious diseases remain a major threat to health and health security in the EU and globally. Viral disease emergence is already being accelerated by climate change, and thus a proactive approach to the development of antiviral prophylactics and therapeutics in preparedness for future infectious disease outbreaks is needed. The availability of antivirals targeting conserved viral or host mechanisms would provide a critical preparedness measure against future health threats caused by (re)emerging infectious disease epidemics or pandemics, due to infectious disease epidemics or pandemics.

This topic contributes to strengthening the Research and Innovation ecosystem within the EU and supports the implementation of the European Medical Countermeasures Strategy²⁴⁴.

Antibodies and antibody derived proteins are excluded from the scope of this topic.

Applicants should explicitly state in their proposal which of the following viruses/groups of viruses is targeted and the proposed work should address only this specific virus/group of viruses. The proposed work should aim to advance the development of novel or existing antiviral candidates targeting exclusively one of the following viruses/groups of viruses:

- Junin mammarenavirus and/or Lassa mammarenavirus
- Tick-borne encephalitis virus and/or Japanese encephalitis virus
- Andes virus and/or Hantaan virus and/or Sin Nombre virus
- Hendra virus
- Enterovirus D68
- Venezuelan equine encephalitis virus

²⁴⁴ https://health.ec.europa.eu/health-emergency-preparedness-and-response-hera/preparedness/medical-countermeasures-strategy_en

Identifying a specific virus/group of viruses does not preclude the exploration of these antiviral candidates' effects on other viruses/groups of viruses. Proposals should thus aim to diversify and accelerate the global prophylactic and therapeutic research and development portfolio for emerging and re-emerging viral infections, and to strengthen the leading role of the EU in prophylactic and therapeutic research and development.

Proposals should address some of the following



Disclaimer

Report generato automaticamente da Horizon Radar v1.0 – Strumento decisionale basato su Analytic Hierarchy Process (AHP) + Linear Programming (Gurobi). I risultati sono indicativi.

Generative Bionics R&D Team

15/04/2026 01:04